

GOVERNMENT OF ALBERTA



Mental Health and Addiction

Annual Report
2024-25

Alberta 

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Annual Report

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Table of Contents

Preface	3
Minister's Accountability Statement	4
Message from the Minister	5
Management's Responsibility for Reporting	7
Results Analysis	9
Ministry Overview	10
Discussion and Analysis of Results	16
Performance Measure and Indicator Methodology	64
Financial Information	67
Reporting Entity and Method Consolidation	68
Ministry Financial Highlights	69
Financial Statements of Other Reporting Entities	76
Financial Information	128
Annual Report Extracts and Other Statutory Reports	131

Preface

The Public Accounts of Alberta are prepared in accordance with the *Financial Administration Act* and the *Sustainable Fiscal Planning and Reporting Act*. The Public Accounts consist of the annual report of the Government of Alberta and the annual reports of each ministry.

The 2024-25 Annual Report reflects the 2024-27 Ministry Business Plans, the Government of Alberta Strategic Plan, as well as the ministry's activities and accomplishments during the 2024-25 fiscal year, which ended on March 31, 2025.

The Annual Report of the Government of Alberta contains *Budget 2024* Key Results, the audited Consolidated Financial Statements and Performance Results, which compares actual performance results to desired results set out in the government's strategic plan.

This annual report of the Ministry of Mental Health and Addiction contains the Minister's Accountability Statement, the ministry's Financial Information and Results Analysis, a comparison of actual performance results to desired results set out in the Ministry Business Plan. This ministry annual report also includes:

- other financial information as required by the *Financial Administration Act* and *Sustainable Fiscal Planning and Reporting Act*, as separate reports, to the extent that the ministry has anything to report.

All Ministry Annual Reports should be considered along with the Government of Alberta Annual Report to provide a complete overview of government's commitment to openness, accountability, and fiscal transparency.

Minister's Accountability Statement

The ministry's annual report for the year ended March 31, 2025, was prepared under my direction in accordance with the *Sustainable Fiscal Planning and Reporting Act* and the government's accounting policies. All the government's policy decisions as at June 5, 2025, with material economic or fiscal implications of which I am aware have been considered in the preparation of this report.

[Original signed by]

Honourable Rick Wilson
Minister of Mental Health and Addiction

Message from the Minister



Over the past year, Alberta's government has made significant progress building a comprehensive, recovery-oriented system of care—the Alberta Recovery Model. This model is a first-of-its kind approach to support people in maintaining their wellness and ensure those facing mental health challenges or addiction have access to evidence-based services and supports to pursue recovery.

Since 2019, Alberta's government has increased mental health supports, added more than 10,000 publicly funded addiction treatment spaces, eliminated user fees for live-in addiction treatment, and expanded access to the award-winning Virtual Opioid Dependency Program, which offers same-day addiction medicine treatment.

The next monumental step was to prioritize mental health and addiction care in Alberta through the establishment of Recovery Alberta and the Canadian Centre of Recovery Excellence (CoRE). Since September 2024, publicly funded mental health, addiction, and correctional health services are delivered by a dedicated provincial health agency—Recovery Alberta. More than 10,000 staff and approximately 800 physicians transferred from Alberta Health Services to Alberta's first provincial health agency focused solely on mental health and addiction. This structural reform—a core part of refocusing Alberta's health system—was completed without disruptions to patient care, and represents a crucial step forward in delivering more coordinated, focused, and recovery-oriented services across the province. Building on this critical work is CoRE's mission to empower decision-makers with data and evidence to support those with mental illness or addiction in their pursuit of recovery. CoRE is the first organization of its kind in Canada to focus on system-level evaluation and research, comprehensive data analytics and leadership collaboration to enable strong recovery outcomes for people across Alberta.

A significant component of the Alberta Recovery Model is expanding long-term addiction treatment capacity by building 11 world-class recovery communities across the province. Once completed, these facilities will provide no-cost, long-term, holistic addiction treatment to more than 2,000 people living in Alberta annually. In July 2024, the Lakeview Recovery Community opened in Gunn, joining facilities already operating in Red Deer and Lethbridge. These communities supported more than 400 people in their pursuit of recovery over the past year.

Police and Justice partners continue to have an important role in responding to addiction challenges. Alberta's government works with police services in Edmonton and Calgary to connect those in need with critical supports through Human-centred Engagement and Liaison Partnership teams and Police and Crisis Teams. Therapeutic living units, correctional health, and transitional services are also offered in provincial correctional and remand centres, ensuring incarcerated individuals are supported in addressing their health needs and rebuilding aspects of their life eroded by addiction.

A key part of strengthening recovery-oriented care across the province is the continued focus on partnering with Indigenous communities to remove barriers to comprehensive, culturally appropriate services. Five Indigenous recovery communities are underway in partnership with Siksika Nation, Tsuut'ina First Nation, Enoch Cree Nation, Métis Nation of Alberta, and Blood Tribe.

The mental health and well-being of Albertans, regardless of where they live, is also a top priority. Through the department's continued partnership with Counselling Alberta, people can access low-cost, no-waitlist counselling services online or in-person. Those struggling can also connect with local mental health and addiction supports through 211 Alberta.

Supporting younger people is another vital part of the Alberta Recovery Model, ensuring children and youth receive the mental health care they need to live healthy, full lives. In partnership with CASA Mental Health, the department invested in critical mental health supports for children and youth, including funding for new inpatient CASA House sites, expanding youth day treatment programs, and adding 20 new mental health classrooms in 13 communities.

Data from Recovery Alberta and the Alberta Substance Use Surveillance System is highlighting early signs of success in our government's commitment to the Alberta Recovery Model:

- In 2024 compared to 2023, Alberta recorded a 37 per cent reduction in opioid-related deaths and EMS responses for opioid related events decreased by 37 per cent.
- Also in 2024, emergency department visits related to opioids decreased by 26 per cent and hospitalizations related to opioids decreased by 16 per cent compared to the previous year.
- Prescribed opioid overdoses are near zero, and deaths from methamphetamine, alcohol, and cocaine are down significantly from previous years.
- Recovery Alberta's opioid dependency program supported a record number of about 12,300 unique clients in 2024, with more than 10,000 treated. This includes about 8,300 unique clients supported by the Virtual Opioid Dependency Program, with 6,400 receiving treatment.
- More than 10,000 unique clients were served in addiction treatment beds in 2024-25.
- From April 1, 2024, to December 31, 2024, more than 12,000 unique clients were treated for an addiction or mental health concern in acute care settings.
- More than 151,000 unique clients received mental health and addiction treatment through nearly 1.9 million visits in community outpatient settings in 2024-25.

While these data indicators are encouraging, efforts to strengthen Alberta's recovery-oriented system of care continue. Over the past year, the groundwork was laid for delivering on promised compassionate intervention legislation to support the health, wellness, and recovery of people facing severe addiction challenges and in turn, restore safety for families and communities. Planning ahead, Alberta's government has committed to building two new 150-bed compassionate intervention centres in Edmonton and Calgary, with initial spaces for launch in 2026.

The results achieved by the department over the past year are significant steps forward in ensuring those with mental health or addiction challenges have access to evidence-based supports and services to pursue recovery and live a purpose-filled life. With Recovery Alberta and CoRE now operational, legislation in place, and significant capital investments underway, our province is leading Canada in building a system rooted in recovery, compassion, evidence, and results.

I look forward to working with the department and all our partners in building on our many successes and continuing to move the Alberta Recovery Model forward.

[Original signed by]

Honourable Rick Wilson
Minister of Mental Health and Addiction

Management's Responsibility for Reporting

The Ministry of Mental Health and Addiction includes the Department of Mental Health and Addiction, Recovery Alberta: Mental Health and Addiction Services, and the Canadian Centre of Recovery Excellence. The executives of the individual entities within the ministry have the primary responsibility and accountability for the respective entities. Collectively, the executives ensure the ministry complies with all relevant legislation, regulations and policies.

Ministry business plans, annual reports, performance results and the supporting management information are integral to the government's fiscal and strategic plan, annual report, quarterly reports, and other financial and performance reporting.

Responsibility for the integrity and objectivity of the accompanying ministry financial information and performance results for the ministry rests with the Minister of Mental Health and Addiction. Under the direction of the Minister, I oversee the preparation of the ministry's annual report, which includes the financial information, performance results on all objectives and initiatives identified in the Ministry Business Plan, and performance results for all ministry-supported commitments that were included in the 2024-27 Government of Alberta Strategic Plan. The financial information and performance results, out of necessity, include amounts that are based on estimates and judgments. The financial information is prepared using the government's stated accounting policies, which are based on Canadian public sector accounting standards. The performance measures are prepared in accordance with the following criteria:

- Reliable – information used in applying performance measure methodologies agrees with the underlying source data for the current and prior years' results.
- Understandable – the performance measure methodologies and results are presented clearly.
- Comparable – the methodologies for performance measure preparation are applied consistently for the current and prior years' results.
- Complete – outcomes, performance measures and related targets match those included in the ministry's *Budget 2024*.

As Deputy Minister, in addition to program responsibilities, I am responsible for the ministry's financial administration and reporting functions. The ministry maintains systems of financial management and internal control which give consideration to costs, benefits and risks that are designed to:

- provide reasonable assurance that transactions are properly authorized, executed in accordance with prescribed legislation and regulations, and properly recorded so as to maintain accountability of public money;
- provide information to manage and report on performance;
- safeguard the assets and properties of the province under ministry administration;
- provide Executive Council, the President of Treasury Board and Minister of Finance and the Minister of Mental Health and Addiction the information needed to fulfill their responsibilities; and
- facilitate preparation of Ministry business plans and annual reports required under the *Sustainable Fiscal Planning and Reporting Act*.

In fulfilling my responsibilities for the ministry, I have relied, as necessary, on the executives of the individual entities within the ministry.

[Original signed by]

Evan Romanow
Deputy Minister of Mental Health and Addiction
June 5, 2025

Results Analysis

Table of Contents

Ministry Overview..... 10

 Organizational Structure 10

 Operational Overview..... 12

 Key Highlights 14

Discussion and Analysis of Results..... 16

 Actions that support the priorities of the Government of Alberta Strategic Plan..... 16

 Outcome One: Alberta has a comprehensive, integrated, and responsive
recovery-oriented addiction and mental health system 18

 Outcome Two: Alberta’s communities and families have the capacity to support
individuals pursuing recovery 36

 Outcome Three: Albertans have access to high-quality, person-centred mental
health and addiction programs and services 45

Performance Measure and Indicator Methodology..... 64

Ministry Overview

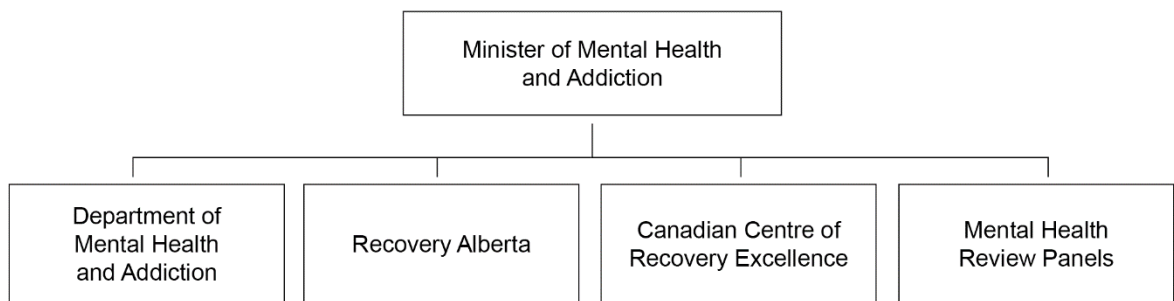
The Ministry of Mental Health and Addiction leads implementation of the Alberta Recovery Model and coordinates all Government of Alberta funding dedicated to planning and delivering mental health and addiction services across the province spanning prevention, intervention, treatment, and recovery.

The ministry is working to establish a comprehensive recovery-oriented system of care, prioritizing initiatives that build mental health and addiction system capacity to increase the recovery capital¹ for Albertans pursuing recovery. In collaboration with health, justice, education, and social sector partner ministries, Mental Health and Addiction provides support to develop integrated, coordinated networks of community-based services and supports that are person-centred and build on the strengths and resilience of individuals, families, and communities. Alberta’s recovery-oriented system of care is focused on keeping communities safe while treating mental health and addiction as health and social issues, rather than criminal issues.

The Ministry of Mental Health and Addiction relies on collaboration with external organizations responsible for service delivery, Indigenous partners and communities, academic and research institutions, a range of health professionals, and other government jurisdictions to deliver high-quality mental health and addiction services and implement the Alberta Recovery Model.

Organizational Structure

As of March 31, 2025, the ministry consists of the Department of Mental Health and Addiction, Recovery Alberta: Mental Health and Addiction Services (Recovery Alberta), the Canadian Centre of Recovery Excellence (CoRE), and the Mental Health Review Panels.



The department sets strategic direction for the mental health and addiction system in alignment with ministerial priorities and community needs. Responsibilities include policy, legislation,

¹ Recovery capital is the combination of personal, interpersonal and community resources that an individual can draw upon to begin and sustain recovery. These addiction and mental health interventions and recovery-oriented supports fall under eight recovery domains: physical and mental health; safe housing and healthy environments; employment and resolution of legal issues; vocational skills and education development; safe and meaningful family, social and leisure activities; peer-based supports, community engagement and cultural supports; and (re)discovering meaning and purpose of life.

funding allocation and management, and regulatory development; system planning, oversight, governance, and accountability; licensing and monitoring service providers for regulatory compliance; capital planning; and leading the provincial hazards psychosocial response. The department coordinates with Recovery Alberta and CoRE on partnerships to address the mental health and addiction needs of Albertans; procurement, contract, and funding management; and data governance, collection, management, analytics, and public reporting.

Recovery Alberta (<https://www.recoveryalberta.ca/>) began operating on September 1, 2024 as the provincial health agency providing comprehensive and accessible recovery-oriented mental health, addiction, and correctional health services to Albertans to help them improve their overall health and sustain recovery. Recovery Alberta's responsibilities include delivery, oversight, coordination, and provision of clinical services, such as emergency and crisis response, community treatment and support services, acute and specialized services, prevention and mental health promotion, education, and early intervention, as well as all health services in provincial correctional settings.

Recovery Alberta is focused on:

- Ensuring every Albertan struggling with addiction and/or mental health challenges is supported in their pursuit of recovery.
- Providing Albertans with access to a full continuum of recovery-oriented supports that help them improve overall health and sustain recovery.
- Improving mental health and addiction care for Albertans by further expanding access to treatment and recovery supports across Alberta.

CoRE (<https://recoveryexcellence.org/>) is a statutory corporation that empowers decision makers with data and information to support people with mental illness or addiction in their pursuit of recovery. CoRE's responsibilities include conducting system-level evaluation and research; monitoring system performance and analyzing data to develop evidence-based recommendations and inform policies and practices; providing advice and guidance on the effectiveness of the Alberta Recovery Model; and collaborating with stakeholders to ensure innovative and effective strategies related to mental health and addiction issues.

Mental Health Review Panels (<https://www.alberta.ca/mental-health-review-panel-roster>) are adjudicative bodies that hear applications under the *Mental Health Act* (<https://open.alberta.ca/publications/m13>). Panels play an important role in protecting the rights of patients detained in facilities and individuals subject to community treatment orders under the act. The Minister of Mental Health and Addiction appoints a roster of members who are called to serve on review panels.

The Mental Health Patient Advocate (MHPA) (<https://www.alberta.ca/alberta-mental-health-advocate>) is an independent body that is not part of a provincial health agency, hospital, clinic or treatment team. The MHPA reports annually to the Legislature through the Minister of Mental Health and Addiction and is responsible for providing support to people who have been involuntarily detained in designated mental health facilities or who are the subject of community treatment orders under the *Mental Health Act*. The MHPA helps Albertans understand their rights under the *Mental Health Act*, navigate the mental health system, and investigates complaints.

Operational Overview

With direction from the Minister of Mental Health and Addiction, the Deputy Minister is responsible for the daily operations of the Department of Mental Health and Addiction, structured as follows:

Deputy Minister's Office

The Deputy Minister's Office (DMO) supports the Minister in achieving the strategic goals of Alberta's government, the ministry, and public service. The DMO provides leadership within the mental health and addiction system to drive innovation and deliver quality programs and services to Albertans. The DMO plays a critical role in building and maintaining collaborative and effective relationships with Recovery Alberta and CoRE, across government ministries, and within Alberta's mental health and addiction system to ensure the effective operations of the mental health and addiction system. The DMO provides leadership to the department in issues management and policy coordination, as well as guidance in priority setting, decision-making, and coordinating all correspondence for the Minister and Deputy Minister.

Policy and Programs Division

The Policy and Programs Division develops and implements government mental health and addiction legislation and policy as well as administers and oversees mental health and addiction program funding to support the Alberta Recovery Model. The division is comprised of two branches—Legislation and Community Partnerships and Community Response and Programs—and a grant administration support team, all of which work with other government ministries, Recovery Alberta, CoRE, and other community partners to achieve the ministry's mandate. Key initiatives led and supported by the division include developing and overseeing legislation and legislative tools; child and youth mental health and addiction; Indigenous community partnerships; supporting community-led responses to mental health and addiction issues across the continuum of care; and expanding access to mental health and addiction supports throughout the province.

System Overview and Strategic Services Division

The System Overview and Strategic Services Division provides leadership that defines and supports the department's strategic direction as well as oversight to ensure safe and high-quality mental health and addiction services. The division is comprised of two branches—Strategic and Corporate Services and Regulatory Compliance and Data Stewardship—that coordinate strategic initiatives and provide advice, evidence-based data and information, and regulatory oversight to advance Alberta's recovery-oriented system of care. Programs and initiatives led by the division include mental health and addiction system transformation; strategic policy development; legislative and intergovernmental relations coordination; agency governance oversight and support; business continuity and emergency response planning; occupational health and safety; privacy legislation compliance; oversight of data analytics, performance measurement, and business intelligence; and licensing, regulatory enforcement, and monitoring for service provider compliance to the *Mental Health Services Protection Act* (MHSPA) (<https://open.alberta.ca/publications/m13p2>).

Financial Services Division

The Financial Services Division provides strategic financial advice related to legislation, regulations, programs, and policies. The division is comprised of two branches—Capital Projects and Planning and Financial Services and Planning—and a Corporate Planning and Reporting unit. The division develops and manages the ministry's budget and forecasts; monitors financial activities of the department; oversees financial reporting, corporate planning and reporting, and ensures Recovery Alberta and CoRE fiscal accountability; coordinates the department's grant approval process; and leads capital projects and planning for mental health and addiction facilities, in collaboration with the Ministry of Infrastructure as well as with Indigenous communities.

Communications (Communications and Public Engagement [CPE] – Mental Health and Addiction)

CPE provides Albertans and system partners with information about ministry policies, programs, and initiatives. The team offers communications support, such as media relations, issues management, writing and editing services, product development, and online communications services. The team also works closely with Recovery Alberta, CoRE, and other reporting entities to coordinate ministry communications.

Human Resources

Human Resources services are provided by the Public Service Commission. The team dedicated to Mental Health and Addiction provides strategic advice, support, and expertise in employee and labour relations, occupational health and safety, workforce development, and engagement. Human Resources works in partnership with managers and employees to build and sustain workforce capacity and create an environment where employees are respected, valued, and heard.

Key Highlights

The Ministry of Mental Health and Addiction focused on accomplishing three outcomes identified in the 2024-27 Ministry Business Plan:

- Alberta has a comprehensive, integrated, and responsive recovery-oriented addiction and mental health system.
- Alberta's communities and families have the capacity to support individuals pursuing recovery.
- Albertans have access to high-quality, person-centred mental health and addiction programs and services.

Key highlights and results achieved by the Ministry of Mental Health and Addiction in 2024-25 include:

- **Established Recovery Alberta as part of refocusing health care system:** Transitioned more than 10,000 Alberta Health Services staff and about 800 physicians to Recovery Alberta, the new provincial health agency for mental health and addiction. Invested \$1.5 billion² to deliver mental health and addiction services province-wide (details on pages [18-20](#)).
- **Launched Canadian Centre of Recovery Excellence (CoRE):** Established CoRE as a statutory corporation to lead system-level evaluation and research to support decision makers, shape best practices, and inform data-driven policies that support advancement of the Alberta Recovery Model (details on pages [20-21](#)).
- **Advanced Compassionate Intervention:** Introduced the proposed *Compassionate Intervention Act* to address the needs of those whose substance use or addiction causes harm to themselves or others. Announced plans to develop two 150-bed compassionate intervention centres in Edmonton and Calgary to support implementation of the act (details on pages [27](#) and [38](#)).
- **Continued to build and operate recovery communities:** Invested more than \$106 million (about \$85.4 million in capital and more than \$21 million in operating) to build and operate 11 recovery communities, including five in direct partnership with Indigenous communities. Opened the Lakeview Recovery Community in Gunn. Served more than 410 individuals at the operational recovery communities in Red Deer, Lethbridge, and Gunn (details on pages [36-38](#)).
- **Expanded children and youth mental health supports:** Investing \$66 million (2023-26) to establish up to 60 mental health classrooms. Opened 12 new classrooms bringing the total to 20 operational classrooms in 13 communities. Provided \$29 million (\$18 million in capital and \$11 million operating) to expand the CASA House program to Fort McMurray, Calgary, and Medicine Hat over the next three years (details on pages [41](#) and [54](#)).
- **Strengthened prevention and intervention services:** Provided more than \$2.9 million to enhance 211 Alberta's navigation services for rural and underserved communities. The service logged about 117,000 interactions by phone, text, and chat, up 33 per cent from previous year. Invested more than \$6.5 million to deliver accessible, affordable, and no-wait-list counselling services through Counselling Alberta. (details on pages [48-49](#)).
- **Enhanced culturally relevant supports for Indigenous communities:** Continued to plan and build five Indigenous-led recovery communities. Broke ground at the Siksika Nation Recovery Community and announced the Healing Waters Recovery Community in partnership with the

² Financial figures attributed to Recovery Alberta in the Results Analysis section of this report are based on restated numbers (April 1, 2024-March 31, 2025). This differs from the audited financial statements in the Financial Information section of this report which are based on the nine months Recovery Alberta has been a legal provincial health agency (July 1, 2024-March 31, 2025).

Métis Nation of Alberta. Provided \$4.6 million to support culturally appropriate addiction and mental health initiatives (details on pages [42-44](#)).

- **Partnered with police services:** Supported Police and Crisis Teams (PACT/RPACT) and Human-centred Engagement and Liaison Partnership (HELP) teams. PACT in Calgary and Edmonton facilitated over 4,760 crisis interactions and rural-based RPACT logged more than 1,000 interactions. HELP teams in Calgary and Edmonton provided more than 5,300 community referrals (details on pages [30-31](#)).
- **Improved treatment referral pathways in the justice system:** Enabled immediate access to opioid treatment for individuals detained by police via Virtual Opioid Dependency Program (VODP). Police services referred more than 2,000 detainees to VODP and there were more than 1,700 opioid agonist treatment starts for individuals in custody (details on page [28](#)).

The Ministry of Mental Health and Addiction remains committed to regulatory approaches and program delivery that reduces unnecessary government oversight and emphasizes outcomes, to improve access to government services, attract investment, support innovation and competitiveness and grow Alberta businesses.

Discussion and Analysis of Results

Actions that support the priorities of the Government of Alberta Strategic Plan

Key Priority Two:

Investing in Albertans

Objective One: Ensuring an accessible, responsive, and refocused health care system

- Refocusing the health care system to improve access and patient outcomes for Albertans and make health care better for all Alberta frontline health care workers. This refocusing will create a unified health system with provincial sector-based health authorities that centre around four dedicated organizations: primary care, acute care, continuing care, and mental health and addiction. Alberta Health Services will continue to deliver acute care services, and an Integration Council will ensure the system is in alignment to better deliver health outcomes.

Detailed reporting found on pages [18-21](#) (Key Objective 1.1).

Objective Two: Accelerating mental health and addiction support

- Establishing a new centre of recovery excellence to guide mental health and addiction service delivery in Alberta. The new centre will apply an evidence-based approach to help government improve public policy in alignment with the Alberta Recovery Model and support better outcomes for those experiencing addiction and mental health issues.

Detailed reporting found on pages [20-21](#) (Key Objective 1.1).

- Working with municipalities and community-based organizations to improve access to recovery-oriented systems of care. This approach is based on a comprehensive continuum of person-centred care including prevention, intervention, treatment, and recovery supports giving hope to Albertans experiencing or at risk of mental health and addiction challenges.

Detailed reporting found on pages [38-39](#) (Key Objective 2.1), [48](#) (Key Objective 3.2), and [60-61](#) (Key Objective 3.5).

- Implementing therapeutic living units and supports in correctional facilities in alignment with, and commitment to, a recovery-oriented system of care. The therapeutic living units offer structured, long-term and evidence-based interventions for those in custody, in order to address their substance use and break the cycle of recidivism.

Detailed reporting found on page [28](#) (Key Objective 1.4).

- Collaborating with government partners, First Nations, and Métis communities to expand treatment options available to Albertans experiencing addiction and improve their overall well-being. In 2024-25, more than \$167 million is allocated to build and operate recovery communities throughout Alberta, including in Indigenous communities, to provide long term residential addiction treatment.

Detailed reporting found on pages [36-38](#) (Key Objective 2.1) and [42](#) (Key Objective 2.3).

Key Priority Two:

Investing in Albertans

Objective Two: Accelerating mental health and addiction support (continued)

- Expanding efforts to ensure culturally appropriate addiction and mental health community supports are in place for First Nations, Inuit and Métis peoples in Alberta. In 2024-25, approximately \$7 million is allocated to strengthen community-based mental health and addiction programs and services for Indigenous people in Alberta.

Detailed reporting found on pages [42-44](#) (Key Objective 2.3).

- Enhancing treatment referral pathways in the criminal justice system and ensuring police services can access tools that support Albertans' wellness and recovery while keeping communities safe. In 2024-25, over \$23 million is allocated to provide tools to help police services throughout Alberta respond safely and effectively to mental health and addictions emergencies.

Detailed reporting found on pages [28-31](#) (Key Objective 1.4).

- Expanding access to mental health supports for children and youth in schools, including establishing mental health classroom teams in partnership with CASA Mental Health, giving children and youth the opportunity to seek out mental health supports sooner. In 2024-25, \$36 million is allocated to build and \$11 million is allocated to operationalize youth mental wellness centres (CASA Houses) throughout Alberta to provide live-in and day programs for youth facing severe mental health challenges.

Detailed reporting found on pages [41](#) (Key Objective 2.2) and [54](#) (Key Objective 3.3).

- Removing barriers and improving access to mental health supports by investing in Counselling Alberta to provide same day, no wait list, and affordable counselling sessions for all Albertans. Individuals who receive early intervention for mental health and addiction concerns tend to have better treatment outcomes.

Detailed reporting found on page [49](#) (Key Objective 3.2).

Outcome One: Alberta has a comprehensive, integrated, and responsive recovery-oriented addiction and mental health system

Mental Health and Addiction is implementing the Alberta Recovery Model—a comprehensive recovery-oriented system of care to support Albertans to pursue recovery. The department is responsible for oversight of the mental health and addiction system, including coordinating government funding dedicated to delivering mental health and addiction services across the province. The ministry is committed to creating positive outcomes for people experiencing mental health and addiction challenges through its work to reduce barriers and ensure every Albertan has access to high-quality recovery-oriented care to improve their well-being and help sustain recovery.

Key Objectives

1.1 Develop a new mental health and addiction system, as part of a refocused health system, to better support Albertans experiencing mental health and addiction challenges.

Refocusing the health care system

In November 2023, Alberta's government announced an initiative to refocus Alberta's health care system, striving to enhance health outcomes for Albertans and empower health care workers to deliver exceptional care across the province. Throughout 2024-25, the Government of Alberta unveiled four new provincial health agencies, including Recovery Alberta, the provincial health agency responsible for delivering mental health, addiction, and correctional health services. To help ensure continuity and stability during this significant transition, the Ministry of Mental Health and Addiction received an initial transfer of \$1.13 billion from the Ministry of Health. In addition to Recovery Alberta, the Canadian Centre of Recovery Excellence (CoRE)—a provincial statutory corporation dedicated to evaluation and research—was established.

The refocusing initiative represents whole-system transformational change, enabling Alberta's government to build a robust, accessible, and fully integrated mental health and addiction system capable of effectively supporting individuals in their pursuit of recovery. Through a unified effort, the Department of Mental Health and Addiction, Recovery Alberta, and CoRE are strategically

In April 2024, the Ministry of Mental Health and Addiction held four **engagement sessions** with Alberta Health Services staff and partners to share information about Recovery Alberta and respond to participant questions about the upcoming transition. This was an important first opportunity for staff to receive information, ask questions, and hear directly from the Minister, department and emerging provincial health agency. A total of 1,719 listeners participated, with 523 comments or questions submitted, either online or by telephone.

directing resources to enhance the quality, effectiveness, and efficiency of mental health and addiction services in alignment with the Alberta Recovery Model. This work helps ensure Albertans have seamless access to comprehensive support across the continuum of care—from prevention and early intervention to treatment and recovery.

Establishing Recovery Alberta

The creation of Recovery Alberta builds upon the Government of Alberta's extensive efforts since 2019 to develop the Alberta Recovery Model. Prioritizing mental health and addiction services through the refocusing initiative is helping to ensure these services receive the attention needed to effectively address the addiction crisis and provide essential support for those experiencing mental health challenges.

In May 2024, the *Health Statutes Amendment Act, 2024* received royal assent, facilitating the transition from a single regional health authority—Alberta Health Services—to an integrated

network of sector-based provincial health agencies: acute care, primary care, continuing care, and mental health and addiction services. The act included the following key amendments to the *Regional Health Authorities Act* to further health system refocusing:

- Change the name of the *Regional Health Authorities Act* to the *Provincial Health Agencies Act*.
- Clarify the roles and responsibilities of the new provincial health agencies.
- Facilitate the transfer of staff from AHS to the new provincial health agencies, while offering stability for the workforce, unions, and government throughout implementation of the health system refocusing initiative.

Recovery Alberta was legally established as the provincial health agency for the mental health and addiction health services sector—the first of four new provincial health agencies—effective July 1, 2024, reporting to the Minister of Mental Health and Addiction. Within the province’s mental health and addiction system, Recovery Alberta is the primary service provider.

On September 1, 2024, Recovery Alberta officially commenced operations under the Recovery Alberta banner, with responsibility for delivering mental health, addiction, and correctional health services to Albertans in a variety of service settings across the province, including:

- Acute care (emergency services; inpatient psychiatric care in hospitals)
- Outpatient (community health clinics and walk-in services)
- Virtual (Virtual Opioid Dependency Program; addiction and mental health helplines)
- Medical withdrawal management (detoxification services)
- Standalone psychiatric (targeted designated psychiatric services, primarily bed-based inpatient care)
- Community outreach (addiction and mental health counselling; mental health promotion)
- Community bed-based (addiction treatment; complex service needs)

Recovery Alberta is also accountable for all health services within 10 provincial correctional facilities including providing primary care services such as vaccines, emergency response, pharmacy, urgent dental care, and addiction and mental health care.

Upon launching, Recovery Alberta successfully transitioned more than 10,000 AHS staff and approximately 800 physicians into the new provincial health agency without disrupting patient or client care. Services remained consistently stable and accessible across the province, provided by the same dedicated frontline staff in familiar locations, with no increase in wait times. Throughout the transition, Recovery Alberta delivered, partnered, or managed contracts for approximately 1,350 community mental health beds; 1,650 psychiatric beds; and 1,350 addiction treatment, medical withdrawal management, and recovery beds. Mental health services and supports continued to be provided at more than 130 outpatient community clinics and virtual teams, alongside health services delivered in 10 provincial correctional facilities. In 2024-25, Recovery Alberta spent \$1.5 billion (\$900 million between September 2024 and March 2025) to deliver mental health and addiction services to Albertans, which is an increase of \$100 million compared to budget. The spending breakdown includes \$149.5 million for acute care, \$189.1 million for outpatient services, \$900,000 for virtual services, \$96.8 million for medical withdrawal management, \$22.7 million for standalone psychiatric services, \$42.8 million for community outreach, and \$472.3 million for community bed-based care. This increase in spending is related to new investment in beds and expanded services, and staffing pressures and costs relating to the increased acuity of patients requiring constant care.

Recovery Alberta's priorities for 2025-26 include:

- Providing programs and services in line with its mandate.
- Ensuring Albertans have access to safe, quality programs and services.
- Finalizing a service delivery model for compassionate intervention.
- Continuing to build and support an engaged, highly skilled workforce.
- Publishing its first health and business plan.
- Developing outcome measures and a long-range performance measurement framework.
- Participating in system-wide, long-range planning with the department and CoRE.
- Continuing to support Alberta's health system refocusing activities.

Establishing CoRE

In April 2024, Alberta's government announced its intention to create CoRE, providing an initial budget of \$5 million to support the organizational stand up and to initiate work aimed at finding the most effective ways to support recovery and transform how it is understood and achieved. CoRE's creation reinforces the Government of Alberta's ongoing commitment to ensure those experiencing mental health and addiction challenges receive high-quality, recovery-oriented care informed by robust data and evidence-based practices.

In June 2024, the *Canadian Centre of Recovery Excellence Act* was proclaimed, establishing CoRE as a statutory corporation reporting to the Minister of Mental Health and Addiction. CoRE began operating immediately with a mission to empower decision-makers with data and evidence to support people with mental health or addiction issues in their pursuit of recovery. This work is being advanced through four aims:

- System level evaluation and research to improve and build upon recovery-oriented systems of care for those with mental illness or addiction.
- Comprehensive data and analytics to identify new insights that can improve outcomes.
- Expert advice and guidance on effective recovery-focused mental health and addiction programs, including practical policy recommendations, clinical practice guidelines, and quality standards.
- Leadership and collaboration on recovery-oriented systems of care to enable the best possible outcomes for Albertans.

Ongoing rigorous evaluation and analysis of the Alberta Recovery Model is expected to inform best practices, support the development of data-driven policies in mental health and addiction, and continue to evolve the mental health and addiction system to one that is person-centred and focused on producing strong outcomes for Albertans on the path to recovery.

Since launching and through March 31, 2025, CoRE:

- Operationalized the new organization including human resources, finance, governance, corporate, and legal responsibilities.
- Recruited 20 full-time equivalents (FTEs) to deliver on CoRE's mandate to support decision-making with research, data, and evidence.
- Engaged experts to inform the development of quality standards for bed-based addiction treatment services.

- Launched the first of 12 guidance documents promoting practical initiatives to foster and sustain recovery friendly workplaces with the remaining to be completed in 2025-26.
- Published a policy white paper with evidence-based recommendations to inform Alberta's compassionate intervention strategies for severe addiction.

CoRE measures its success across the following domains: advancing knowledge, building capacity, informing decision-making, impacting health and health systems, and impacting broad economic and societal outcomes. In 2025-26, CoRE will work to build internal capacity to continue progress on priorities identified in its business plan (<https://recoveryexcellence.org/wp-content/uploads/2025/02/2025-28-Business-Plan-Final.pdf>), validate system-level evaluation and research needs and direction, and advance data access to enable evaluation of recovery-oriented systems of care.

1.2 Expand mental health and addiction system oversight to reduce barriers and ensure every Albertan has access to high-quality recovery-oriented care to improve their overall well-being and help sustain recovery.

Overseeing the mental health and addiction system

The transformation of Alberta's mental health and addiction system is based on the belief that recovery is possible, and that anyone struggling with mental health or addiction challenges deserves the opportunity to pursue recovery and live a purpose-filled life. This drives the vision for Alberta's mental health and addiction system: that every Albertan dealing with mental health and addiction challenges is supported in their pursuit of recovery through a full continuum of recovery-oriented services and supports. The ministry's efforts to build strong system oversight, foster collaboration and integration, and develop a clear and actionable path forward for the system are helping make progress toward achieving this vision.

Over the past year, the ministry took significant steps to advance Alberta's overarching mental health and addiction system with a goal to strengthen consistency of care and make life better for those experiencing mental health and addiction challenges. One of the key accomplishments was the delineation of clear roles and responsibilities within the mental health and addiction system. Each entity within the Ministry of Mental Health and Addiction has a unique role in helping to ensure the refocused mental health and addiction system is effective, efficient, and working to advance the Alberta Recovery Model:

- The Department of Mental Health and Addiction oversees system planning, policy, legislation, funding, and capital planning.
- Recovery Alberta coordinates, monitors, and delivers mental health, addiction, and correctional health services.
- CoRE provides critical support through evaluation, research, knowledge sharing, and system performance monitoring.

In 2024-25, Recovery Alberta and CoRE received mandate letters outlining strategic priorities and accountabilities. The ministry began developing an outcomes framework to define the mental health and addiction system's purpose, vision, goals, and desired outcomes, aiming to create a recovery-oriented system that meets Albertans' needs with compassion and innovation.

Collaborative system planning work was initiated to guide service, program, and capacity planning for implementing the Alberta Recovery Model. This will align strategies to address challenges like increased service demand and workforce pressures, transforming mental health and addiction care. Success relies on collaboration across ministries, agencies, service providers,

health professionals, private sector partners, and Indigenous leaders, ensuring an integrated, inclusive, and accessible recovery-oriented system.

The Mental Health Review Panel Roster is a provincial adjudicative agency responsible for protecting the rights of patients detained in facilities and individuals subject to community treatment orders under the *Mental Health Act*. The agency reports directly to the Minister of Mental Health and Addiction. Each panel includes a lawyer, a psychiatrist, and a member of the public.

2024-25 Mental Health Review Panel Roster geographical breakdown

Location	Chairs/Vice Chairs	Psychiatrists	Public Members	Total
Edmonton/North	2 (33%)	10 (43%)	14 (47%)	26 (44%)
Central	2 (33%)	1 (4%)	4 (13%)	7 (12%)
Calgary/South	2 (33%)	12 (52%)	12 (40%)	26 (44%)
Total	6	23	30	59

Application hearings can occur in person, or by phone or videoconference and can be made to:

- Cancel admission or renewal certificates for patients who are detained in a designated facility.
- Order a facility to issue a community treatment order.
- Return a patient to a correctional facility.
- Cancel community treatment orders.
- Cancel a certificate of incompetence to make treatment decisions.
- Order the administration of treatment to patients who object to it.

Most applications are heard within 21 days of receipt and decisions are made within 24 hours of hearing the application. In 2024-25, the percentage of applications heard within 21 days was 82.8 per cent.

In 2024-25, the number of applications and completed hearings remained relatively stable, with 3,654 applications for review panel hearings and 2,257 hearings held compared to 3,516 applications and 2,264 hearings in 2023-24. In the upcoming year, the ministry will conduct a required review of the Mental Health Review Panel Roster to assess whether its structure and operations can be modernized for continuous improvement, with recommendations aimed at strengthening Alberta’s recovery-oriented system of care.

Supporting workforce development and capacity building initiatives

Alberta’s government is committed to strengthening the recovery-oriented system of care by building workforce capacity for mental health and addiction services in alignment with the Alberta Recovery Model. The Recovery Training Institute of Alberta (RTIA), established in July 2024 and operated by ROSC Solutions Group Inc., provides practical skills and recovery-focused training for individuals employed in publicly funded recovery communities, addiction treatment centres, correctional services, and related recovery-oriented roles. Since opening and through March 31, 2025, 52 people have received on-site training at the RTIA, including 35 recovery community employees, 13 Indigenous community leaders and four other recovery professionals.

The RTIA also offers online education, training, and professional development, including targeted skill development for correctional services staff working in Therapeutic Living Units (TLUs) and Transition Services. In 2024-25, 20 individuals from the Calgary Correctional Centre TLU completed training. Additional targeted support was provided to program teams from the five Indigenous communities establishing recovery communities on their lands, including orientation to Alberta's recovery community model and guidance on operational planning aligned with their unique strengths and healing approaches. This support will continue until the facilities open.

In 2024-25, the Department of Mental Health and Addiction provided nearly \$1.2 million to support operationalization of the RTIA and continued programming. This funding has resulted in more individuals gaining the knowledge and skills necessary to make a meaningful impact in recovery-oriented care and the lives of Albertans. Looking ahead to 2025-26, the RTIA will expand its offerings by collaborating with community partners and industry experts to host specialized learning sessions in response to the evolving needs of Alberta's recovery communities and broader mental health and addiction system.

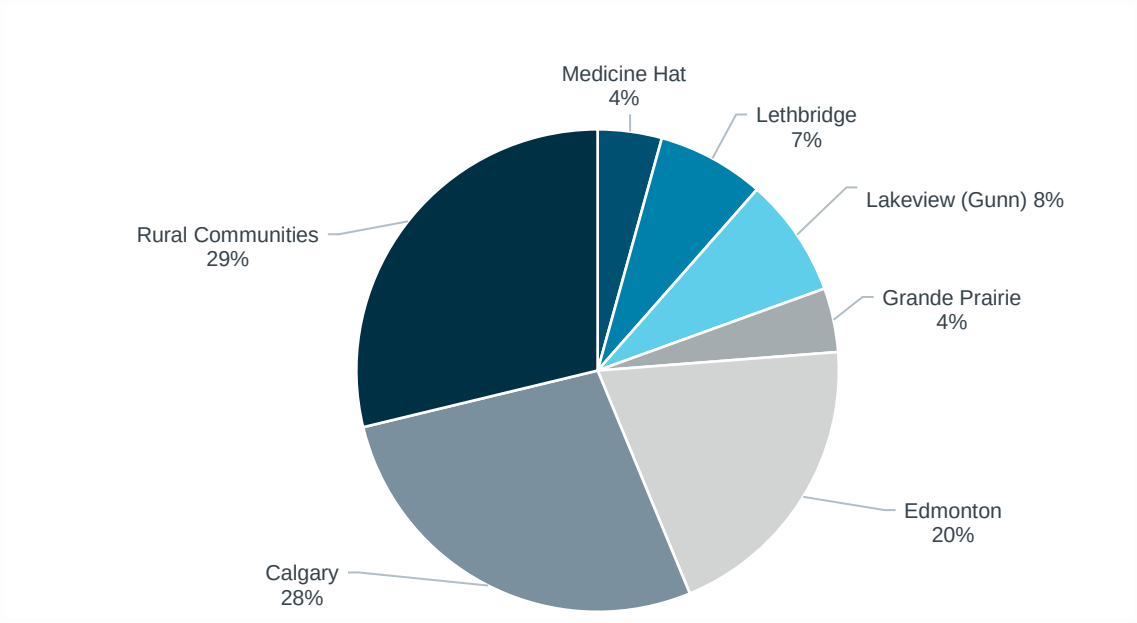
Recovery coaches are trained peer workers, often with lived experience, who provide a range of non-medical, non-clinical support to individuals in their pursuit of recovery. Operating both virtually and in person, they provide a critical service, helping more people lead fulfilling lives in long-term recovery in alignment with the Alberta Recovery Model. To support Albertans in achieving their recovery goals, recovery coaches:

- Facilitate system navigation and streamlined referrals to help individuals access appropriate recovery programs.
- Help individuals develop recovery wellness plans tailored to their unique needs and hold them accountable to their plan.
- Offer ongoing support, guidance, motivation, and encouragement through regular one-on-one sessions.
- Help individuals build coping strategies and resilience.
- Provide transitional assistance for individuals who are re-entering the community after leaving structured programs.
- Connect participants with community resources and support networks.
- Reduce the burden on emergency health care services through intervention and referral to appropriate treatment.

Recognizing the increasing demand for recovery support to prevent relapse or escalation of addiction, nearly \$2.4 million was invested in 2024-25 to enhance availability and accessibility of recovery coach services. Recovery coach teams were integrated into the Edmonton and Calgary Navigation and Support Centres and introduced at the Safe Harbour shelter in Red Deer, providing critical support to marginalized individuals. Recovery coaches made more than 2,000 client connections and referrals during this period.

In 2024-25, the Department of Mental Health and Addiction provided \$1 million to ROSC Solutions Group Inc. to establish the Recovery Coach Academy of Canada within the RTIA to help expand Alberta's recovery-oriented workforce through cultural awareness, lived experience, and ethical, participant-centred care. Launched in April 2024, the Recovery Coach Academy of Canada offers specialized training and skill development programming for recovery coaches at no cost for approved individuals. As of March 31, 2025, more than 520 people have completed training and courses offered by the Recovery Coach Academy of Canada.

Recovery Coach Academy of Canada training enrolment by location



In September 2024, the Virtual Recovery Coaching Centre was established, staffed by professionals certified through the Recovery Coach Academy of Canada, to support individuals in recovery from substance use and addiction and awaiting treatment placement at a recovery community. As of March 31, 2025, the Centre provided virtual coaching to 229 waitlisted participants, with 136 actively engaging in sessions, 44 successfully completing addiction treatment, and an average coaching duration of 46.8 days. Participant engagement continued to grow, reaching over 340 active participants in February 2025, including new participants—a 10 per cent increase from January 2025. Over the coming year, approximately 250 additional recovery coaches are expected to be trained to meet the target of 500 coaches by March 31, 2026, including recovery coaches needed to support compassionate intervention.

The ministry is continuing to collaborate with partner ministries to support targeted capacity building programs and projects that will enhance Alberta’s recovery-oriented system of care and provide mental health and addiction support to more people in need. In 2024-25, the Ministry of Mental Health and Addiction worked with the Ministry of Health to implement the Health Workforce Strategy. The strategy focuses on recruitment, retention and support, as well as education and training for health care workers, including in the mental health and addiction sector. The ministry also collaborated with the Ministry of Advanced Education to add more mental health professional spaces in Alberta post-secondary institutions. In August 2024, Advanced Education announced \$10 million in post-secondary seat expansion funding for mental health professional education programs. This will create 761 new seats in various high-demand mental health programs over three years, such as psychiatric nursing, psychology, social work, child and youth care, and addiction counselling. These opportunities will help ensure consistent high-quality services are available in communities across the province while preparing a new generation of mental health professionals to meet emerging needs.

Improving data and evidence for decision making

The Ministry of Mental Health and Addiction is committed to improving the sector’s evidence base to inform decision making and effective policy development. CoRE is responsible for conducting system level evaluation and research to improve and build upon recovery-focused systems of care

and evaluating policies, programs, and services within the mental health and addiction system. Recovery Alberta's evaluation activities relate to service delivery, ensuring the mental health, addiction, and correctional health services delivered are aligned with the Alberta Recovery Model and provide the desired impact and value to Albertans. The department's evaluation activities relate to oversight functions, ensuring policy and legislation supports Alberta's recovery-oriented system of care and that funded programs provide a good return for the investment.

Looking ahead, the ministry will focus on facilitating coordinated evaluation and research activities, continuing to expand its evidence base, fostering a culture of continuous improvement, and ensuring that all programs and policies are aligned with the Alberta Recovery Model.

Ministry evaluation, research, and measurement activities (completed, underway, and planned)

CoRE	• Developing a comprehensive system-level performance measurement framework. This is a joint endeavour by CoRE, the department, and Recovery Alberta with input from stakeholders and experts across the province intended to measure progress toward achieving goals identified through the system planning work (early planning was initiated in 2024-25 with work continuing into 2025-26). • Continuing to develop expert guidance to inform quality and licensing standards for bed-based addiction treatment services (2025-26 and beyond). • Advancing survey science to determine rates of mental illness and addiction as well as pathways to recovery (2025-26 and beyond). • Evaluating outcomes related to the Alberta Recovery Model programs and services (ongoing).
Recovery Alberta	• Evaluating and measuring grant-based initiatives such as overdose prevention services and the VODP (ongoing). • Provisioning data for broader ministry evaluation activities (ongoing). • Comprehensively evaluating Narcotic Transition Services (2025) and the Medical Detoxification and Residential Addiction Treatment Expansion (MDRATE) project focusing on the scope of service delivery (2025-26). • Evaluating addiction and mental health clinical services for post-secondary institutions focusing on standardization of service delivery (2025-26 and beyond). • Routinely evaluating efforts to improve specific operational areas of delivery, including those that span all services, e.g., client experience (ongoing).
Department	• Providing foundational support to CoRE's leadership by planning the evaluation work, assessing evaluability, and helping establish the conditions necessary for a comprehensive and collaborative evaluation of the compassionate intervention initiatives. • Initiated baseline work to support CoRE's evaluation of recovery communities, including aligning outcome reporting requirements to enable consistent and meaningful measurement (initiated in 2024-25, continuing into 2025-26).

	• Assessing the impact of urban-focused initiatives on the health care and correctional systems and perceptions of safety in downtown areas of Calgary and Edmonton (initiated in 2024-25, continuing into 2025-26). • Building insights about Alberta’s addiction and mental health initiatives through creative use of cross-government and partner data holdings.
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Comprehensive data linking is subject to legislative and privacy-based constraints, which must be considered and navigated. In 2024-25, the department, CoRE, and Recovery Alberta became full custodians of relevant health administrative data under the *Health Information Act* to guarantee consistent data flows to meet the data needs of the ministry. The ministry also made progress on several important initiatives aimed at improving data quality to better inform decision making and policy development to advance the Alberta Recovery Model. All identified initiatives will continue into 2025-26.

- Exploring opportunities to align grant agreement reporting requirements with recovery-oriented outcomes to demonstrate programs and services’ effectiveness in supporting the Alberta Recovery Model.
- Conducting data linkage for health service utilization among navigation centre clients in collaboration with Recovery Alberta and Seniors, Community and Social Services to understand their mental health and addiction-related needs, and identify potential service gaps to facilitate coordinated decision-making, policy development, and program implementation within and across ministries.
- Transitioning the Alberta Substance Use Surveillance System (ASUSS) from Alberta Health to the department to enable full access to timely substance use data for faster and accurate responses and ensuring availability of data for surveillance purposes.

ASUSS—accessible through <https://www.alberta.ca/substance-use-surveillance-data>—is a publicly available online reporting dashboard containing point in time, comprehensive system-level population data related to substance use in Alberta, including opioid and other substance-related deaths, Emergency Medical Services (EMS) responses to opioid-related events, naloxone kit distribution, hospitalizations and emergency department visits, and high-level details about location of overdose deaths. ASUSS data helps the ministry monitor and analyze trends related to the Alberta Recovery Model to understand whether desired outcomes are being achieved. It also provides valuable insight that informs the ministry’s decision making and strategic direction regarding supports for individuals experiencing addiction. The trends observed in Alberta are regularly compared with trends observed in other jurisdictions to provide an understanding of addiction challenges on a broader scale. While it is still early in the health care system transformation, data from ASUSS is demonstrating positive trends for several key system indicators, for example, in the 2024 calendar year:

- Recovery Alberta’s opioid dependency program supported a record number of nearly 12,300 unique clients, including VODP, compared to almost 11,500 unique clients in 2023, an increase of seven per cent.
- Opioid deaths per 100,000 population decreased by 37 per cent compared to 2023.
- Deaths from methamphetamine, alcohol and cocaine were down significantly compared to 2023.
- Opioid-related emergency department visits declined by 26 per cent and the number of opioid-related hospitalizations reduced by 16 per cent compared to 2023.

In 2024-25, the ministry began exploring ways to improve and expand ASUSS to increase public accessibility and comprehension, e.g., simplifying terminology and incorporating VODP visits to reinforce the commitment to transparency and accountability and ensure all aspects of a client's care are accounted for and every interaction contributes to their overall well-being. As individuals accessing opioid dependency program services have access to a range of supports beyond treatment interventions, the expanded data gives a more comprehensive look at the care journey of those accessing the program.

1.3 Develop strategic approaches, including a Compassionate Intervention Framework, to expand access to timely and appropriate mental health and addiction services across the province to support individuals who are a danger to themselves or others.

Advancing compassionate intervention

Alberta's government remains committed to addressing addiction, protecting lives, and enhancing community safety. Following policy approval in May 2024, the Ministry of Mental Health and Addiction began developing compassionate intervention legislation, actively engaging government partners and community stakeholders to ensure robust safeguards are in place to respect civil liberties and personal rights, while providing comprehensive, recovery-oriented treatment. This legislative framework is designed to support individuals facing severe substance use and addiction issues who have not benefited from conventional, voluntary treatment options.

The *Compassionate Intervention Act*, introduced in April 2025, aims to provide essential support to individuals at the highest risk—those whose substance use causes harm to themselves or others and who need serious interventions to ensure their safety and well-being. A Compassionate Intervention Commission will be established as an independent adjudicative body responsible for making decisions about addiction assessments and care plans. The Commission will consist of qualified professionals, including lawyers, physicians, and public representatives, which may include people who identify as Indigenous, have lived experience, or who have supported those with lived experience to help ensure diverse perspectives and expertise guide decisions about patient care and intervention.

Compassionate intervention will allow adult family members, guardians, health care professionals, peace or police officers, or social workers to request a treatment order for those whose addiction or substance use has made them a danger to themselves or others. Eligible individuals will receive assessment, stabilization, treatment, and aftercare support. By enabling timely, targeted, life-saving interventions, the Government of Alberta is helping ensure that vulnerable individuals receive critical care and treatment when it is most needed—when they have lost the capacity to make these decisions on their own.

In 2024-25, the ministry initiated detailed planning to ensure the necessary regulatory and service delivery frameworks are ready to implement compassionate intervention effectively within Alberta's broader recovery-oriented system of care. As part of this comprehensive effort, two dedicated compassionate intervention centres for adults and one for youth are being planned to provide secure care for those experiencing severe addiction challenges who present a substantial harm to themselves or others. Planning is underway with Recovery Alberta to deliver a compassionate intervention service delivery model (see Key Objective 2.1 for further details on this work). Once implemented, this framework is expected to significantly improve individual health outcomes, enhance public safety, and reduce pressure on emergency and justice system resources.

1.4 Continue to advance treatment referral pathways in the criminal justice system and support initiatives to expand police services' access to tools that support Albertans' wellness and recovery while keeping communities safe.

Providing treatment and recovery supports for people in the justice system

Therapeutic Living Units (TLUs) provide Albertans in custody the opportunity to initiate voluntary, holistic evidence-based addiction treatment in a therapeutic community environment adapted for a correctional setting. Operated by Beccarian Correctional Care, these specialized units in provincial correctional facilities are available to eligible inmates who demonstrate a high need for treatment and are anticipated to remain in custody for 90 days or more. The goal of the program is to protect communities and prevent individuals from reverting to substance use or criminal activity. This is achieved by helping program participants build recovery capital.

In 2024-25, the Department of Mental Health and Addiction and Ministry of Public Safety and Emergency Services invested a combined total of \$11 million to sustain TLU operations and transition services. All four planned TLUs—Red Deer Remand Centre, Lethbridge Correctional Centre, Fort Saskatchewan Correctional Centre, and Calgary Correctional Centre—were fully operational by November 2024. Since opening and through March 31, 2025, 299 Albertans have been admitted to the four TLUs in Alberta: 60 per cent of those admitted completed 75 days of treatment and nearly half stayed over 90 days, with many ending programming due to their planned release from corrections.

Individuals who take part in the TLU program prepare for life outside the TLU near the end of their release. The final phase is focused on preparing individuals for admission to a recovery community and to engage in a formal vocational linkage program for approximately six months. Transitioning to a recovery community following release is voluntary. In 2024-25, 29 per cent of Albertans who completed the TLU program transitioned to recovery communities and the remaining 71 per cent transitioned to other community programs or returned to the community with their families.

Select remand facilities across Alberta provide specialized transition services that connect individuals in custody with recovery coaches who encourage positive behavioural changes and assist with referrals and warm handoffs to community-based recovery supports. Since opening in June 2024, and through March 31, 2025, transition services at Calgary, Edmonton, Peace River, and Medicine Hat had assessed almost 600 clients, with nearly 130 actively engaged in ongoing support. More than 30 clients successfully transitioned to TLUs and recovery communities to initiate treatment programming.

Alberta was the first province to offer immediate access to addiction treatment for individuals detained by police. In partnership with the Virtual Opioid Dependency Program (VODP), health care practitioners, such as on-site paramedics within police and RCMP detainment units, facilitate prompt connection to opioid agonist treatment for individuals in withdrawal from opioid use. Existing partnerships include police services in Edmonton, Calgary, Lethbridge, and 111 RCMP detachments throughout the province. In addition, on-site clinicians and mental health navigators in Edmonton and Calgary provide secure intake, assessment, and service referrals for those presenting with addiction and/or mental health issues.

The Department of Mental Health and Addiction has invested more than \$2.1 million over two years (2023-24 and 2024-25) to support on-site paramedic services within police and RCMP detainment units in Lethbridge, Red Deer, and Grande Prairie. In 2024-25, there were more than 2,000 referrals to VODP from police services and RCMP detachments and more than 1,700 opioid agonist treatment starts for people detained in custody (further information on the VODP is provided in Key Objective 3.5).

Recovery Alberta's Opioid Agonist Treatment in Corrections Facilities program aims to enhance accessibility and continuity of opioid agonist treatment and psychosocial supports in all 10 adult and youth provincial correctional and remand centres by:

- Increasing initiation and maintenance on opioid agonist treatment for individuals with opioid use disorder.
- Decreasing maintenance assessment wait times.
- Enhancing access to addiction counselling for individuals with opioid use disorder.
- Partnering with Corrections and Law Enforcement agencies to support transitions to community treatment upon release.

This program is part of an ongoing effort to improve access to services for people in correctional settings in alignment with the Alberta Recovery Model. In 2024-25, Recovery Alberta spent nearly \$4.2 million to support this program. There were more than 3,500 opioid agonist treatment starts for those in corrections in 2024-25 and 1,000 people who were receiving treatment in the community maintained opioid agonist treatment upon admission to correctional and remand centres. All individuals meeting criteria for opioid use disorder who accepted treatment started in less than 24 hours after admission. Recovery Alberta referred more than 4,000 people who received or requested opioid agonist treatment in a correctional facility to an addiction counsellor for either group or individual treatment and provided handoffs to community treatment providers for more than 4,500 people released to the community.

The VODP partners with correctional health services to enable transitions in treatment for clients released from incarceration to avoid disruptions to their established treatment plan. Individuals released from correctional facilities are at significant risk for overdose, particularly within the first week after release. The partnership between the VODP and correctional health services can help prevent individuals from reoffending and offers wraparound health and social supports to address their unique needs. In 2024-25, the VODP completed more than 2,200 consults for treatment transition referrals for clients in correctional health services.

Partnering with police to connect vulnerable Albertans with supports

HealthIM is a digital tool that equips police officers with essential resources to effectively manage mental health crises. In providing real-time assessments and de-escalation information, HealthIM enhances officer response capabilities for mental health calls and increases public safety. In 2024-25, the Department of Mental Health and Addiction provided nearly \$740,000 to support the continued operation of HealthIM in collaboration with the Ministry of Public Safety and Emergency Services. As of March 31, 2025, HealthIM was used in eight police services and in all four RCMP districts across the province. The tool has been successfully rolled out to all interested police services; further expansion anticipated in 2024-25 was not necessary. HealthIM was used in more than 9,760 mental health related calls to police and the service logged almost 9,100 interactions with Albertans in 2024-25, contributing to safer and more supportive interactions for people experiencing mental health emergencies. Enhancements to the tool are ongoing to align with the *Mental Health Act* and to strengthen the response to mental health crisis across the province.

Alberta's government is committed to increasing public safety and supporting vulnerable populations in Calgary and Edmonton by addressing underlying issues such as addiction, mental health challenges, and homelessness. In 2024-25, the Department of Mental Health and Addiction provided more than \$5.5 million combined to fund targeted strategies and initiatives aimed at tackling these issues. To improve clinical supports as part of these strategies, Recovery Alberta spent \$4 million to add more mental health clinicians and therapists to work alongside the police services in the two cities.

Police and Crisis Teams (PACT) operate in both Edmonton and Calgary, pairing police officers with mental health professionals to respond to 911 calls involving mental health crisis. Operated in partnership between Recovery Alberta and each city's respective police service, PACT teams assess, stabilize, and connect individuals in crisis with appropriate mental health services in the community, helping divert them away from emergency departments or unnecessary involvement with the justice system.

In Calgary, a team of 10 clinicians and one clinical supervisor support 10 PACT and PACT-related teams throughout the city. Between July 1, 2024, and December 31, 2024, the Calgary PACT had 3,940 client interactions and engaged with 555 unique clients and 493 repeat clients. Teams received 277 incoming referrals and conducted 319 outgoing referrals. In Edmonton, all new PACT clinicians are embedded within Recovery Alberta's Access 24/7. In 2024-25, 18 full time positions were onboarded to enhance the Access 24/7 team to effectively support districts. The first

In Calgary, 211 Alberta distress workers are integrated into Calgary 911 to **enhance crisis response and recovery outreach**. Calls that are believed to be mental health and addiction related, including suicide crisis calls, are transferred to these crisis workers who are trained in suicide prevention and crisis de-escalation and help identify the right team to respond to calls. As of March 31, 2025, a total of 8,235 calls have been responded to by the 211 crisis workers.

divisional PACT was launched in June 2024, and as of March 31, 2025, five out of six divisions have PACT, with support provided to the sixth division by the existing team. Between July and December 2024, Edmonton PACT divisions had 820 client interactions and engaged with 679 unique clients and 134 repeat clients.

Regional Police and Crisis Teams (RPACT) is a partnership between Recovery Alberta and the Alberta RCMP that pairs mental health clinicians with law enforcement to attend mental health crisis calls in rural, remote, and Indigenous communities throughout Alberta that are serviced by the RCMP. The goal of the initiative is to

increase access to mental health and addiction supports in underserved areas. Services provided through RPACT include mental health assessments, community service referrals, counselling, case planning and follow-up care through a combination of in-person and phone consultations to serve Albertans in need more effectively and efficiently.

Over the past two years (2023-24 and 2024-25), Recovery Alberta has spent \$1.3 million to support the continued operation and expansion of RPACT to help increase access to mental health supports and services in rural and remote Alberta communities. As of March 31, 2025, Recovery Alberta supported 12 operational RPACT based in Airdrie, Athabasca, Camrose (in partnership with Camrose Police Service), Coaldale, Drumheller, Gleichen, rural Grande Prairie, High Level, Hinton, Strathcona and Parkland counties, and Wainwright that serve more than 85 communities throughout the province.

There was a total of 1,886 referrals to RPACT in 2024-25 corresponding to 1,591 unique clients and 225 repeat clients. Of these referrals, 1,008 client interactions occurred, including crisis assessments, medication administration, patient collateral visits, follow-up visits, telephone encounters, and law enforcement support with general mental health apprehensions.

The Human-centred Engagement and Liaison Partnership (HELP) teams provide outreach services to individuals experiencing unstable housing or homelessness who may also face mental health and/or addiction challenges by connecting them to essential community resources and supports. In Edmonton, the HELP unit is part of the Edmonton Police Service and is staffed with social service navigators to support at-risk individuals by providing referrals, follow-ups, and case management plans. Social service navigators are also embedded within the Edmonton Police Service Diversion and Desistance Branch, which conducts proactive outreach to individuals with a history of causing

the greatest harm in the community. The goal is to help these individuals stabilize and reduce their use of the health care services and justice systems. Navigators are contracted from social agencies such as the Mustard Seed, Hope Mission, George Spady Society, Radius Health and Healing, and Bent Arrow. In September 2024, HELP navigators were added from the Wihcihaw Maskokamik Society from Enoch Cree Nation, adding to the continuum of care for Indigenous Peoples.

In 2024-25, the Edmonton HELP team received a total of 7,343 requests for services and the team provided 5,048 referrals or warm handoffs to support programs with clients indicating housing, addiction and mental health services, and identification cards as their top needs. In addition, a total of 115 clients were served by the Diversion and Desistance Branch.

In Calgary, the HELP team is operated by Alpha House. The organization responds to substance use and public intoxication issues when emergency services are not needed, which helps lessen the demand on police, bylaw, hospital, and emergency medical services. The HELP team also facilitates access and entry to shelters, health care, addiction treatment, and supportive housing. In 2024-25, the Department of Mental Health and Addiction provided \$500,000 to Alpha House to support continued operation of the Beltline HELP team.

In 2024-25, the Calgary Beltline HELP team served 11,209 total clients and 2,200 unique clients, responded to 4,845 service requests, provided 296 referrals, and completed 4,283 transports to shelters, drug self-administration locations, or social service agencies.

1.5 Enhance quality standards and legislative requirements guiding regulatory oversight and enforcement to improve service provider accountability and protect communities and Albertans accessing services.

Improving legislation, regulations, and standards

Alberta's government is working to provide the best mental health and addiction care by continuously reviewing and updating policy and legislation to meet the evolving needs of Albertans. Originally passed in 2018, the *Mental Health Services Protection Act* (MHSPA) provides a legislative foundation for safe, quality mental health and addiction care in Alberta. The act regulates bed-based addiction treatment services, narcotic transition services, supervised consumption services (also known as drug consumption services), and psychedelic drug treatment services. The Government of Alberta initiated amendments to the MHSPA in July 2024, with the aim to update the standards for bed-based addiction treatment services to improve the quality and safety of care for medical withdrawal management and treatment programs.

Amendments introduced in February 2025 position the act as framework legislation, significantly strengthening the mental health and addiction system by enhancing regulatory oversight and quality standards. These amendments categorize bed-based addiction treatment into three distinct service types, each governed by separate licensing standards:

- **Withdrawal management services:** medically supervised services to manage or support an individual through the process of withdrawal from one or more substances.
- **Intensive treatment services:** intensive and structured residential care services for individuals with addiction.
- **Non-intensive recovery services:** services in a recovery-oriented environment that provide less-intensive treatment compared to intensive treatment services.

The legislation also includes administrative amendments to address regulatory inconsistencies, clarify requirements, and better align the act with the Alberta Recovery Model. These changes will

take effect in fall 2025, ensuring all Albertans accessing these services receive consistent and effective care to support their recovery.

The Ministry of Mental Health and Addiction is actively engaged in cross-departmental collaboration and analysis regarding Medical Assistance in Dying (MAID) led by the Ministry of Justice. MAID is a legal process that allows an eligible person to receive assistance from a medical practitioner in ending their life. Alberta's government is reviewing how MAID is regulated to ensure there is a consistent process as well as oversight that protects vulnerable Albertans, specifically those living with disabilities or who have mental health challenges. In late 2024, Alberta's government conducted public engagement that will help inform planning and policy decision making to establish supplementary safeguards, ensuring MAID remains an exceptional process and protecting vulnerable Albertans from potential misuse. The Government of Canada has temporarily paused expansion of MAID eligibility to include people facing depression or mental illness until March 2027.

Since March 2024, Alberta's government has been working with the College of Alberta Psychologists and the Association of Counselling Therapy of Alberta to examine legislative amendments to the *Health Professions Act*. This could put in place standards of practice, a code of ethics, and minimum entry to practice requirements for Albertans who work as counsellors. Many components of the policy development for regulating counsellors are being led by the profession and is also dependent on the regulatory college being ready to register and regulate the profession. Alberta's government is committed to regulate this profession to ensure appropriate standards, accountability and oversight are in place to support safe, high-quality patient care.

Ensuring Albertans receive safe, quality care

The Compliance and Monitoring unit provides crucial system oversight as the dedicated regulatory enforcement body within the Department of Mental Health and Addiction. Current accountabilities are related to adherence to legislative requirements and standards under the MHSPA, regulation, and standards for four unique service types: residential addiction treatment services, drug consumption services, narcotic transition services, and psychedelic drug treatment services. Service providers who offer or provide regulated services meet core licensing requirements, giving assurance that Albertans accessing these facilities receive safe, standardized, and high-quality care. The unit conducts regular inspections, investigations, and follow-ups on reported incidents and complaints to determine compliance and enforce as necessary. This proactive enforcement approach reduces risk, ensuring providers either maintain compliance or cease operations to prevent unregulated or unsafe practices.

The unit contributes to system-wide continuous improvement by identifying gaps in treatment quality, safety, and compliance, providing valuable insights to legislative, policy, and program development processes. Compliance and Monitoring works closely with policy and legislation teams and has helped drive many aspects of the upcoming MHSPA amendment implementation. The number of licences issued is expected to increase once the proposed amendments take effect. The unit is also preparing for a new regulatory role in 2025-26 for compassionate intervention. Work will continue over the next few years to plan for and replace the current database and interim document portal with a more streamlined information technology solution, and to develop interactive webpages to improve access to information for providers, clients, and the public.

As of March 31, 2025, the unit licensed and determined compliance for 132 locations operated by 68 providers across Alberta, representing an increase of 21 locations and nine service providers over the year prior. This increase is attributed to licensing and inspections of psychedelic drug treatment services that commenced in 2024-25 and an increase in applications for residential addiction treatment service.

2024-25 Compliance and Monitoring Overview

Enforcement/Licensing Activity ¹	Result
Initial licensing inspections	17
Renewal licensing inspections	119
Exception (follow up) visits	20
Consultations (information sharing, educational awareness)	8
Investigations completed	14
Inspector orders issued	3
Non-compliances issued ²	481
Non-compliances rectified	370
Licences issued	88

¹ Figures presented in the table exclude psychedelic drug treatment services. These services will be incorporated into the tracking system over the coming year and will be included in reporting beginning in 2025-26.

² A non-compliance occurs when the service provider has been unable to provide evidence to confirm compliance to a requirement and may be issued through inspection, investigation, or follow up to a critical incident. Non-compliance can vary in severity and therefore the enforcement measure also varies from a notice of deficiency that outlines all non-compliances and the expected timeline for submitting required evidence, to an inspector order in circumstances where the non-compliance presents an immediate threat to client health or safety, or a non-compliance has not been resolved in required timelines. If a provider fails to comply with an inspector order, escalated enforcement measures are taken (e.g., a court order, amending or canceling a licence).

Performance Measure 1.a:

Mental health and addiction-related emergency department visits with no interaction with publicly funded mental health and addiction services in the previous two years (per cent)

Prior Years' Results				2023-24 Target	2023-24 Actual
2019-20	2020-21	2021-22	2022-23		
20.7%	20.6%	19.6%	19.5%	17.9%	19.7%

Source: Ministry of Health

Notes: Results lag by up to 10 months. Only the first visit is counted in a given fiscal year for individuals with multiple emergency department visits. Individuals with invalid or missing PHNs and less than two years of eligibility under the Alberta Health Care Insurance Plan are excluded.

Description: This performance measure describes the proportion of mental health or addiction-related emergency department visits in a fiscal year with no interaction with publicly funded mental health and addiction services (such as visits to physician office, emergency

department visits, hospitalizations, and prescription drug dispensations for mental health or addiction issues) in the previous two fiscal years.

Rationale: A decreasing score over time suggests improvement, indicating that individuals' needs are being addressed in the community, reducing reliance on the emergency department for delivery of mental health and addiction related services.

Results and variance explanation: The result for this measure was 19.7 per cent in 2023-24, which is 1.8 percentage points above the target of 17.9 per cent and only 0.2 percentage points above the 2022-23 result of 19.5 per cent. Results from the previous fiscal years demonstrate a gradual decreasing pattern.

The difference between the 2023-24 target and the actual result could reflect variation over time. However, insufficient workforce support and capacity at the community level, along with barriers affecting treatment and referral pathways from police services to mental health and addiction care, could contribute to an increasing number of Albertans seeking emergency department care for their mental health and addiction-related concerns. Alberta also experienced a record population growth from April 2023 to April 2024, the highest among Canadian provinces, primarily driven by immigration, which may have contributed to increased demand for health care services, including a rise in emergency department visits for mental health and addiction needs.

The results for this measure from 2019-20 to 2023-24 have underscored the actual need for mental health and addiction service use. Overall health care use decreased during the COVID-19 pandemic, with a considerable decrease in physician visits and hospitalizations among individuals with mental health and addiction issues. Natural disasters like wildfires and other emergency events also impacted mental health and addiction needs, leading to an increase in emergency department visits for such conditions. In addition, there are rural and remote areas in Alberta where mental health and addiction services are not readily available in the community, leading residents in these areas to rely on the emergency department for their mental health and addiction concerns.

Performance Indicator 1.b:

Percentage of Albertans receiving care at an appropriate level (under development)

The performance indicator on the percentage of Albertans receiving care at an appropriate level was originally designed to demonstrate how investment and cost-avoidance across multiple systems can reduce the need for more costly and less effective interventions, but over time, this indicator has evolved. In previous years, significant work was undertaken to explore the development of a more meaningful measure of system impact, including leveraging the expertise of the Recovery Expert Advisory Panel. The focus shifted in 2023-24 to better capture clients' interactions with different programs, aiming to develop metrics that more effectively reflect the holistic impact of the Alberta Recovery Model. Although the ministry conducted a preliminary analysis of available data, this work was limited by legislative and privacy constraints.

In 2024-25, the performance indicator remained unchanged, consistent with a broader shift toward long-term planning and system-wide improvements in performance measurement. Like 2023-24, the analysis focused on VODP, drug consumption services, bed-based addiction services, and mental health-related hospital admissions.

As part of the ministry's ongoing evolution and maturation of performance measurement, this indicator has been discontinued and will be replaced in 2025-26 with a new performance measure: the percentage of individuals primarily admitted for more than 30 days to hospital inpatient care for a mental health issue, which is more aligned with the outcome (Outcome 1 in the 2025-28 Business Plan). By focusing on hospital inpatient admissions over 30 days, the new indicator allows

the system to better assess whether individuals are receiving timely, appropriate care in the community—before their needs escalate to the point of requiring prolonged hospitalization.

As Alberta continues to build a recovery-oriented system of care, efforts to reduce barriers to data access are expected to support more holistic analyses of client pathways and outcomes across sectors. This will strengthen the ministry's ability to evaluate the long-term impact of recovery-focused policies and programs in areas such as health, justice, education, and housing. Over time, improved data integration may also support a better understanding of how individuals' access and interact with services, contributing to more coordinated, person-centred care.

Preliminary results: In 2024-25, 17,335 Albertans received care from one or more of VODP, drug consumption services, and bed-based addiction services—an increase of 1,237 individuals or 7.7 per cent, compared to 2023-24 (16,098 individuals). The increase may reflect reduced financial barriers, greater public awareness, and the expansion and specialization of programs offering more opportunities and individualized services. Bed-based addiction services comprise three distinct services: withdrawal management, residential (intensive) treatment and residential recovery services. In 2024-25, bed-based addiction services had the most clients, followed by VODP. Of the clients enrolled and discharged from VODP, nearly 28 per cent were transitioned to another Opioid Dependency Program (ODP) clinic and/or another prescriber within 30 days of discharge. This represents a decrease of five percentage points compared to the previous year, when 33 per cent of clients were transitioned. Such fluctuations can be expected, as clients may choose to transition their care to their family physician or primary care provider instead of another ODP clinic or may choose to transition to non-pharmaceutical treatment options. Data about drug consumption services has some limitations because Personal Health Number (PHN) disclosure by a client is not required to access these services, therefore clients who do not produce a validated PHN are excluded from the data used for the analysis. However, PHN data quality has improved over recent years, with PHN being provided nearly 90 per cent of the time, supporting more reliable reporting.

The *Mental Health Act* enables care for individuals with serious mental health illnesses who are at significant risk of causing harm to themselves and/or to others. In 2024-25, the proportion of individuals admitted three times or more to a hospital under the *Mental Health Act* was 7 per cent³ compared to 8.7 per cent⁴ in 2023-24—a decrease of 1.7 percentage points. The decrease could suggest that more accessible and effective mental health care with appropriate prevention and intervention supports are available in the community and barriers to access to care are being reduced. The metric is included as performance indicator 4.a in the 2025-28 Mental Health and Addiction Business Plan and will be reported on in detail in the ministry's 2025-26 Annual Report.

In 2024-25, the proportion of individuals primarily admitted for more than 30 days to hospital inpatient care for a mental health issue also decreased. This is a broader segment of people that is not specific to admission through the *Mental Health Act*. The proportion of patients who had a length of stay of more than 30 days for a mental health-related illness decreased from 16.9 per cent⁴ in 2023-24 to 13.9 per cent³ in 2024-25. This decrease may reflect the positive effects of enhanced community-based care and improved coordination between hospital and community services, leading to more effective care and fewer long hospital stays. This metric is included as performance measure 1.a in the 2025-28 Mental Health and Addiction Business Plan and will be reported on in detail in the ministry's 2025-26 Annual Report.

³ Figures reflects data available as of April 2, 2025, and may change once the full data set is available in June 2025.

⁴ Figure as been updated based on the release of additional data and does not match what was presented in the 2023-24 Mental Health and Addiction Annual Report.

Outcome Two: Alberta's communities and families have the capacity to support individuals pursuing recovery

The Ministry of Mental Health and Addiction is working with government and community partners to ensure consistent resources, supports, and services are available across the province to support effective implementation of the Alberta Recovery Model. The ministry is responsible for building recovery communities and mental wellness centres for children, youth, and adults and funding their operation. This expansion is helping to remove barriers and provide better access to mental health and addiction treatment and recovery services. The ministry is committed to enhancing the continuum of culturally appropriate services available to Indigenous Peoples in Alberta and building capacity in communities to support individuals to pursue recovery.

Psychosocial supports during emergency events

The Ministry of Mental Health and Addiction has developed a community **psychosocial response plan** to strengthen the Government of Alberta's processes in addressing psychosocial needs during emergency events and ensure roles and responsibilities are clear in supporting Albertans during difficult times. Mental Health and Addiction and the Ministry of Seniors, Community and Social Services have crucial roles in the government's coordinated psychosocial response during such emergencies—Mental Health and Addiction leads in overseeing Alberta's recovery-oriented system of mental health and addiction care, and Seniors, Community and Social Services provides essential social supports, including financial benefits and emergency shelter, that contribute to the overall psychosocial well-being of affected populations.

The Psychosocial Response and Recovery Coordination Team, co-chaired by the Department of Mental Health and Addiction and Recovery Alberta, coordinates communication and actions between levels of government, service providers, Indigenous communities, and municipalities impacted by disaster events to address the psychosocial needs of affected Albertans and to mitigate the psychological and social impacts of emergencies on vulnerable populations. Psychosocial supports coordinated at the provincial level with the local incident command during the **2024 Jasper Wildfires** included information sharing about available supports and mental health services such as counselling, delivering peer-based first responder supports, providing on-site mental health and addiction staff at reception centres, and ensuring continuity of addiction treatment, harm reduction, and recovery services for Albertans evacuated from their communities.

Key Objectives

- 2.1 Expand oversight responsibility for and increase the number of mental health and addiction treatment facilities in Alberta to streamline service delivery and improve access to a consistent continuum of supports spanning prevention, intervention, treatment, and recovery.**

Expanding access to bed-based addiction treatment

Bed-based addiction treatment in Alberta involves inpatient services where individuals remain on-site for a designated length of time. These services are currently provided in recovery communities and about 110 licenced facilities that provide short- and medium-term intensive and non-intensive treatment and recovery services and medical withdrawal management services. The Ministry of Mental Health and Addiction is building system capacity for addiction treatment

services by establishing additional bed-based options for individuals with specific needs, including compassionate intervention centres, and a youth and recovery centre.

Recovery communities are long-term (up to one year) bed-based treatment facilities offering holistic rehabilitation and comprehensive supports in alignment with the Alberta Recovery Model. They help individuals restore their physical, mental, and emotional health, rebuild relationships, and develop life skills, while providing access to medical care (e.g. opioid agonist treatment), counselling, vocational training, and financial and housing supports. Each client follows an individualized care plan and is paired with a peer mentor who is in later stages of recovery.

As residents progress in their recovery, they take on increasing personal and social responsibility within the community. Extensive reintegration supports—including employment training, continuing education opportunities, and connections to community resources—ensure a smoother transition back into society and set clients up for success in their recovery.

The Ministry of Mental Health and Addiction has committed to establish 11 recovery communities across the province, including five in direct partnership with Indigenous communities, to reduce barriers and improve access to recovery-oriented supports for more Albertans. About 700 new long-term addiction treatment beds will be added and available to all Albertans when the 11 recovery communities are operational, helping more than 2,000 people over the age of 18 each year in their pursuit of recovery and increasing the recovery capital in communities.

In early April 2025, the **Lethbridge Recovery Community** received a **community development award** of merit from the Consulting Engineers of Alberta that recognized the innovative modular design of the recovery community, with components built off-site and assembled on-site, providing adaptable, scalable support tailored to diverse addiction recovery needs.

The project also earned an Honourable Mention for Permanent Modular Healthcare award at the Modular Building Institute's 2025 World of Modular - Awards of distinction in Las Vegas, which celebrates global excellence in modular construction. The **Red Deer Recovery Community** received the same award, as well as the Judge's Choice, at the 2024 event in Florida.

In 2024-25, Alberta's government provided more than \$106 million (more than \$21 million in operating and about \$85.4 million in capital) to build and operate recovery communities throughout Alberta. This includes an investment of about \$72.5 million to build recovery communities in partnership with Indigenous communities. As of March 31, 2025, recovery communities in Red Deer, Lethbridge, and Lakeview at Gunn are operational. Collectively, these facilities served a total of 412 unique individuals in 2024-25. Wait time data for the operational recovery communities is not currently reported as the ministry plans to engage evaluation experts and facility operators in 2025-26 to develop a standard suite of performance metrics and establish a consistent reporting cadence that supports comparability across recovery community sites.

The Calgary Recovery Community is near completion and expected to open in summer 2025, and the remaining sites in Edmonton, Grande Prairie, Blood Tribe, Enoch Cree Nation, Siksika Nation, Tsuut'ina Nation, and with the Métis Nation of Alberta are in various stages of development (see Key Objective 2.3 for details on the Indigenous partner recovery communities).

2024-25 Recovery Community Operations

	Red Deer	Lethbridge	Lakeview at Gunn
Operator	Edgewood Health Network Canada Inc	Fresh Start Recovery Centre	ROSC Solutions Group Inc
Opened	May 2023	September 2023	July 2024
Bed capacity	75 beds (58 male, 17 female)	50 beds (male)	75 beds (male)
Clients per year	Up to 300	Up to 200	Up to 300
Average length of stay (days)¹	153.6	158	56.9
Total admissions	180	106	170
Total unique admissions	146	101	165
Total program completions	78	80	54 ²
Key Results	97.8 per cent of clients who completed the program secured safe housing and a stable income	92 per cent of clients who completed the program secured safe and stable housing	85 per cent of clients who completed the program connected with ongoing treatment providers, secured stable housing, returned to their own homes and/or reunited with their families

¹ Average length of stay reported by Red Deer is based on admissions. Lethbridge and Gunn are based on discharge.

² The Lakeview Recovery Community in Gunn has been operating for less than eight months and initially employed a staggered intake approach. This accounts for the low level of successful completions recorded.

In February 2025, the Minister of Mental Health and Addiction announced plans to develop two new compassionate intervention centres in the province to support the implementation of the compassionate intervention legislation: the Southern Alberta Compassionate Intervention Centre in Calgary and the Northern Alberta Compassionate Intervention Centre in Edmonton, each with capacity for 150 beds.

The compassionate intervention centres are expected to serve as secure, multi-functional facilities where patients over 18 years of age with complex addiction needs can access a spectrum of bed-based treatment and supports, including assessment, medical withdrawal management and stabilization, and individual and group therapy. Comprehensive discharge planning and aftercare supports are critical to ensure individuals can successfully transition to community-based supports, such as a recovery community or into outpatient treatment with appropriate secure housing, to continue their recovery journey. Construction on these facilities is anticipated to begin

in 2026 and be completed in 2029. In 2025-26, the ministry will explore early implementation options to begin delivering compassionate intervention services to the first patients in fall 2026.

In 2024-25, \$5.9 million in capital funding was spent to support early planning activities for the compassionate intervention centres, including initial work to develop new information technology solutions to facilitate full implementation of compassionate intervention. Functional programming for the facilities is complete, and procurement documents are being developed. With the compassionate intervention legislation now passed, construction will move forward.

Announced in November 2024, the Northern Alberta Youth Recovery Centre is being built to address the need for more bed-based addiction treatment capacity for youth, utilizing existing infrastructure at the Edmonton Young Offenders Centre. The new centre will be separate from the correctional facility with no shared living areas and will have its own dedicated entrance. Planned renovations include a refreshed interior design, new clinical, educational, and office spaces, and living units tailored for bed-based addiction treatment. These upgrades will create an environment focused on health and wellness, following the model used throughout Alberta's recovery-oriented system of care. Leveraging existing infrastructure will reduce the timelines and costs for this new facility, putting the centre on a path to open to clients in 2026. Upon completion, the Northern Alberta Youth Recovery Centre will have a capacity of 105 beds and be able to serve up to 300 youth annually, increasing the total number of youth addiction treatment beds in Alberta from 70 to 175. In 2024-25, \$1.1 million in capital funding was spent on planning, facility design, and construction for this project, and the total budget to complete the build is \$23 million.

Alberta's government is committed to making addiction treatment accessible for Albertans in need through the ongoing expansion of programs that build addiction treatment capacity as part of the Alberta Recovery Model. This commitment extends beyond large scale projects like recovery communities and compassionate intervention centres to localized initiatives that are adding capacity on a smaller scale. Every new treatment space added to the system removes barriers, improves access to services, and gives more people the opportunity to pursue recovery.

The George Spady Centre Society's new medical withdrawal management facility is expected to open in 2025. Supported by \$8 million in funding over three years from the Government of Alberta, the facility will offer a 41-bed unit, including 10 net-new beds, where clients will receive self-directed individualized care to safely withdraw from substances. It will also provide 19 post-withdrawal management recovery beds, including nine net new, for individuals who have completed the withdrawal management program but require additional non-clinical supports to safely transition to their next care provider.

In 2024-25, more than \$2.5 million supported additional bed-based expansion projects (see details on people served through bed-based treatment in Key Objective 3.2), including:

- Renovating the Safe Harbour Society's existing facility in Red Deer facility to add four new post-withdrawal management recovery beds in 2025.
- Operating 10 addiction recovery beds for men over the age of 18, with a capacity to support up to 40 clients per year at Opportunity Home Treatment and Recovery Centre in Drayton Valley.
- Opening seven youth/young adult addiction treatment beds at the Okisikow Iskwew Youth Addiction Center on Frog Lake First Nation in February 2025.
- Providing funding to open 20 adult recovery beds at the Northeast Addiction and Mental Health Centre for Holistic Recovery in Calgary.
- Opening 20 adult recovery beds in August 2024 at the Red Deer Dream Centre.

- Adding two medical withdrawal management beds for a total of 23 beds, and four post-withdrawal management recovery beds at the Foothills Centre in Fort Macleod.
- Enhancing Indigenous programming and bringing bed rates up to the standard rates for medical withdrawal management, addiction treatment, and young adult addiction treatment beds at the Poundmaker's Lodge Treatment Centres.

Expanding oversight responsibility for mental health and addiction treatment facilities

The Department of Mental Health and Addiction is expanding oversight responsibility for mental health and addiction treatment facilities by developing and implementing a right-touch approach to quality assurance and oversight. In 2024-25, the department's Compliance and Monitoring unit developed a regulatory model based on the "right-touch approach" stressing engagement with service providers that is relational, fair, transparent, and professional. The approach transcends the MHSPA and can be applied to any regulatory, oversight, or quality assurance framework, facilitating overarching government or ministry objectives such as the Alberta Recovery Model. The embedded risk-matrix approach ensures any action taken is consistent, proportional, and effective at mitigating unnecessary health and safety risks to Albertans early and to the fullest extent possible. The model is expected to roll out in 2025-26.

2.2 Invest in mental health facility expansion to increase access to a suite of community-based mental health services across the province that provide consistent supports to youth experiencing mental illness.

Building mental health system capacity

The Ministry of Mental Health and Addiction is investing in prevention and early intervention programs that focus on youth mental health and building resiliency, and ensuring specialized treatment is available for youth who require more intensive support.

Kickstand Centres (<https://mykickstand.ca/>) make it easier for youth to find help and access support in their community. These fully integrated centres serve as a hub, or single point of access, for youth aged 11 to 25 and offer a range of no-cost, confidential supports spanning mental health and addiction services, primary health care, social services (such as employment, recreation, and life-skills programs), peer support, and Indigenous wellness supports. Kickstand Centres address common barriers to care by integrating services across providers, offering extended hours, and providing walk-in therapy sessions with no waiting list. Clients are not required to have a referral or meet admission criteria.

In 2024-25, Kickstand Centres opened in Bonnyville, Fort Saskatchewan, Grande Prairie, and Edmonton. Development of the Edmonton centre was made possible through philanthropic funding. Collectively, Kickstand Centres in Bonnyville, Fort Saskatchewan, and Grande Prairie, received more than 7,000 visits with more than 1,200 unique youth receiving services. Kickstand Edmonton, which opened in February 2025, is seeing an average of 62 new service users per month. Of respondents to the post-visit survey, 88 per cent reported agreement or strong agreement that they felt better after participating in the program and 17 per cent indicated that if they had not gone to a Kickstand Centre, they would not have accessed help.

Kickstand also operates a virtual service platform, Kickstand Connect, which offers no-cost, confidential online access to appointments for mental health, substance use, peer support, and employment services, as well as workshops and support groups. In 2024-25, Kickstand Connect reported 816 completed appointments, with 83 per cent of these being for mental health or addiction support services. This represents an increase of about 300 per cent in appointments

compared to the prior year. In 2024-25, the Department of Mental Health and Addiction provided \$3.6 million to continue to implement Kickstand Centres and support Kickstand Connect.

Over the coming year, Kickstand will focus on establishing Kickstand Centres in seven additional communities across Alberta (Drayton Valley, Medicine Hat, Strathmore, the Tri-Region, Enoch Cree Nation, Alexis Nakota Sioux First Nation, and Samson Cree Nation) with a goal of completing implementation before March 2026. Kickstand is also exploring opportunities to expand Kickstand Connect programming to include psychiatric and primary care services through complementary health care professional teams.

In partnership with CASA Mental Health, the province is expanding the CASA House program (<https://casamentalhealth.org/getting-help-2/casa-house/>) to three additional communities over the next three years: Fort McMurray, Calgary, and Medicine Hat. CASA Houses offer referral-based live-in and day treatment programming. The live-in CASA House program is available to youth in grades 7 to 12 with complex mental illness and social challenges, often with concurrent addiction or learning issues, who have not responded as expected to previous interventions. The program is family-centred, with parents and caregivers actively participating in treatment while youth live at a CASA House facility. Treatment includes individual, group, and family therapy, social and life-skills training, and on-site schooling in small classrooms. The goal is to help young people closer to home and reduce the need for hospital stays; the average length of stay in the live-in program is about four months. The day program is available to youth in grades 8 to 12 who have a diagnosed mental illness and struggle in a conventional school setting, allowing them to attend school at a CASA House while receiving ongoing therapy.

In 2024-25, the department provided \$29 million (\$18 million in capital and \$11 million in operating funding) to support planning and construction of the three new CASA facilities, as well as relocation of the existing CASA House from Sherwood Park to Edmonton. The existing CASA House in Sherwood Park will continue operating until the new location opens. The intent is to co-locate live-in and day treatment programs in the new and expanded facilities to allow for a seamless continuum of care, where clients can transition to and from the two programs without disruption.

The new CASA Houses in Fort McMurray and Calgary are expected to open in 2027, while the Medicine Hat and Edmonton facilities are expected to open in 2029. Once the three new facilities are fully operational, CASA House programs will be able to serve more than 300 additional youth with complex mental illnesses per year. In 2024-25, the existing CASA House program served more than 20 youth with complex mental illnesses. In addition, CASA began delivering Trauma Program services out of temporary spaces in Calgary in February 2025.

The Ministry of Mental Health and Addiction is continuing to fill a critical gap in mental health care by providing support and services to help Albertans recover from complex eating disorders. In 2024-25, the Department of Mental Health and Addiction and Recovery Alberta spent nearly \$3.3 million to support the establishment and operation of the EHN Sandstone Recovery Centre in Calgary for youth and young adults aged 12 to 24 with complex eating disorders. The Silver Linings Foundation also raised and contributed \$4 million in capital funding for this 12-bed facility, operated by EHN Canada, that provides intensive, specialized treatment at no-cost for up to 52 young people each year. By offering care in a community setting, the centre helps reduce the need for prolonged hospital stays and improves health outcomes for patients. The centre delivers an individualized approach to care with 24/7 support from a multidisciplinary team of physicians, nurse practitioners, registered dietitians, nurses, counsellors, and support staff. Services include weekly individual and family therapy, daily group therapy, nutritional rehabilitation, structured meal support therapy, self-care and wellness activities, nutritional education, and distress tolerance skills. The length of treatment varies based on individual needs, generally ranging from three weeks to four months.

A total of \$10 million is budgeted over three years (2023-24 to 2025-26) for this initiative. As of March 31, 2025, 24 patients have been admitted to the facility with additional referrals being triaged. Within the first year of opening, there have been significant enhancements in clinical care, including the implementation of family programming and therapy groups. In addition, substantial investments have been made in staffing, including hiring therapists, clinical supervisors, dietitians, nursing staff, and physicians to meet the unique needs of the patient population. Collaborative partnerships with community agencies have been established, and future plans include increasing bed capacity and managing higher acuity patients, setting a strong foundation for long-term success (see Key Objective 3.2 for additional information on eating disorder supports).

2.3 Develop a comprehensive continuum of culturally appropriate mental health and addiction services to support Indigenous people in Alberta.

Partnering with Indigenous communities to provide culturally safe addiction treatment

As part of the ongoing effort to build the Alberta Recovery Model and remove barriers to land-based, culturally safe services for Indigenous Peoples, Alberta's government is establishing five recovery communities in direct partnership with Indigenous communities across the province. These facilities are being built on Indigenous lands with Enoch Cree Nation, Tsuut'ina Nation, Siksika Nation, Blood Tribe, and the Métis Nation of Alberta. Included among the 11 new recovery communities province-wide, these facilities will provide culturally relevant, community-based services and holistic healing.

The Indigenous-led recovery communities are at various stages of planning and development. Once operational, each recovery community will add about 75 long-term treatment beds to the system, with each being capable of supporting up to 300 people per year in their pursuit of recovery. In 2024-25, \$72.5 million in capital funding was provided to support planning and construction of these recovery communities, which is \$26.5 million below the budget of \$99 million. The lower spending was due to reprofiling of capital grants for project delays in some Indigenous-led recovery community builds.

In 2024-25, Siksika Nation worked with the province on planning and design and a groundbreaking ceremony was held in June 2024. As of March 31, 2025, the recovery community was under construction and is expected to be completed by fall 2025. In August 2024, Alberta's government announced the Healing Waters Recovery Community in partnership with the Métis Nation of Alberta. Together with the province, the Métis Nation of Alberta worked to plan and design a facility that reflects and honours Métis culture. As of March 31, 2025, Healing Waters and the Enoch Cree Nation the recovery community were under construction and are expected to be complete by the end of 2025-2026.

Construction progressed on the recovery community at Blood Tribe and Nation-led community engagement and site selection were completed for the Tsuut'ina Nation recovery community in 2024-25. The ministry provided capacity-building support to project teams from each community starting in fall 2024 (see Key Objective 1.2 for additional details).

Supporting culturally relevant community-based programs and services

Mental Health and Addiction continues to fund a suite of programs and initiatives delivered by Indigenous partners to ensure culturally relevant mental health and addiction supports are available to First Nations, Inuit, and Métis peoples in Alberta across the continuum of care. In 2024-25, the department provided more than \$4.6 million to support distinct initiatives pertaining to Indigenous ways of knowing and healing. These community-led investments aim to

deliver frontline mental health and addiction services closer to home and address service gaps through culturally grounded, locally informed solutions. Key actions in 2024-25 included:

- Expanding community navigator capacity through initiatives like the Stoney Nakoda Tsuut'ina Tribal Council First Nation Client Liaisons and the Coordination for Integrated Mental Health and Addiction Programming project. These initiatives support trusted local navigators in connecting individuals to culturally relevant care and strengthening partnerships between Indigenous communities and the public health system.
- Helping individuals overcome barriers to accessing timely, culturally relevant care through supports such as Metis Settlements Mental Health and Addictions Navigators the Community Wellness Advocate Program. These programs provide navigation assistance, crisis financial support to help overcome waitlists, and facilitate connections to services. Population health surveillance, provided under the Community Wellness Advocate Program, helps inform planning and service delivery in alignment with local needs and priorities.
- Supporting life-skills and reintegration programs, such as Treaty 6 Wellbriety Movement and Mental Health and Addiction Supports Program that combine traditional healing with personal development to help individuals maintain sobriety, address trauma, and re-integrate into family, community, and work life.

Program highlights

The Building Bridges from Care to Home Program, delivered by the Kainai Transition Centre Society (KTCS), provides support to individuals transitioning back to their families and communities who are challenged by addiction, trauma and mental health issues. The program is located on Blood Tribe land and provides 22 transitional beds where clients can stay for up to one year to engage in Indigenous healing practices and counselling. In 2024-25, 280 individuals accessed psychological services within the program, almost double the number of individuals over the prior year. A total of 600 education sessions, 180 recovery support meetings, and 200 one-on-one sessions with Elders were held throughout the year, all of which represent significant increases compared to 2023-24. In addition, KTCS enhanced its programming to include more cultural on-the-land activities, such as cultural camps, participating in the Sundance, and learning at various spiritual/historical sites. They also collaborated with the Blood Tribe Department of Health to plan the transition from a medical to a social model, set for execution in 2026.

Bringing the Spirit Home, delivered by the Blood Tribe Department of Health Inc., is an Indigenous-led, voluntary program that is no cost to clients. It includes a voluntary medical withdrawal management program and a post-detox/pre-treatment program.

The medical withdrawal management program offers 24 beds for stays of up to 14 days, providing trauma-informed care and culturally appropriate, medically supported withdrawal management services. Clients are connected by a case manager to wraparound supports such as professional counselling, peer support, traditional healing, and ongoing medical care. In 2024-25, 459 individuals were admitted to the program, an increase of more than 19 per cent compared to 2023-24. This included 448 through self-admission, and 238 successfully completed detox and stabilization in opioid replacement therapy.

The post-detox/pre-treatment program provides 20 spaces for individuals who are awaiting treatment or recovery placements, helping to prevent relapse during this critical and transitional phase. Clients stay for up to three months, participating in meaningful daily activities delivered by a diverse team including persons with lived experience, recovery coaches, social workers, addictions counsellors and Elders, strongly informed by Blackfoot culture. In 2024-25, 147 individuals were admitted to the program: 13 were discharged to the community, 43 participated in Day Treatment Programming, and 87 transferred to various recovery facilities.

In response to the Jasper wildfire relief efforts, funding was provided to Little Red River Cree Nation through the Honouring Life Program to support activities intended to:

- Bring community members together and provide cultural learning opportunities to help build their sense of community and connection.
- Support community members' mental wellness during the evacuation period.
- Provide opportunities for Elders and youth to connect and find joy and strength in Cree language and culture.

All activities were led by Elders and Knowledge Keepers. The impact of these activities included creating a sense of community and learning about the strength of Cree culture and teachings. During stressful and difficult times, the activities allowed youth and community members to connect with Elders and each other, and to share emotional difficulties while providing support to others. The cultural workshops and activities provided an opportunity for community members to spend time away from crowded evacuation centres and offered a distraction from the realities of evacuation from their homes and community and feelings of isolation and hopelessness. Multiple workshops and activities were offered in locations where members were evacuated.

Performance Indicator 2.a:

Recovery-supportive community scores for selected Alberta communities (under development)

In 2024-25 the work on this performance measure was put on hold due to several limitations such as costly methodology, lack of historical data for system linkages and trend analysis, and the risk of unrepresentative samples. The ministry remains committed to exploring opportunities to measure and understand community support for recovery.

Going forward, performance metrics for the community-focused outcome (Outcome 3 in the 2025-28 Mental Health and Addiction Business Plan) include:

- Mental health and addiction emergency department visits, which reflects the benefit of individual Albertans' improved access to community-based mental health and addiction programs and services.
- Operational and newly created mental health and addiction facilities, which demonstrates the progress and success of targeted strategies to build mental health and addiction system capacity and improve the availability of supports and services for Albertans pursuing recovery.

Outcome Three: Albertans have access to high-quality, person-centred mental health and addiction programs and services

The Ministry of Mental Health and Addiction works with government and community partners to provide access to a comprehensive continuum of supports and services spanning prevention, intervention, treatment, and recovery for Albertans at risk of, or experiencing, mental health and addiction challenges. The Department of Mental Health and Addiction is responsible for directly funding the delivery of a wide range of community-based programs and services that provide mental health and addiction support for children and youth, adults, Albertans in rural and remote areas, and other vulnerable and underserved communities. Many of these programs are delivered by Recovery Alberta.

Key Objectives

3.1 Extend the My Recovery Plan platform roll out to support evidence-based decision making and help more Albertans living with addiction build their recovery capital and improve their ability to pursue and maintain recovery.

Assessing recovery outcomes

The My Recovery Plan (MRP) digital platform is enabling evidence-based, person-centred care by helping individuals measure their recovery capital—the personal and community resources that support recovery (see page nine for a full description of recovery capital, including the eight domains). MRP is being implemented in all publicly funded bed-based addiction treatment services and recovery communities in Alberta.

Building and sustaining recovery capital is essential to the Alberta Recovery Model. MRP helps individuals assess, plan, monitor, and measure their own recovery capital as they follow a recovery pathway unique to them. Recovery Alberta provides operational oversight of MRP in its operated and contracted facilities, including protection of client information through strong privacy management processes. As part of this oversight responsibility, Recovery Alberta frequently engages with all sites that have implemented MRP to provide ongoing training and support as needed. This ensures processes are being followed and accurate data is being captured.

MRP includes a recovery capital assessment that generates a Recovery Capital Index (RCI) score for consenting clients at admission. Depending on the length of a program, assessments may be conducted at several points during an individual's stay to monitor changes in recovery capital and inform care planning. Increases in recovery capital are associated with positive outcomes for individuals experiencing addiction and speak to the strength, quality, and person-centred nature of addiction programs and services for people pursuing recovery.

In 2024-25, the Department of Mental Health and Addiction spent almost \$888,000 to implement and operate the MRP platform across Alberta's publicly funded bed-based treatment facilities and operational recovery communities. As of March 31, 2025, MRP has been adopted in 39 bed-based addiction treatment facilities, including all three operational recovery communities, all 10 Recovery Alberta-operated programs, and 26 of 33 Recovery Alberta-contracted service providers. MRP has not yet been implemented in Indigenous-specific bed-based treatment programs; engagement with Indigenous communities is ongoing to ensure the tool and its implementation are culturally appropriate. Since launching in July 2022 through March 31, 2025, more than 7,480 Albertans have enrolled in MRP. Of this total, almost 6,000 individuals had at least one assessment of their recovery capital. In the coming year, the ministry will continue to expand its efforts in recovery capital measurement, including embedding measurement and care planning in more seamless and nuanced ways across services in the continuum of care.

In January 2025, all three operational recovery communities began using MRP. As of March 31, 2025, a total of 164 recovery community clients had been enrolled in the MRP platform, representing 36 per cent of all recovery community admissions in 2024-25. Of these clients, 96 per cent had completed at least one recovery capital assessment. MRP will be implemented in new recovery communities as they open. Over the next year, clients will begin completing their second assessments, allowing the ministry to calculate improvements in RCI scores among recovery community clients and gain further insight into recovery community outcomes.

Most publicly funded bed-based addiction treatment service providers have been collecting individual-level recovery capital data through MRP since late 2022-23, which has allowed providers to monitor the effectiveness of their programs and meet reporting requirements more easily. Anonymized aggregate data from MRP on treatment access, changes in recovery capital, and system capacity is helping the department provide effective oversight to ensure Albertans receive the supports they need for long-term recovery. As more MRP data is generated, it is expected to help identify trends, recovery barriers, and service gaps, providing evidence to guide future policy and funding decisions. Data on social supports, justice system involvement, and vocational and housing needs will enable analysis of recovery barriers for specific client groups or service locations. MRP data is also helping service providers support their clients by highlighting barriers and deficits affecting each person’s recovery capital. Recovery Alberta is continuing to work with operational leaders and providers to better understand the applicability and uses of MRP data.

3.2 Expand access to a range of virtual and in-person recovery-oriented supports to improve mental health and addiction care and support better outcomes for communities in need, including underserved, rural, and remote populations.

Supporting and delivering bed-based mental health and addiction care

Implementation of the Alberta Recovery Model is helping to remove barriers and ensure every person has an opportunity to pursue recovery. Since 2019, Alberta’s government has worked to establish a system of care that provides treatment and recovery options for Albertans when and where they need it, including establishing more than 10,000 new publicly funded addiction treatment spaces that are provided at no-cost to all Albertans.

2024-25 Provincially Funded Bed-Based Addiction Treatment Client Statistics^{1 2}

- 36 per cent received intensive addiction treatment services.
- 13 per cent received non-intensive addiction recovery services.
- 79 per cent³ received medical withdrawal management services.
- 63 per cent identified as male.
- 37 per cent identified as female.
- The client average age is 38.

¹ Service totals do not add up to 100 per cent as some clients used more than one service.

² Excludes recovery communities.

³ Excludes clients presenting under the influence of intoxicants who receive overnight shelter accommodation.

Recovery Alberta delivers 1,414 addiction treatment beds. Addiction beds are in community within self-contained settings or separate units within a facility to provide treatment for clients with substance use disorders and gambling problems. Treatment includes withdrawal management, clinical care, assessment, counseling, treatment, and recovery. These beds are operated by Recovery Alberta and contracted service providers. Data for addiction services in Alberta indicates occupancy rates of between 85 and 90 per cent in 2024-25 for most of the addiction treatment

services Recovery Alberta delivers. Ideal occupancy rates are near 90 per cent since this minimizes unused capacity while also allowing for patient flow and flexibility in responding to patient need. In 2024-25, more than 10,000 unique clients were served in addiction treatment beds.

Recovery Alberta's 1,350 community mental health beds, transitional, and long-term services are delivered to clients with varying degrees of mental health issues. This may include integrating safe and stable housing with targeted addiction and/or mental health recovery-oriented services. It may also involve supporting clients' behavioural, social, physical, and medical needs along a continuum. In 2024-25, more than 1,375 clients received support from Recovery Alberta in these settings.

Recovery Alberta delivers inpatient services through 742 beds within 17 active treatment hospitals across the province. These nursing unit beds are designated for Albertans who need treatment for general psychiatric or psychological disorders that cause moderate to severe impairment, or require care related to psychoactive substance use. Inpatient psychiatry beds deliver assessment, stabilization, and treatment of mental, psychiatric, and psychological disorders to patients who require 24/7 psychiatrically supervised care in a safe and secure hospital location. From April 1, 2024, to December 31, 2024, more than 12,300 unique clients were treated for an addiction or mental health concern in acute care settings.

Alberta has 936 dedicated beds in five standalone psychiatric hospital facilities that deliver services exclusively for psychiatric care and psychiatric supervision. These facilities provide targeted designated psychiatric services across the addiction and mental health continuum, primarily, but not limited to, bed-based inpatient care. All five sites offer inpatient, outpatient, and day hospital care. Each site has a unique combination of services that work in alignment with other local psychiatric resources. Recovery Alberta operates four of these psychiatric facilities (Southern Alberta Forensic Psychiatric Centre, Claresholm Centre for Mental Health and Addictions, Centennial Centre for Mental Health and Brain Injury, Alberta Hospital Edmonton), while Covenant Health operates one location in Edmonton Zone (Villa Caritas). From April 1, 2024, to December 31, 2024, the five facilities served 3,150 unique clients.

Change in Addiction and Mental Health Beds

Bed type	Total – March 31, 2024	Total – March 31, 2025	Difference	% Change
Addiction (bed-based, other)	1,354	1,414	60	4.4%
Inpatient Care (acute, standalone)	1,677	1,678	1	0.1%
Community Mental Health (supported living)	1,386	1,350	-36	-2.7%
Total Addiction and Mental Health Beds	4,417	4,442	35	0.6%

It is important to note that in addition to bed-based care, Recovery Alberta provides a variety of supports through community-located treatment services for mental health, addiction, or concurrent disorders. These include individual, group, and family services. A large range of clinical and peer supports helps clients recover and maintain health in their community.

In 2024-25, Recovery Alberta treated more than 151,000 unique clients through nearly 1.9 million visits in community outpatient settings.

Providing accessible prevention and early intervention supports

The Government of Alberta works with community partners to ensure province-wide access for those seeking help with mental health or addiction challenges early on. Whether Albertans reach out by phone, online, or in person, they can quickly find help and be connected to care in their communities. In 2024-25, the ministry partnered with several service providers to link people to resources in their local community, including health and social services, to improve mental well-being and outcomes regardless of where they live.

Access Services for Addiction & Mental Health are Recovery Alberta telephone-based resources spanning the province that acts as the first access point for non-urgent mental health and addiction information and services. Phone supports include navigation, screening, assessment, referral, consultation, crisis intervention, outreach, and short-term stabilization. Callers requiring access to a mental health or addiction service are screened, triaged, and provided with a first appointment booking, or are provided with information on appropriate community resources. The program is divided into three geographic areas covering (1) Edmonton zone, (2) Calgary zone and (3) Northern, Central and Southern zones. In 2024-25, Access Services received nearly 135,000 calls.

211 Alberta (<https://ab.211.ca/>) is a free, confidential, 24/7 service that provides immediate support and helps connect Albertans to the right resources or services based on their needs. It offers a single point of contact for high-quality system navigation, connecting callers to information and community supports such as health, social, government, and addiction and mental health services. 211 Alberta is available by phone, text, or online chat, with phone service in more than 170 languages.

211 Alberta is an initiative of the United Way Alberta Capital Region and is delivered in partnership with the Canadian Mental Health Association Edmonton Region, and Distress Centre Calgary. This collaboration combines expertise and resources to strengthen 211 Alberta's ability to deliver high-quality and sustainable services. Over the past year, 211 Alberta expanded specialized navigation services to include more supports for individuals with complex needs and worked to improve seamless access with partner organizations. (e.g., crisis lines, Alberta Support Contact Centre/Income Support Contact Centre, Caregivers Alberta, Alberta One Line, and the Métis Nation of Alberta). Collaborative relationships with services such as the 9-8-8 Suicide Crisis Helpline, 911 Calgary Police for call diversions, and REACH Edmonton for 24/7 Crisis Diversion further enhance its ability to support individuals in distress or seeking mental health and social resources.

In 2024-25, the department provided more than \$2.9 million for 211 Alberta to meet growing demand, enhance community resource listings in rural areas, improve responses for underserved populations, better connect callers to culturally appropriate local supports, and develop specialized navigation services for children, youth, and families. 211 Alberta logged more than 117,000 interactions in 2024-25 (approximately 105,000 phone calls, 4,400 texts, 6,700 chats, and 1,250 other interactions), up 33 per cent from previous year. Of this total, more than 19,500 contacts were related to mental health or substance use disorder needs, and almost 3,800 involved crisis de-escalation. Notably, more than 1,400 contacts involved both crisis de-escalation and a mental health or substance use-related concern. Beyond typical operations, 211 Alberta adapted its services to meet the needs of those impacted by the Jasper wildfires, providing tailored support and in-person assistance at the Jasper Re-Entry Welcome Centre.

Kids Help Phone (<https://kidshelpphone.ca/>) is a no-cost national mental health support service accessible 24/7 by phone, text, or chat for children and youth ages four to 29. The service offers mental health promotion and interventions, crisis support, anonymous and confidential

professional counselling, and community resource referrals in both English and French. It also connects users to thousands of local resources through its online Resources Around Me portal. In 2024-25, the Department of Mental Health and Addiction provided \$2 million to support continued operation of Kids Help Phone services for Albertans. Kids Help Phone handled almost 28,000 interactions from Alberta in 2024-25, including nearly 3,100 phone calls, more than 21,200 texts, and about 3,600 chats. The service also provided almost 5,100 referrals to services via phone, text, and chat, and facilitated more than 7,500 connections to community resources through the Resources Around Me portal.

Mental health supports

- The **Mental Health Helpline** (1-877-303-2642) is a 24/7 confidential phone service that provides crisis intervention, support, information, and referrals for Albertans dealing with mental health challenges. The line is jointly operated with the Addiction Helpline (1-866-332-2322) and Alberta Health Link 811, and is staffed by an expert team of psychiatric nurses, psychologists, and referral specialists who ensure callers receive appropriate support and connection to services.
- The **Alberta One-Line for Sexual Violence** is 24/7 service (<https://aasas.ca/get-support> and 1-866-403-8000) that provides emotional support, information and referrals to anyone province-wide who has experienced or been impacted by sexual violence.
- The **Indigenous Support Line** (1-844-944-4744) assists Indigenous people seeking service navigation assistance including cultural support; primary care support; addiction and mental health support; general Indigenous health care support; referrals; and/or patient concerns. Services are available Monday to Friday from 12 p.m. to 8 p.m.
- The **9-8-8 Suicide Crisis Helpline** (<https://988.ca/>) provides emotional support and resource information for individuals dealing with thoughts of suicide or those who are worried about someone else. Services are available by phone or text 24/7.

Counselling Alberta (<https://www.counsellingalberta.com/>), a division of the Calgary Counselling Centre, provides accessible, affordable, outcomes-driven counselling across the province with no waitlist, through online and in-person services. Online counselling via video, phone, or chat is available anywhere in Alberta, reducing barriers to access, especially for those in rural and remote areas where in-person services may be limited or unavailable. In-person counselling, delivered in partnership with local agencies, is offered in nine locations across Alberta: Calgary, Edmonton, Fort McMurray, Red Deer, Lethbridge, Medicine Hat, Grande Prairie, Brooks, and Vegreville. Counselling Alberta ensures consistent, high-quality support by providing clinical oversight, centralized intake, outcomes measurement, and coaching for its partner agencies. In 2024-25, the program added partner agencies in Edmonton, Brooks, and Vegreville, with plans underway to extend in-person counselling to communities including Pincher Creek and Cypress County in the coming year.

In 2024-25, the department provided more than \$6.5 million to support the continued operation and expansion of Counselling Alberta's online and in-person services. More than \$19.7 million is budgeted over three years (2024-25 to 2026-27) for this initiative. Counselling Alberta delivered more than 21,000 counselling sessions to more than 4,200 unique clients in 2024-25, compared to almost 12,300 sessions for more than 2,150 clients in 2023-24. This significant increase—almost 71 per cent in sessions and more than 95 per cent in unique clients—indicates a growing demand for counselling services and reflects the expanded reach and accessibility of services. Reporting from Counselling Alberta indicates that 98 per cent of clients who accessed services in 2024-25 experienced improvement, recovery, or stabilization in their mental health conditions.

Helping Albertans sustain recovery

The Provincial Recovery College Program (<https://alberta.cmha.ca/what-we-do/cmha-in-albertas-projects/alberta-recovery-colleges>) delivered by The Canadian Mental Health Association Alberta Division and Centre for Suicide Prevention, provides free recovery-oriented mental health education to support Albertans pursuing recovery. Recovery Colleges offer in-person and virtual courses that help individuals with mental health or addiction challenges—and their supporters—build peer support skills, resiliency, wellness, connection, and hope. All courses are co-developed and co-facilitated with community members and peers to address local needs, blending lived experience with clinical expertise. In 2024-25, the department provided \$1.2 million to support Recovery Colleges and help improve access to timely, cost-effective, outcomes-driven care.

As of March 31, 2025, seven Recovery Colleges funded under the Provincial Recovery College Program were operational in Camrose, Grande Prairie, Lethbridge, Red Deer, Edmonton, Wood Buffalo, and Medicine Hat. The newest location in Medicine Hat re-opened on March 4, 2025, in partnership with Medicine Hat Family Services. A total of 1,339 courses were offered in 2024-25, more than doubling the number of courses offered the previous year, including 36 new courses, with a more than 10,700 course registrations. Almost 3,300 individuals participated in recovery-focused courses, including more than 2,300 first-time participants, and more than 330 people were referred to Recovery Colleges from other services.

Recovery Colleges continue to meet Albertans' needs and offer value to their communities. According to a 2024-25 Recovery College participant survey, 90 per cent of respondents felt more hopeful about the future after attending a recovery college course, 87 per cent reported increased capacity to manage stress, and 85 per cent reported enhanced connection to an engaged personal/social support network.

Recovery on Campus Alberta (<https://www.recoveryoncampusalberta.ca>) is a province-wide initiative helping to establish peer-led recovery community programs on campuses at each of the 26 publicly funded post-secondary institutions to support students pursuing recovery from addiction or mental health challenges. The goal is to provide virtual support to students in or seeking recovery through addiction and recovery training, outreach, and community-building activities (e.g., awareness campaigns), and recovery-safe events. The Department of Mental Health and Addiction provided nearly \$400,000 in 2024-25 to support the ongoing operations of this initiative. A total of 483 capacity-building information sessions, workshops, and seminars were held reaching more than 8,100 participants Alberta-wide. Program reports indicate 82 per cent of the Recovery on Campus peer support participants found the programming helpful for their recovery journey. As part of this initiative, the University of Calgary launched the UCalgary Recovery Community (UCRC) Hub in early 2024—Alberta's first dedicated peer-driven drop-in space supporting all pathways to recovery, building community, and reducing addiction stigma on campus. Collectively, Recovery on Campus and the UCRC Hub had more than 5,500 total interactions (program attendees and drop-in visitors), hosted more than 375 events, and welcomed 383 campus members to its peer support space in 2024-25.

Another initiative for post-secondary students delivered by Recovery Alberta ensures students in Edmonton and southern Alberta have access to clinical mental health and addiction services as well as seamless linkages into the public health system and back to campus. In 2024-25, Recovery Alberta spent \$1.7 million to support this initiative which served a total of 931 students. About 95 per cent of students who completed follow-up surveys reported satisfaction with the services they received.

The Parents Empowering Parents (PEP) Society empowers families and caregivers of loved ones struggling with substance use or addiction to step into health and recovery. PEP delivers a range of free recovery-oriented services and educational supports for youth, families, and caregivers

including a toll-free family support line, educational webinars and forums, virtual Family Recovery Group meetings, and an in-person Family Recovery Group facilitated by professionals trained in addictions, recovery, and trauma-informed care. Over the past year, a new program for youth aged 15 to 18 called URise was implemented to foster resilience and equip young people with skills for holistic wellness. In 2024-25, the Department of Mental Health and Addiction provided \$500,000 to support ongoing operation of PEP programs and to add capacity and expand the in-person Family Recovery Group to additional communities beginning with Red Deer as the first new community. More than 880 clients interacted with PEP programs and services in 2024-25.

Connecting vulnerable people to mental health and addiction services and supports

Aligned with Alberta's recovery-oriented care approach, Navigation and Support Centres offer coordinated, integrated services involving multiple provincial government ministries and community partners to help ensure vulnerable Albertans have access to necessary supports.

Building on the successful Edmonton model, the Calgary Navigation and Support Centre opened in July 2024, offering access in one convenient downtown location to critical health and social services. The Department of Mental Health and Addiction provides funding to connect people to essential resources they need to stabilize, including immediate opioid agonist treatment via the VODP (see Key Objective 3.5 for additional details on the VODP). Recovery Alberta clinical staff provide on-site mental health and addiction services, including screenings, brief interventions, referrals, opioid use disorder treatment, and harm reduction education.

In 2024-25, on-site services expanded to include health system navigators and trained recovery coaches to support recovery journeys. Since opening and through March 31, 2025, more than 10,000 people accessed a Navigation and Support Centre. Of this total, more than 4,000 people were connected to mental health and addiction services, including more than 500 connections to the VODP.

Recovery Alberta maintains data on individuals receiving mental health and addiction services at the Navigation and Support Centres. This enhances patient continuity and service efficiencies, ensuring clinical staff and recovery coaches at the centres have access to accurate and timely data regarding treatment access and availability, including access to medical withdrawal management services (see Key Objective 1.2 for more information on how the ministry is improving and leveraging data).

The Department of Mental Health and Addiction continues to support people of all ages with eating disorders in their recovery and wellness. The community-based Eating Disorder Support Network of Alberta (EDSNA) provides information, guidance, a community hub, and virtual professionally facilitated support groups for individuals and families affected by eating disorders. EDSNA's support groups are open to anyone in Alberta at low or no cost. In 2024-25, the department provided more than \$320,000 to EDSNA to facilitate these support groups and to deliver public education and awareness activities about eating disorders. EDSNA offered 20 multi-week support groups and 22 multi- and single session groups and workshops last year. More than 230 people with eating disorders, caregivers, health care providers, and members of the public participated.

The Association of Alberta Sexual Assault Services (AASAS) provides leadership, coordination, and collaboration on issues of sexual abuse and sexual assault, ensuring Albertans have access to timely, affordable, and specialized sexual violence counselling services that are based on the most recent research, information, and evidence-based practice. AASAS supports a network of 15 not-for-profit member agencies that provide specialized and distinct core sexual assault services to individuals, families, and rural and urban communities across the province. AASAS also provides oversight and therapeutic standards to these member agencies.

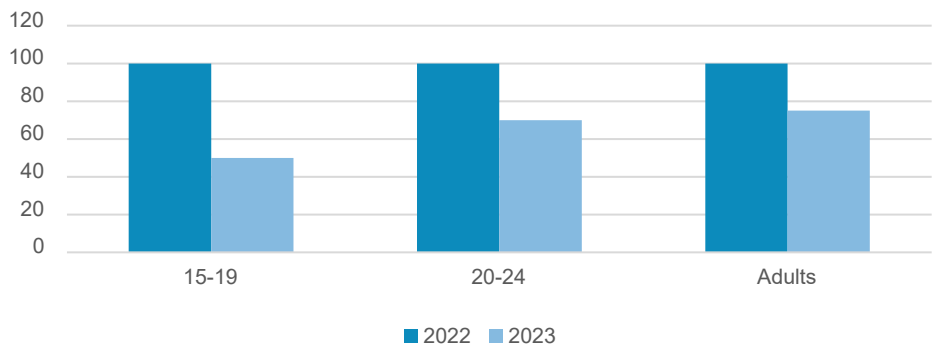
In 2024-25, the Department of Mental Health and Addiction provided nearly \$2.2 million to AASAS to support the delivery of specialized trauma counselling services to those who have experienced sexual violence through 12 of its 14 Sexual Assault Centres. Through these supports, 5,840 people received counselling services, including 3,156 new clients. The AASAS delivered more than 34,300 individual and group counselling sessions throughout the year. Program reports demonstrated 84 per cent of group counselling participants gained awareness of available external supports and resources. In 2024-25, an increase in diversity of clients accessing counselling services was also noted, including more male identifying, Indigenous, transgender and 2SLGBTQIA+ clients. Over the course of the year, AASAS facilitated 41 network-wide Community of Practice meetings to foster a coordinated, collaborative, and evidence-based response to sexual violence.

The Domestic Abuse Response Team education program is delivered by Recovery Alberta and focuses on building health care providers’ capacity across Alberta to effectively identify, screen and refer individuals experiencing domestic violence through standardized, accessible training. In 2024-25, the Department of Mental Health and Addiction provided \$263,000 to support the development of e-learning modules, aimed at improving awareness and screening for domestic violence, family violence, and sexual violence among health care providers. Recovery Alberta is continuing to expand the program’s training scope to include sexual violence, elder abuse, and 2SLGBTQIA+ clients, and offer training in more settings including corrections health, cancer clinics, and primary care locations. Recovery Alberta expects to launch the modules in fall 2025.

The Department of Mental Health and Addiction provides support to The Canadian Mental Health Association Alberta Division and Centre for Suicide Prevention to deliver the Actions Training and Information Services for Albertans project. This project provides suicide prevention training and workshops, including for Indigenous people, educational and research resources, engagement, and coordination of stakeholders for provincial planning, and facilitation of community-based suicide prevention plans including community campaigns. In 2024-25, the department provided \$500,000 to support the project, which trained 637 individuals in suicide prevention. Engagement reached 53 addiction and recovery organizations across Alberta, the Buddy Up Campaign expanded with new skills training and two new organizational champions, and evaluations showed significant increases in participants’ confidence and ability to intervene with those at risk. Moreover, outreach efforts expanded substantially, reaching over 5,500 subscribers through general information mailouts with growing engagement across social media and community networks.

Based on data from the Centre for Suicide Prevention, Alberta saw a 50 per cent decline in suicide rates among youth aged 15 to 19, a 30 per cent decline among those aged 20 to 24, and a 25 per cent decline among adults between 2022 and 2023.

Decrease in Youth and Adult Suicide Rate in Alberta (2022 to 2023)



Source: Centre for Suicide Prevention: Alberta suicide stats by region and age (January 2025)

Enhancing mental health resiliency

The Ministry of Mental Health and Addiction is working to ensure all communities within the province have access to a continuum of mental health supports that build resiliency and meet their unique needs. One way this is accomplished is through direct support to ethnocultural community-based organizations to provide mental health and addiction supports and services within their communities. The Department of Mental Health and Addiction is continuing to partner with several organizations serving ethnocultural communities in Alberta's urban areas, providing more than \$1.6 million in 2024-25 to help ensure program supports, such as counselling, education, and skill building, are inclusive and reflective of the communities they serve. Programming provided by these organizations supports both adult and youth populations. Key actions in 2024-25 included:

- Providing culturally appropriate mental health services and wraparound supports for immigrants and refugees facing multiple barriers to accessing traditional services, and educational sessions to organizations that work with these populations in Edmonton. More than 800 people accessed these services with 80 per cent demonstrating a reduction in the severity of their clinical symptoms of mental illness and 90 per cent indicating they had learned new and effective coping strategies which they were incorporating in their daily lives.
- Delivering therapeutic and psycho-educational mental health and addiction programming for individuals aged 18 and over with concurrent disorders in Calgary's South Asian community. A total of 783 people accessed services.
- Promoting and fostering of positive mental and physical well-being in immigrant and refugee youth ages 12-29 in Edmonton, helping youth increase their mental health awareness; build skills, knowledge, and confidence; explore career choices and pathways; and support personal development. A total of 540 people within 16 ethno-cultural communities accessed these supports and 455 counselling sessions were delivered.
- Providing specialized therapeutic services in Calgary for immigrant and refugee children and their families to build resiliency, including culturally sensitive mental health services focused on addressing traumatic stress and education for community stakeholders. A total of 55 people accessed services, with 70 per cent of participants demonstrating improvement in functioning and reduction in problem severity.

The Rural Mental Health Project (RMHP) is intended to address the mental health needs of rural Albertans by building rural communities' recovery capital and capacity to meet their residents' mental health needs through training, community grants, and partnerships. The program is delivered by The Canadian Mental Health Association Alberta Division and Centre for Suicide Prevention with a goal to foster resiliency by empowering community members to become changemakers in their own communities and leveraging whole-community, evidence-based, and inclusive strategies to build healthy communities. Funding provided through the program supports a sub-grant process, which allows for the development of needs-based action plans for rural communities. In 2024-25, the Department of Mental Health and Addiction provided more than \$1.7 million to support the RMHP, which included funding 41 rural communities from across the province through the sub-grant process, an increase of six communities compared to the prior year. Over the course of the year, the Canadian Mental Health Association Alberta Division and Centre for Suicide Prevention delivered 12 capacity building training sessions through the program, reflecting ongoing commitment to empowering local projects and initiatives.

3.3 Evaluate and expand access to mental health supports for youth in schools and in community to promote positive mental health, prevent severe mental health issues from developing, and provide treatment for students with mental illness.

Expanding access to mental health supports in schools

As part of the Alberta Recovery Model, the province is partnering with CASA Mental Health to expand classroom-based services and the continuum of support for children and youth. CASA Mental Health Classrooms (<https://casamentalhealth.org/getting-help-2/casa-classrooms/>) provide a unique opportunity for students facing complex mental health challenges to receive professional care closer to home without disrupting their education, allowing students to maintain their social lives and stay connected to their peers. The goal of the expansion is to help address psychiatric symptoms early, prevent hospitalization, and support every young person in their pursuit of improved mental health in Alberta.

CASA Mental Health Classrooms are referral-based for students in grades 4 to 12 who:

- Have not responded as expected to previous therapy and continue to have symptoms that impact their home, social and school life.
- Are currently under the care of a physician, nurse practitioner, or psychiatrist.
- Are willing to participate in individual and group therapy with their family.

Students in mental health classrooms have access to a full-time teacher, mental health therapist, mental health aide and other staff who support them in their recovery. Caregivers are also provided with resources to support their child during and after the program. Students from different schools within the community who require mental health supports can be admitted to a mental health classroom. Programming aligns with the school year; students are supported in the classroom for approximately half of the school year, followed by another half-year of tapered treatment and caregiver support as students transition back to their home classroom and care team. The program works closely with 211 Alberta to provide closed loop referrals (client needs were met), connecting students and families to appropriate services available in the area.

The Department of Mental Health and Addiction is investing \$66 million over three years (2023-24 to 2025-26) to bring up to 60 mental health classrooms online that will serve a catchment area of almost 140 communities Alberta-wide. In 2024-25, CASA Mental Health opened 12 new classrooms in nine communities, more than doubling the number of operational classrooms over the previous year. As of March 31, 2025, there are 20 mental health classrooms in operation in 13 communities throughout the province. In 2024-25, more than 500 students received support through a mental health classroom. More than 1,500 students are expected to be served each year once all the mental health classrooms are established.

The Integrated School Support Program (ISSP) is a wellness initiative delivered by the Calgary Police Youth Foundation that targets children and youth aged 5 to 15 at risk of addiction and mental health challenges. The program provides prevention and early intervention wraparound supports such as meal programs, after school care, structured physical education, a positive police presence, and access to mental health professionals to help children and youth building strength and resilience to improve their mental health, academic performance, social and emotional wellness, and quality of life. The program works closely with 211 Alberta to provide closed loop referrals, connecting students and families to appropriate services available in the area. The Department of Mental Health and Addiction invested \$8.5 million over two years (2023-24 and 2024-25) to expand the ISSP. In 2024-25, the program was funded in 11 additional high needs

schools throughout the province. As of March 31, 2025, there are 31⁵ schools in 22 communities across the province offering the ISSP and serving more than 10,000 young Albertans.

In its oversight function, the Department of Mental Health and Addiction continues to support research and evidence-based programming through important work that provides insight into the effectiveness of new mental health and behavioural supports and programs for children and youth that are being implemented across Alberta.

Led by the Mathison Centre for Mental Health Research and Education at the University of Calgary, the school-based mental health supports evaluation project (previously known as the Child and Youth Health Services Initiative) is examining the community level impact of new investments in mental health and behavioural supports in Alberta across four programs: CASA Mental Health Classrooms, ISSP, Kickstand Centres, and 211 closed loop referrals. In 2024-25, the Department of Mental Health and Addiction provided \$1 million for qualitative data collection, completing 10 focus groups involving 72 mental health service providers and youth (aged 9 to 25) served by Kickstand and CASA Mental Health Classrooms. Key performance indicators were developed to assess whether youth mental health services are being delivered accessibly, safely, equitably, and efficiently across diverse community backgrounds and abilities.

The Mathison Centre's preliminary findings highlighted priorities such as improving mental health service delivery and provision, addressing perceptions and attitudes about health care, and investigating barriers to service access. In 2025-26, qualitative research will be conducted to capture experiences from youth, service providers, and caregivers, alongside a literature review to evaluate existing research on the types and suitability of youth mental health services in Alberta. Findings will inform recommendations in the final evaluation report, which will be used to inform decisions and enhance mental health supports and services for children and youth.

Supporting community-based programs and services for children and youth

CASA specialized services offer a suite of referral-based programs designed to promote, preserve and restore the mental health of children and youth. These services support children and youth aged 3 to 17 and their families through flexible service streams that provide access to timely, high-quality mental health care. Families are matched with the appropriate level of service depending on the complexity of mental health challenges. Services are community-based, and therapy incorporates the child's family, school, and community network in treatment. CASA delivers five specialized programs through this stream of mental health services and early intervention supports, leveraging multidisciplinary teams of mental health therapists, nurses, therapy support navigators, and psychiatrists.

In 2024-25, the Department of Mental Health and Addiction provided more than \$5.1 million to support these services. A total of 539 individuals were referred to programs within this service stream in 2024-25. The suite of specialized programs was accessed by 587 unique clients, 309 of which were new. In 2024-25, the Family Therapy Program served 899 individuals (314 families and 586 family members), up from 652 in 2023-24—an increase of 38 per cent. This surpasses the expected increase in CASA's reach (25 per cent) due to the program's service expansion.

The Heroes Resiliency Program (<https://www.impactociety.com/heroes>), delivered by Impact Society for Children & Families in Turmoil, is a trauma informed social-emotional skill-building program for children and youth aged 11 to 14 providing tools and mental health resources to build

⁵ The 2023-24 Mental Health and Addiction Annual Report identified a total of 18 schools as having implemented ISSP as of March 31, 2024. Due to inconsistencies in defining the implementation period, the total presented was incomplete. The issue has since been rectified and the revised total for 2023-24 is 20 schools.

inner strengths, resilience, and decision-making skills for those at risk of or recovering from mental health and addiction challenges. Facilitators such as teachers and counsellors are trained by the Impact Society to deliver the program to students in schools and youth attending community organizations. In 2024-25, the Department of Mental Health and Addiction provided almost \$500,000 to support program operations at 17 schools delivering the program in 13 communities. More than 2,500 children and youth participated in the program in schools and the community over the course of the year. A peer-reviewed longitudinal study published in March 2025 confirms that Heroes significantly strengthens youth resilience, with students experiencing a 45 per cent average increase in resilience scores that were sustained over five months post-program.

The Summit: Marian & Jim Sinneave Centre for Youth Resilience, operated by Recovery Alberta, is a dedicated centre for child and adolescent (under 18 years of age) mental health services located in Calgary. It provides no-cost therapy for children and families; services to manage acute, escalating mental health symptoms; and day hospital services that provide intensive therapy in a community setting. In 2024-25, Recovery Alberta spent \$1.6 million provided by the Department of Mental Health and Addiction to support Day Hospital program operations. More than 4,100 clients accessed services—an increase of more than 16 per cent from the previous year. Follow up appointments included services such as risk assessments and safety planning, social work, and psychiatric consults, and additional family support such as peer support, spiritual care, and

In partnership with Children and Family Services, the Department of Mental Health and Addiction provided \$1.1 million to continue supporting eight **Child Advocacy Centres** for children and youth up to 18 years of age who have experienced sexual abuse or severe physical abuse or neglect and are going through a legal process.

The Trauma Treatment Services (TTS) at Zebra Child Protection Centre offers timely trauma care for children and families affected by severe abuse and builds community capacity to provide these services. In 2024-25, Recovery Alberta spent more than \$336,000 to support the Centre, including therapy and mental health navigation programs. In 2024-25, TTS received 209 referrals, served 199 unique clients, and delivered 880 counselling sessions.

consultations with a pharmacist or dietitian. The average stay for intensive treatment was 13 days and 2.4 per cent of all clients were directed to hospital due to risk and possible admission. An evaluation launched in 2024, using data from more than 6,000 patients, is set for completion in June 2025 to inform future service improvements.

The Youth Community Support Program (YCSP) provides intensive, evidence-based, therapeutic mental health treatment services in the community for youth and families as an alternative to acute or formal environments. Delivered by agencies in Calgary and Edmonton in partnership with Recovery Alberta, services are offered in homes, schools, and in other community settings to reduce barriers for families. In 2024-25, Recovery Alberta spent more than \$4.5 million to continue operation of the program, which served 64 unique clients, including 32 new clients, provided more than 7,600 individual and group counselling sessions, and discharged 31 clients. Discharge occurs when treatment has been completed and youth are ready to step down from the program.

The referral-based Personalized Care in the Community (PCC) program serves high-risk children and youth under 18 who are receiving child intervention services under the *Child, Youth and Family Enhancement Act*. Often, these children and youth have experienced chronic incidences of trauma, substance misuse, and mental health diagnoses, and have significant emotional and behavioural challenges with impairments across several areas of development. The PCC is delivered by Recovery Alberta in community-based settings in Calgary and Edmonton and offers trauma-informed, integrated treatment through 24/7 on-site care, therapy, ongoing assessment, occupational therapy, psychiatry services, and crisis intervention. In 2024-25, Recovery Alberta spent almost \$1.2 million to support two contracted partner agencies to deliver these services;

the Ministry of Children and Family Services provided additional funding to support other components of the program. From April 1, 2024, to September 30, 2024, the program delivered more than 2,600 sessions demonstrating impact by decreasing emergency department visits and inpatient bed days.

The Mental Health Capacity Building (MHCB) initiative, led by Recovery Alberta, promotes positive mental health in children, youth, and families in the communities where they live. Programs are locally planned, coordinated and implemented; most programming serves rural and remote communities. Supports include mental health promotion and prevention, skill-building, and supports for students, families and school staff. As of March 31, 2025, MHCB reached nearly 700 schools in 266 communities across the province, benefitting nearly 200,000 students. In 2024-25, more than 62,600 skill-building sessions and 2,200 community activities were provided to nearly 46,000 attendees. School staff conducted 4,300 sessions for more than 14,500 students, and summer programming served more than 63,000 youth.

3.4 Support initiatives to decrease the number of opioid-related overdoses in the province.

Preventing overdoses

Alberta's government recognizes the importance of providing a continuum of services, including prevention, intervention, treatment, and recovery supports. Services that reduce harm are an important part of this continuum when they are provided in a way that connects people with recovery-oriented services and considers the safety of the community. By funding intervention programs such as drug consumption services, the Provincial Naloxone Program, the Digital Overdose Response System (DORS app), and Rapid Response Teams, Albertans can gain access to other supports and connections to recovery-oriented services, including health care, social services, treatment and recovery.

Naloxone is a drug that can temporarily reverse the effects of an opioid overdose. The Provincial Naloxone Program, delivered by Alberta Health Services, is a province-wide initiative that aims to reduce harm related to psychoactive substance use by distributing injectable naloxone kits through community organizations and pharmacies at no cost to Albertans at risk of experiencing or witnessing an overdose. Training is available wherever kits are provided. As of March 31, 2025, there are 2,654 registered sites in Alberta communities where people can obtain an injectable naloxone kit. Distribution sites include awareness resources for harm reduction programs, health service providers and more. This program is an important part of the Alberta Recovery Model and offers a critical pathway to supports including health care, social services, treatment, and recovery.

Since 2016, more than 1.3 million injectable naloxone kits have been distributed to people living in Alberta. In 2024-25, more than \$7.3 million was provided to supply free injectable naloxone to all people living in Alberta for individual use and more than 300,100 kits were distributed through the Community Based Naloxone Program. Self-reports indicate that naloxone provided through the program reversed 7,555 overdoses in 2024-25 compared to almost 19,500⁶ in 2023-24. The reduction in overdose reversals is consistent with the overall reduction in reported overdoses between the two years. While the number of reported reversals is valuable, it is important to note that the statistic is limited and likely underrepresents the true number of reversals because it is self-reported.

⁶ This figure does not match what was presented in the 2023-24 Mental Health as more complete data on reversals was made available following publication.

Drug consumption services are facilities that offer hygienic spaces where individuals can use illicit drugs in the presence of trained staff who are able to provide timely emergency care in the event of an adverse drug reaction. These services help to reduce drug use in the community and may connect people to health and social services. As part of a recovery-oriented system of care, these sites can serve as a pathway to treatment and recovery; service providers offer referrals for clients to a variety of recovery-oriented services such as medical withdrawal management, addiction treatment and recovery services, social services, and primary or mental health care services. Alberta's drug consumption service providers also provide basic medical/wound care, harm reduction education, supplies such as naloxone kits and sterile drug use equipment, and connections to testing and treatment for infectious diseases such as HIV and Hepatitis C.

Throughout 2024-25, drug consumption service sites operated in Calgary, Lethbridge, Red Deer, Edmonton (three sites), and Grande Prairie. On March 31, 2025, the drug consumption service in Red Deer was closed permanently with new recovery-oriented and targeted services initiated to help support the population previously served through the site. In 2024-25, Recovery Alberta spent more than \$17.8 million to support the operation of drug consumption services. The supports help to manage adverse drug reactions and divert patients away from emergency departments and reduce the burden on other emergency care including EMS. In 2024-25, drug consumption service facilities were accessed more than 161,800 times with an average of nearly 1,800 unique individuals using drug consumption services per month.

As part of their licensing requirements under the Mental Health Services Protection Regulation, drug consumption service providers must publish good neighbour commitments that reflect their promise to understand and respond to concerns raised by community members and specifically set out a commitment to address any issues in the area surrounding the facility (e.g., discarded drug consumption supplies).

Alberta's government is continuing to support initiatives aimed at preventing drug overdoses and reducing deaths due to addiction. In the immediate area around the Calgary Drop-in Centre's Riverfront Shelter and the Red Deer Safe Harbour Society's Emergency Shelter, a rapid response model is being employed to manage adverse drug reactions and divert clients away from other urgent care services, like EMS and emergency departments. In both locations, Rapid Response Teams of paramedics, nurses, and shelter staff respond to incidents of confirmed or suspected overdose and initiate immediate clinical interventions to address medical emergencies quickly and safely. Teams patrol areas around these shelter locations and monitor designated high observation areas within the facilities where individuals at high risk of overdose are concentrated. In 2024-25, the department provided more than \$2.2 million to support Rapid Response Teams in both the Calgary Drop-in Centre and the Safe Harbour Society emergency shelter.

In Calgary, the Rapid Response Team operates 24/7 and 2024-25 marked the first year of its operations at full capacity. The services provided by the team continued to demonstrate a clear diversion of clients from emergency care, with EMS responding to 20 per cent of the calls for drug overdose events compare to response rates of 100 per cent before the team was installed. Through March 31, 2025, the team:

- Responded to 807 drug overdoses at the Calgary Drop-in Centre.
- Connected 676 individuals to on-site medical supports, including medical withdrawal management.
- Connected 528 individuals to a navigator or recovery coach to access wraparound supports.
- Made 276 referrals to housing or other social services.

Diversion from emergency departments demonstrates success in addiction care since it indicates that individuals experiencing addiction-related emergencies are receiving timely and targeted care that is better suited to their specific needs, helping reduce unnecessary strain on EMS and hospital resources. By investing in initiatives such as Rapid Response Teams, Alberta's government aims not only to provide immediate overdose intervention but also connect individuals promptly to appropriate services like the Rapid Access Addiction Medicine clinic and the Acute Care Addiction Recovery Program (see key Objective 3.5 for program details). The reduction in EMS and emergency department use following the introduction of these teams was an anticipated outcome, and affirms the effectiveness of targeted, integrated addiction interventions. This holistic approach ensures efficient use of health care resources, improves recovery outcomes, and reflects prudent investment of public dollars.

The Red Deer Rapid Response Team began operations in December 2024 for 12 hours per day, seven days a week. Services were gradually scaled up through the first part of 2025, and as of March 31, 2025, the team was operating 24 hours per day, seven days a week. This initiative is part of a coordinated suite of mental health and addiction services for a target population in Red Deer who are unhoused, may be using the homeless shelter or who are living in encampments, and are struggling with addiction.

Rapid Response Teams also coordinate with on-site mental health, addiction, and primary care staff to connect individuals to services, such as basic health care and medical withdrawal management, as well as referrals and peer support to help clients access other recovery-oriented wraparound supports within the continuum like mental health services, housing, and community-based addiction treatment. In 2024-25, the Department of Mental Health and Addiction provided \$1.2 million to the Calgary Drop-in Centre to support individuals with complex needs who face barriers to accessing housing and primary care in the community.

The Digital Overdose Response System (DORS) is a free mobile application (app) that can be used to prevent overdosing and save lives. Through this confidential app, Albertans who use opioids or other substances alone can summon emergency response to their location if they become unconscious. The DORS app coordinates emergency responses based on information provided such as person's location, phone number and any unique access instructions. Information collected is limited to what is required to support an individual in the event they require assistance. The app has a built-in timer that provides an alert to check-in at various intervals to ensure safety; a session can be cancelled or extended at any time. Individuals who do not respond to the timer receive a call from the STARS emergency response centre. If STARS is unable to contact an individual or there is reason to believe they may be experiencing an overdose, EMS is dispatched.

Beyond coordinating emergency responses, the app provides other information on national and provincial addiction recovery supports and services, such as the Addiction Helpline, Virtual Opioid Dependency Program, and opioid agonist treatment, ensuring people have the information they need to move forward in their pursuit of recovery.

In 2024-25, the Department of Mental Health and Addiction provided \$186,000 to support the continued operation of DORS. As of March 31, 2025, the app had been installed almost 5,300 times, with more than 3,100 registered DORS users. In 2024-25, a total of 429 new users registered for the service in 2024-25, which is a decrease of 65 per cent compared to the number of new registered users in 2023-24 (1,230 new users). A total of 700 sessions were initiated in 2024-25 compared to 1,673 in 2023-24, which represents a decrease of almost 58 per cent. There were 64 emergency response escalations to STARS compared to 87 the year prior and 11 emergency response escalations to EMS compared to 19 the year prior, representing a decrease of nearly 26 per cent and nearly 42 per cent, respectively. This decrease may be attributed to reduction in overall substance use but could also indicate a need for a new public awareness campaign.

3.5 Support the delivery of addiction treatment services throughout Alberta to provide safe, variable supports for individuals to pursue recovery from addiction.

Providing accessible, low barrier addiction treatment

The Virtual Opioid Dependency Program (VODP) (<https://vodp.ca>), delivered by Recovery Alberta, ensures accessibility to opioid use disorder treatment, offering same-day opioid agonist treatment medication without a waitlist, and supports transition planning to local community providers and recovery supports. Assessment and treatment are provided virtually, and when indicated, prescriptions for opioid agonist treatment medications are sent to any qualified pharmacist in the Albertan’s local area. In 2024-25, VODP piloted an increase of its hours of operation to 24/7 with frontline clinical support staffed for 18 hours. Following a review of service use, in 2025-26, VODP will return operations to 12 hours a day, seven days a week operation and focus on enhancing services provided during peak service periods.

VODP is an innovative program with far-reaching and meaningful impact on the successful advancement of the Alberta Recovery Model, such as diverting individuals away from emergency departments or the correctional system and into a recovery-oriented system of care. VODP is offered in a variety of settings to help people initiate or maintain on opioid agonist treatment, including in police detainment units, corrections facilities, and Navigation and Support Centres (see Key Objectives 1.4 and 3.2 for details of these programs). The recovery care pathway for many persons accessing VODP includes the transition of care to a local primary care provider to maintain treatment continuity in their home community. As part of the VODP, transitional services are available seven days a week to help ensure smooth transitions for clients who are moving from one care setting to another. The Transitional Service Team employs a significant social work complement to provide expertise around resources for opioid care.

In 2024-25, Recovery Alberta spent \$9.6 million to support the continued operation and expansion of the VODP. As of March 31, 2025, the program had supported clients in more than 400 Alberta communities.

2024-25 VODP Overview for Treated Clients

Indicator	Result
Referrals to VODP	8,934
Admissions	6,292
Clients receiving same day access to opioid agonist treatment	3,711
Active clients	6,276
Discharges	6,045
Clients supported by VODP Transition Team	1,157
Incoming calls to VODP	34,923

The Youth VODP program is available to young Albertans with opioid use issues in community group care, provincial campus-based care, and *Protection of Children Abusing Drugs Act* facilities. The program has the capacity to serve up to 120 youth each year and is primarily focused on youth who are involved with child intervention services. Children and Family Services supports delivery

of the program in partnership with Recovery Alberta and the Department of Mental Health and Addiction. The program is a youth-directed and offers same-day opioid agonist treatment medication without a waitlist. Youth can choose to access the program with their caseworkers, provincial campus-based care, or community agency staff.

Through funding from the Partnership for Research and Innovation in the Health System program, Recovery Alberta is working with First Nations and Métis communities across Alberta to expand and evaluate tailored partnerships with the VODP, assessing the implementation, effectiveness, and client and health care provider experience with the program. It is anticipated that participation in this project will allow for the implementation of the program in a way that is aligned with each community's values and will increase access to services with improved health for community members. Partnership building is ongoing and institutional agreements are in progress, aligning with rolling Community recruitment and onboarding. Data collection in the first Community began in April 2025.

The Virtual Rapid Access Addiction Medicine program (V-RAAM), operated by Recovery Alberta, provides virtual addiction medicine physician consultations across Alberta for behavioural addictions and non-opioid substance use disorders such as alcohol, stimulants, and gambling disorders. The V-RAAM formally launched January 1, 2025, and as of March 31, 2025, there were 106 clients admitted to the program and three clients transitioned from VODP to V-RAAM. Overall, there were 173 referrals to V-RAAM for treatment, and 535 incoming calls. The number of referrals reflects that many clients accessing V-RAAM services are referred more than once prior to making the commitment to engage in treatment. As of March 31, 2025, V-RAAM had supported 109 unique active clients. Services will be sustained in 2025-26 by transitioning V-RAAM under the purview of the VODP.

The Red Deer Mobile Rapid Access Addiction Medicine (M-RAAM) service, operated by Recovery Alberta, launched in December 2024 in response to changing community needs. Located near the Safe Harbour Shelter, M-RAAM offers low-barrier, walk-in addiction treatment for anyone in the community, including unhoused individuals, or those using the shelter or living in encampments. It provides immediate access to care for substance use and behavioural addictions and provides on-site counselling, comprehensive screening, primary health care assessments, harm reduction education and overdose prevention, referrals, and connections to community services.

M-RAAM integrates closely with Recovery Alberta broader service network—including virtual and in-person addiction medicine services—to help ensure seamless and effective transitions across treatment pathways. Since its inception, the program expanded from three hours/five days a week to 12 hours/seven days a week (7AM to 7PM). In 2024-25, Recovery Alberta spent almost \$200,000 to implement this program and more than \$1.2 million is budgeted annually beginning in 2025-26 to sustain full operations.

The Calgary Rapid Access Addiction Medicine (RAAM) clinic supports the need for integrated and comprehensive services by supporting seamless transitions through case management and referrals to needed services. The clinic provides a 'one stop shop' for addiction treatment and assessment for a variety of substance use disorders, behavioural addictions, and concurrently diagnosed mental health issues that are typically subdivided into separate clinics or specialty programs or providers.

The Acute Care Addiction Recovery Program (ACARP), delivered by Recovery Alberta, is a comprehensive and coordinated approach to address a critical gap in access to addiction medicine physician consultation and on-site health care professionals for the treatment of addictive disorders within acute care settings. Throughout 2024-25, services were operational within the Royal Alexandra Hospital (RAH) in Edmonton and Peter Lougheed Centre (PLC) in Calgary.

ACARP provides rapid access to recovery pathways for acute care patients at risk of substance related harms, including death by opioid overdose. ACARP increases initiation into treatment and provides additional resources to emergency departments, where there are high volumes of individuals presenting with addiction related concerns. Services include:

- Immediate initiation of evidence informed pharmacotherapy (e.g., opioid agonist treatment) for treatment of addiction disorders, including alcohol, opioids, nicotine, and stimulants.
- Comprehensive, evidence-based addiction management, including treatment recommendations for complicated intoxication and withdrawal.
- Resources to prevent opioid overdose and harm reduction education.
- Psychosocial supports, such as identification and referral for associated mental health conditions, and social supports.
- Referral to addiction treatment services and wraparound supports.

In 2024-25, Recovery Alberta spent almost \$5 million to sustain ACARP services at the PLC, RAH, and the Calgary RAAM clinic. ACARP at the PLC and RAH completed a combined 3,307 consultations, served a total of 2,531 unique patients, and supported 1,881 patient referrals to outpatient services in 2024-25, while the Calgary RAAM clinic had 15,551 client visits and served almost 2,800 unique clients. Over the coming year, ACARP services are expected to be expanded to the Foothills Medical Centre, the Rockyview General Hospital, and the University of Alberta Hospital. Service expansion will be achieved through staff redistribution to the new sites with the annual operational cost being unchanged.

Alberta continues moving forward with the Alberta Recovery Model in response to the addiction crisis, including providing accessible, low-barrier, person-centred treatment options for Albertans seeking recovery from opioid use issues. Opioid Dependency Program (ODP) clinics assist people living with opioid use disorder by providing evidence-informed opioid agonist treatment medications such as methadone, buprenorphine/naloxone, buprenorphine extended release, and slow-release oral morphine, and offer allied health services including psychosocial support, addiction counselling, and referrals to other community-based services. In 2024-25, Recovery Alberta spent almost \$15.5 million to support ODP clinics which provided care to 12,017 unique clients throughout the year. As of March 31, 2025, Recovery Alberta-operated ODP clinics were operating across the province in Bonnyville, Calgary, Cardston, Edmonton (five sites), Fort McMurray, Grande Prairie, High Prairie, Lethbridge, Medicine Hat, Red Deer, and the Virtual Opioid Dependency Program (VODP).

Evidence-informed pharmacotherapy (e.g., opioid agonist treatment medication) is a key factor in successfully supporting recovery pathways. Addiction is a chronic yet treatable medical condition. Just like diabetes where an individual will require ongoing medication, some individuals living with opioid use disorder may require opioid agonist treatment medication indefinitely.

Narcotic Transition Services (NTS) are highly specialized, evidence-informed addiction treatment services intended for people with severe, treatment-resistant opioid use disorder (e.g., severe fentanyl dependency/withdrawal) who have been unable to stabilize on conventional opioid agonist treatment medications. NTS is available major centres across Alberta, helping to first stabilize patients with designated narcotic drugs, such as hydromorphone, up to three times per day and then transition them to evidenced-informed opioid agonist treatment medication options under expert medical care and comprehensive psychosocial supports. As of March 31, 2025, NTS is available in ODP clinics in Calgary, Edmonton, Red Deer, Lethbridge, Grande Prairie, and Medicine Hat. In 2024-25, there were 178 active clients in the NTS program and more than 35,700 total visits from NTS clients.

Performance Indicator 3.a:**Change in recovery capital scores of Albertans accessing publicly funded treatment (per cent)**

Recovery Capital Tool	Prior Years' Results				
	2020-21	2021-22	2022-23	2023-24	2024-25
Recovery Capital Index (RCI)	N/A	N/A	N/A	43%	36%
Assessment of Recovery Capital (ARC)	N/A	N/A	N/A	43%	38%

Source: MRP System

Notes: The MRP system rollout began in July 2022, and as such, there are no historical full fiscal year results to report prior to 2023-24. Results reported for 2022-23 in the 2024-27 Mental Health and Addiction Business Plan included data from July 2022 to December 2023. The result has been recalculated to align with fiscal year presentation.

Description: This indicator reports on the per cent change in individual recovery capital scores from admission to discharge or most recent assessment of Albertans receiving addiction treatment services in publicly funded facilities where the MRP platform has been implemented. The RCI and ARC scores are calculated using items from the Recovery Capital (REC-CAP) questionnaire, which is facilitated by a health care professional. The REC-CAP questionnaire includes 120 questions on areas such as substance use, housing, legal issues, social supports and service needs, which touch all domains of recovery capital. The ARC scale is a subset of 50 items from the REC-CAP questionnaire representing social and personal recovery capital.

Rationale: Increases in recovery capital are associated with positive recovery outcomes for individuals experiencing addiction issues. Recovery from addiction is among the most important, ultimate goals of treatment for providers, family, clients and society. Improvements in this indicator are intended to reflect improvements in quality and person-centred focus of addiction programs and services.

Results and variance explanation: As of March 31, 2025, MRP had been implemented in 39 medical withdrawal management, residential addiction treatment and recovery sites in Alberta. In 2024-25, 1,933 clients had eligible records, meaning at least two recorded assessments. For those individuals, the mean percentage change in RCI from admission to discharge or most recent assessment was 36 per cent. Although the RCI for 2024-25 is seven percentage points lower compared to 2023-24, the number of individuals included in the analysis in 2024-25 is close to three times the number for last year and this variance was anticipated. The change of 36 per cent in the RCI is an indication of improvement of recovery capital with treatment in the community. The ministry will continue to work with Recovery Alberta to identify opportunities to improve the recovery capital questionnaire or develop a shorter assessment tool for use in low barrier services (e.g., with clients accessing withdrawal management services).

Performance Measure and Indicator Methodology

Performance Measure 1.a

Mental health and addiction-related emergency department visits with no interaction with publicly funded mental health and addiction services in the previous two fiscal years (per cent)

Methodology

The information needed to generate this performance measure is hosted (including cleaning and maintenance) in the Business Intelligence Environment in the Ministry of Health. The Strategic Information, Research and Evaluation Unit in the Ministry of Mental Health and Addiction accesses, extracts and analyzes the needed data to produce the results for this measure. All data extractions and analyses are done using the Statistical Analysis Software data analytics tool. The result for this measure is generated by following these steps:

- Datasets are derived by processing and merging the physician claims data, emergency department visit data, hospitalization data, Pharmaceutical Information Network data for mental health or addiction-related prescription drug dispensation, and population registry data for each fiscal year. This process allows for identification of the unique patients who have “used” and “not used” any publicly funded mental health and addiction services.
- The numerator for the respective fiscal year is determined by the number of individuals presenting in the emergency department for mental health or addiction-related concerns who have not used any mental health and addiction-related services in the last two years.
- The denominator is derived by summing the numbers of individuals presenting in the emergency department for mental health or addiction-related concerns who have “used” and “not used” the mental health and addiction related services in last two years.
- The result is obtained using the following formula:

$$\text{Percentage} = \frac{\text{Number of individuals presenting to an emergency department for addiction or mental health concerns who did not use publicly funded mental health and addiction services in the past two years of the reference year}}{\text{Number of individuals presenting to an emergency department for addiction and mental health concerns in a given fiscal year}} \times 100$$

Source

Alberta Health Business Intelligence Environment: National Ambulatory Care Reporting System; Discharge Abstract Database; Practitioners Claim Database under Alberta Health Care Insurance Plan; Pharmaceutical Information Network; Population Registry Files; and Alberta Health Postal Code Translator File.

Performance Indicator 1.b:**Albertans receiving care at an appropriate level (under development)**

This indicator remained under development in 2024-25 and has been replaced in future Mental Health and Addiction business plans.

Data for the preliminary results is extracted from the following sources:

- Consolidated Addiction and Mental Health Database, Alberta Health Services; and
- Discharge Abstract Data Alberta Hospital Discharge Abstract Database (DAD), Alberta Health.

Data updates may occur in the administrative sources referenced for the preliminary results; as such, results are subject to change.

Performance Indicator 2.a:**Recovery-supportive community scores for selected Alberta communities (under development)**

This indicator was not developed in 2024-25 and has been replaced in future Mental Health and Addiction business plans.

Performance Indicator 3.a:**Change in recovery capital scores of Albertans accessing publicly funded treatment (per cent)****Methodology**

The raw data needed to generate this performance indicator is provided to Alberta Health Services weekly from Last Door Recovery Society – the vendor supplying the MRP system. The data is hosted in an Alberta Health Services data warehouse and Alberta Health Services conducts the required analysis for this indicator.

MRP records are initiated for consenting clients at the time of admission into a publicly funded facility that has implemented MRP. Data is entered into the REC-CAP tool via clinical interviews by health care professionals in residential addiction treatment facilities, recovery communities and medical withdrawal management settings. Some self-entry by clients is also available. The REC-CAP questionnaire includes 120 questions on areas such as substance use, housing, legal issues, social supports and service needs, which touch all domains of recovery capital (see page 10 for a complete listing of the domains). This assessment includes both positive recovery capital items that support recovery and negative recovery capital items that do not support recovery. The ARC scale is a subset of 50 items from the broader REC-CAP questionnaire representing social and personal recovery capital.

All records for clients with at least two assessments are included:

- first assessment – usually upon admission; and
- most recent assessment – usually upon discharge.

ARC scale values are summed for each record and the average is taken across records. Box plots are produced for distributions of scores for each assessment to ensure they are suitable for the intended analyses. Means for each assessment are produced and mean per cent change is calculated along with paired t-tests and 95 per cent confidence intervals for statistical significance.

The ARC numerator is determined by subtracting the mean score for the first assessment (MT1) from the mean score for the most recent assessment (MT2) for the 50 items included in the scale. The ARC denominator is the mean score for the first assessment (MT1). The mean percentage change is calculated using the following formula, where M is the mean, T1 is the first assessment, and T2 is the most recent assessment:

$$\text{Percentage Change} = \frac{\text{MT2} - \text{MT1}}{\text{MT1}} \times 100$$

Values for the remaining items from the REC-CAP questionnaire are combined with the ARC items to produce the RCI. The numerator is determined by subtracting the overall RCI score for first assessment (MT1+100) from the overall RCI score for the most recent assessment (MT2+100). The RCI denominator is the overall RCI score for the first assessment (MT1+100). The scales are normalized, and the percentage change is calculated using the following formula, where M is the mean, T1 is the first assessment, and T2 is the most recent assessment:

$$\text{Percentage Change} = \frac{((\text{MT2} + 100) - (\text{MT1} + 100))}{\text{MT1} + 100} \times 100$$

Data from medical withdrawal management settings are excluded. Clients are often unwell and in distress and unable to complete the assessment at intake. In addition, stays in medical withdrawal management settings are often brief with not enough time to significantly impact recovery capital.

Source

Last Door Recovery Society, My Recovery Plan system database

Financial Information

Table of Contents

Reporting Entity and Method Consolidation.....	68
Ministry Financial Highlights.....	69
Statement of Revenues and Expenses (unaudited)	69
Revenue and Expense Highlights	70
Expenses – Directly Incurred Detailed by Object (unaudited)	74
Financial Statements of Other Reporting Entities	76
Recovery Alberta.....	77
Canadian Centre of Recovery Excellence	109
Financial Information	128
Lapse/Encumbrance (unaudited)	129
Deferred Contributions (unaudited)	130

Reporting Entity and Method Consolidation

The consolidated ministry financial information is prepared in accordance with government's stated accounting policies, which are based on Canadian Public Sector Accounting Standards.

The reporting entity is the Ministry of Mental Health and Addiction for which the Minister of Mental Health and Addiction is accountable. The accounts of the Ministry of Mental Health and Addiction, which includes the Department of Mental Health and Addiction, Recovery Alberta (RA), and the Canadian Centre of Recovery Excellence (CoRE), are consolidated using the line-by-line method. The audited financial statements of RA and CoRE are included in the annual report. RA's financial statements have been restated to reflect a full year of operations, as required by the public sector accounting standards.

Under the line-by-line method, accounting policies of the consolidated entities are adjusted to conform to those of the government and the results of each line item in their financial statements (revenue, expense, assets, and liabilities) are included in government's results. Revenue and expense, capital, investing and financing transactions, and related asset and liability balances between the consolidated entities have been eliminated.

A list of the individual entities making up the ministry are shown on the "Management's Responsibility for Reporting" statement included in this annual report.

Ministry Financial Highlights

Statement of Revenues and Expenses (unaudited)

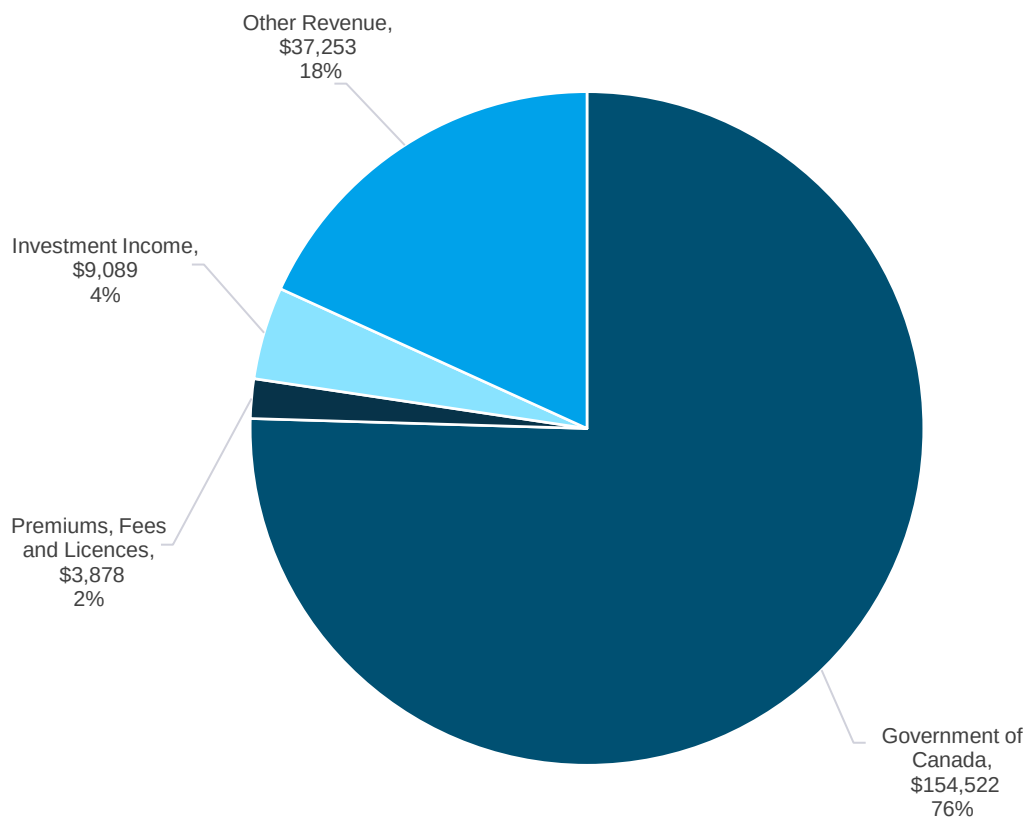
Year ended March 31, 2025

(in thousands)

	2025		2024		Change from	
	Budget Restated	Actual	Actual Restated		Budget	2024 Actual
Revenues						
Government Transfers						
Government of Canada	\$ 168,992	\$ 154,522	\$ 116,869	\$ (14,470)	\$ 37,653	
Premiums, Fees and Licences	5,977	3,878	5,805	(2,099)	(1,927)	
Investment Income	-	9,089	10,612	9,089	(1,523)	
Other Revenue	21,868	37,253	31,284	15,385	5,969	
Ministry Total	196,837	204,742	164,570	7,905	40,172	
Inter-Ministry Consolidation Adjustments	(11,815)	(12,348)	(15,900)	(533)	3,552	
Adjusted Ministry Total	185,022	192,394	148,670	7,372	43,724	
Expenses - Directly Incurred						
Ministry Support Services	13,453	13,592	6,435	139	7,157	
Mental Health and Addiction	305,878	221,866	192,184	(84,012)	29,682	
Recovery Alberta	1,393,500	1,492,151	1,398,372	98,651	93,779	
Canadian Centre of Recovery Excellence	4,300	2,217	-	(2,083)	2,217	
Ministry Total	1,717,131	1,729,826	1,596,991	12,695	132,835	
Inter-Ministry Consolidation Adjustments	(17,521)	(24,285)	(33,541)	(6,764)	9,256	
Adjusted Ministry Total	1,699,610	1,705,541	1,563,450	5,931	142,091	
Adjusted Annual Deficit	\$(1,514,588)	\$(1,513,147)	\$(1,414,780)	\$ 1,441	\$ (98,367)	

Revenue and Expense Highlights

Consolidated Revenues (Prior to Inter-Ministry Consolidation)



Total actual revenue for the year was \$204.7 million, which was \$7.9 million higher than budget and an increase of \$40.1 million over the prior year.

Total revenue was \$7.9 million higher than budget, mainly due to:

- Reallocation of other income from Alberta Health to the ministry for the restatement of Recovery Alberta (\$10.0 million).
- Higher interest and dividend income in Recovery Alberta from stronger market performance than budget (\$9.0 million).
- Refunds of expenditures not budgeted (\$1.2 million), higher donations and contributions (\$1.7 million), and higher external recoveries from the Workers' Compensation Board (WCB) and other miscellaneous revenue (\$2.4 million).

This was partially offset by:

- Lower federal government transfers due to the timing of revenue recognition for bilateral projects, primarily delays in Recovery Community builds (\$14.4 million).
- Lower out-of-province billings in Recovery Alberta compared to budget (\$2.0 million).

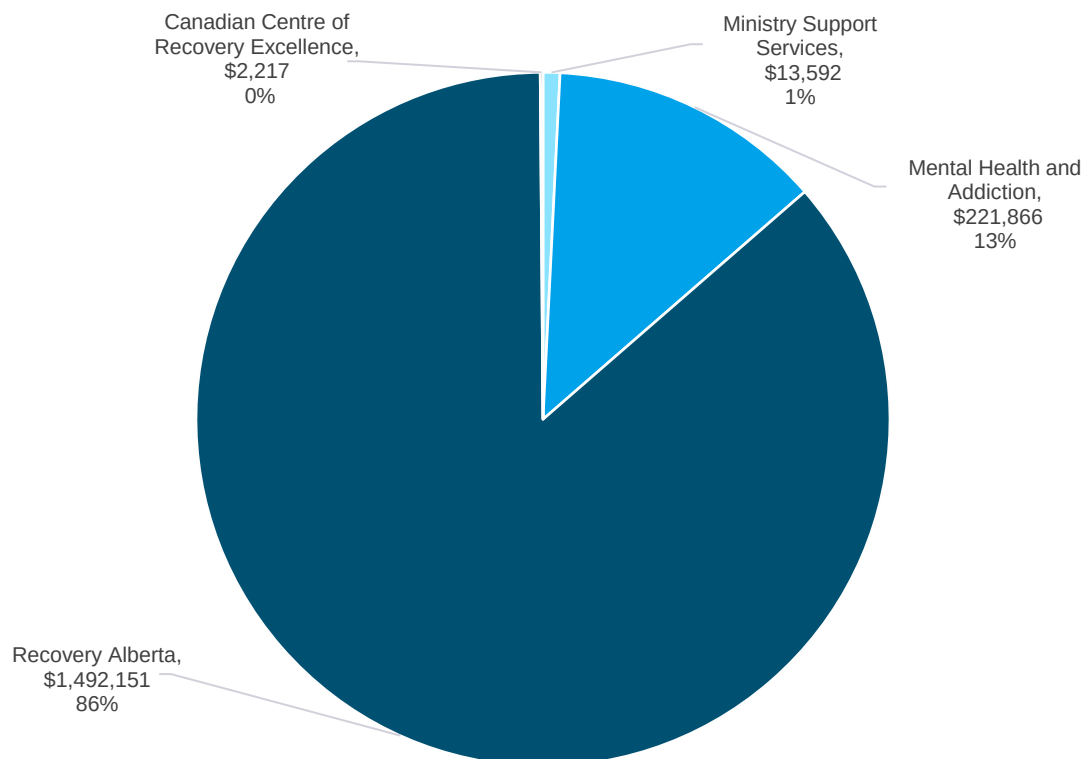
The \$40.1 million increase from the prior year was primarily due to:

- \$37.6 million funding increase provided through the Canada Health Transfer agreement.
- \$10.0 million reallocation of other income from Alberta Health to the ministry for the restatement of Recovery Alberta.

This was partially offset by:

- \$3.5 million decrease in interest income and health fees and charges.
- \$2.7 million decrease in unused restricted grant funding returned from grant recipients.
- \$1.0 million decrease in funding received from other Government of Alberta entities.

Consolidated Expenses (Prior to Inter-Ministry Consolidation)



Total actual expense for the year was \$1,729.8 million, which was \$12.7 million higher than budget and an increase of \$132.8 million from the prior year.

Total expense was \$12.7 million higher than budget, mainly due to:

- \$98.6 million increase in Recovery Alberta due to collective bargaining, increased demand for services driven by population growth, increased resources at correctional facilities, and general inflation.

This was partially offset by:

- Reprofiting of capital grants for project delays in Indigenous-led Recovery Community builds and deferral of CASA House payments (\$44 million).
- Reallocations of grant funding to reflect the changing needs in the community, through grant recipient prior year surpluses and uncommitted grant funding (\$40 million).
- Delays in staffing and project initiation at the Canadian Centre of Recovery Excellence (\$2.0 million).

The \$132.8 million increase from prior year was primarily due to:

- \$58.7 million in Community Care, related to the impact of collective bargaining increases, expansion of crisis teams in partnership with the Calgary and Edmonton Police service, and increased capacity for opioid use recovery and eating disorder treatment programs.

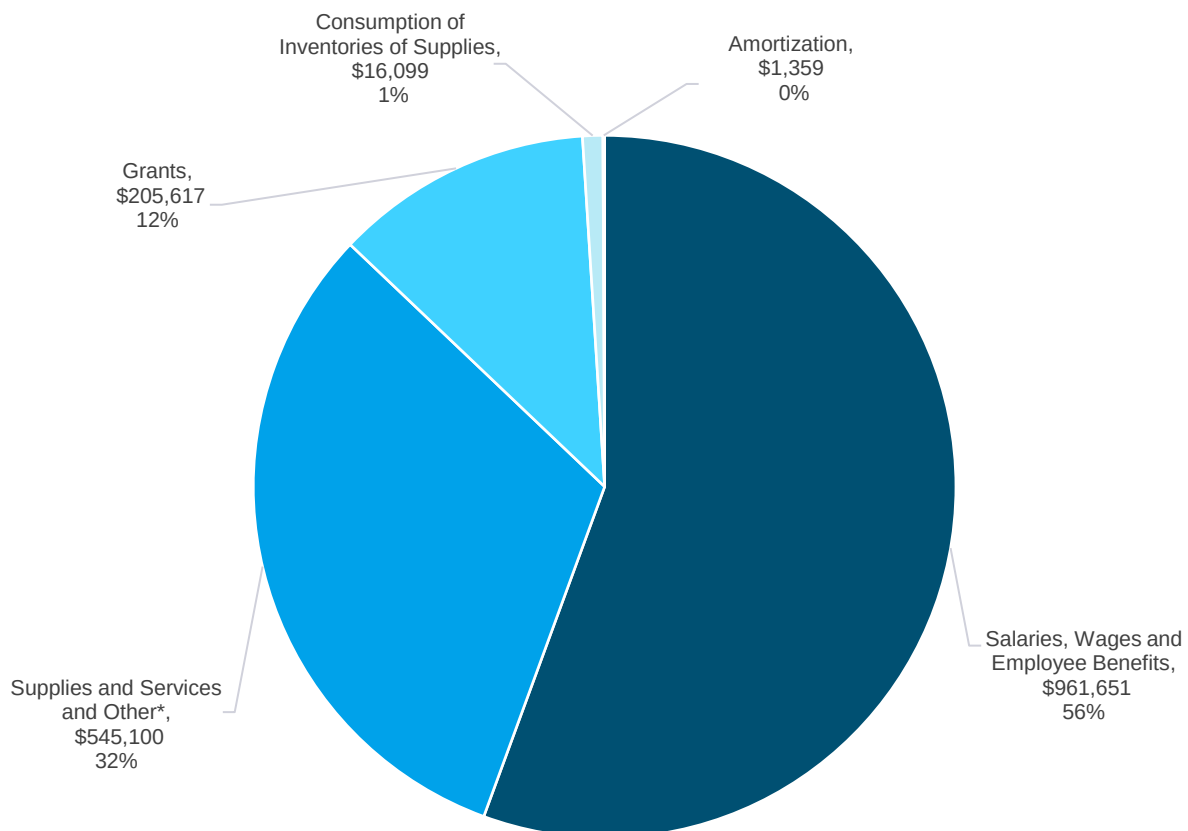
-
- \$22.0 million in Acute Care, due to collective bargaining impacts and higher patient acuity costs, particularly from Constant Care usage.
 - \$13.3 million in Diagnostic and Therapeutic Services and Administration, also related to collective bargaining increases.
 - \$36.7 million related to additional staff hires and increased capital funding within the Department.
 - \$2.2 million related to the operations of the Canadian Centre of Recovery Excellence, which was established in June 2024.

Expenses – Directly Incurred Detailed by Object (unaudited)

	2025		2024
	Budget Restated	Actual	Actual Restated
Expense by object	<i>In thousands</i>		
Salaries, Wages, and Employee Benefits	\$ 861,624	\$ 961,651	\$ 1,031,732
Supplies and Services	548,363	\$ 545,100	371,574
Grants	290,678	205,617	177,647
Consumption of Inventories of Supplies	16,404	\$ 16,099	13,989
Amortization	-	\$ 1,359	1,549
Other ⁽¹⁾	62	-	500
Total ministry expenses	\$ 1,717,131	\$ 1,729,826	\$ 1,596,991
Inter-Ministry Consolidation Adjustments	(17,521)	(24,285)	(33,541)
Adjusted total ministry expenses	\$ 1,699,610	\$ 1,705,541	\$ 1,563,450

⁽¹⁾ Includes financial transactions and other expenses of \$nil in 2024-25 (\$500 in 2023-24).

2025 Actual (unaudited)



The ministry's expenses were primarily for salaries, wages and employee benefits, supplies and services, and grants, which together accounted for 99 per cent of total expenses.

- Salaries, wages and employee benefits were \$961.6 million or 56 per cent of total expenses, supporting delivery of mental health and addiction services across the province.
- Supplies and services were \$545.1 million or 32 per cent of total expenses, mainly attributed to contracts with health service providers and various purchased supplies and services.
- Grants were \$205.6 million or 12 per cent of total expenses, primarily supporting Treatment and Recovery, Initiatives that Reduce Harm, and Children and Youth programs. Other grant expenses included restricted funding to support organizations and communities through various programs or initiatives.
- Consumption of inventory (\$16.0 million) and amortization of tangible capital assets (\$1.3 million) comprised the balance of the ministry's total expenses.

Financial Statements of Other Reporting Entities

Table of Contents

Index of Audited Financial Statements

Recovery Alberta..... 77

Canadian Centre of Recovery Excellence109

Recovery Alberta Financial Statements



Table of Contents

Management's Responsibility for Financial Reporting.....	78
Independent Auditor's Report	79
Statement of Operations	82
Statement of Financial Position	83
Statement of Change in Net Financial Assets	84
Statement of Cash Flows.....	85
Notes to the Financial Statements	86
Schedule 1: Expenses – Detailed by Object.....	104
Schedule 2: Remuneration and Benefits	105
Schedule 2A – Board Remuneration	105
Schedule 2B – Executive Remuneration and Benefits	105
Schedule 2C – Executive Supplemental Pension Plan	106
Footnotes to the Schedules of Remuneration and Benefits	107
Schedule 3: Segment Disclosures.....	108

Management’s Responsibility for Financial Reporting

The accompanying financial statements for the nine months ended March 31, 2025 are the responsibility of management and have been reviewed and approved by senior management. The financial statements were prepared in accordance with Canadian Public Sector Accounting Standards and include certain disclosures required by the financial directives issued by the Ministry of Mental Health and Addiction, and of necessity include some amounts based on estimates and judgement.

To discharge its responsibility for the integrity and objectivity of financial reporting, management maintains systems of financial management and internal control, which give consideration to costs, benefits and risks that are designed to:

- provide reasonable assurance that transactions are properly authorized, executed in accordance with prescribed legislation and regulations, and properly recorded so as to maintain accountability of public funds;
- safeguard the assets of the “Province of Alberta” that are the responsibility of Recovery Alberta.

Recovery Alberta carries out its responsibility for the financial statements through the Board. The Board meets with management and the Auditor General of Alberta to review financial matters, and approves the financial statements upon finalization of the audit. The Auditor General of Alberta has free access to the Board.

The Auditor General of Alberta provides an independent audit of the financial statements. His examination is conducted in accordance with Canadian Generally Accepted Auditing Standards and includes tests and procedures which allow him to report on the fairness of the financial statements prepared by management.

[Original signed by Kerry Bales]

Kerry Bales
Chief Executive Officer
Recovery Alberta

[Original signed by Jennifer Ocloo]

Jennifer Ocloo
Lead, Finance
Recovery Alberta

June 3, 2025

Independent Auditor's Report



**Auditor
General**
OF ALBERTA

To the Chief Executive Officer and Board Chair of Recovery Alberta

Report on the Financial Statements

Opinion

I have audited the financial statements of Recovery Alberta, which comprise the statement of financial position as at March 31, 2025, and the statements of operations, change in net financial assets, and cash flows for the nine-months then ended, and notes to the financial statements, including a summary of significant accounting policies.

In my opinion, the accompanying financial statements present fairly, in all material respects, the financial position of Recovery Alberta as at March 31, 2025, and the results of its operations, its changes in net financial assets, and its cash flows for the nine-months then ended in accordance with Canadian public sector accounting standards.

Basis for opinion

I conducted my audit in accordance with Canadian generally accepted auditing standards. My responsibilities under those standards are further described in the *Auditor's Responsibilities for the Audit of the Financial Statements* section of my report. I am independent of Recovery Alberta in accordance with the ethical requirements that are relevant to my audit of the financial statements in Canada, and I have fulfilled my other ethical responsibilities in accordance with these requirements. I believe that the audit evidence I have obtained is sufficient and appropriate to provide a basis for my opinion.

Emphasis of matter

I draw attention to Note 3 and Note 22 of the financial statements, which describes the restructuring of Alberta Health Services and establishment of Recovery Alberta, and subsequent events. My opinion is not modified in respect of this matter.

Other information

The financial statements of Recovery Alberta are included in the *Annual Report of the Ministry of Mental Health and Addiction* that is prepared by the Ministry of Mental Health and Addiction. The other information comprises the information included in the *Annual Report of the Ministry of Mental Health and Addiction* relating to Recovery Alberta, but does not include the financial statements of Recovery Alberta and my auditor's report thereon. The *Annual Report of the Ministry of Mental Health and Addiction* is expected to be made available to me after the date of this auditor's report.

My opinion on the financial statements does not cover the other information and I do not express any form of assurance conclusion thereon.

In connection with my audit of the financial statements, my responsibility is to read the other information identified above and, in doing so, consider whether the other information is materially inconsistent with the financial statements or my knowledge obtained in the audit, or otherwise appears to be materially misstated.

If, based on the work I will perform on this other information, I conclude that there is a material misstatement of this other information, I am required to communicate the matter to those charged with governance.

Responsibilities of management and those charged with governance for the financial statements

Management is responsible for the preparation and fair presentation of the financial statements in accordance with Canadian public sector accounting standards, and for such internal control as management determines is necessary to enable the preparation of the financial statements that are free from material misstatement, whether due to fraud or error.

In preparing the financial statements, management is responsible for assessing Recovery Alberta's ability to continue as a going concern, disclosing, as applicable, matters related to going concern and using the going concern basis of accounting unless an intention exists to liquidate or to cease operations, or there is no realistic alternative but to do so.

Those charged with governance are responsible for overseeing Recovery Alberta's financial reporting process.

Auditor's responsibilities for the audit of the financial statements

My objectives are to obtain reasonable assurance about whether the financial statements as a whole are free from material misstatement, whether due to fraud or error, and to issue an auditor's report that includes my opinion. Reasonable assurance is a high level of assurance, but is not a guarantee that an audit conducted in accordance with Canadian generally accepted auditing standards will always detect a material misstatement when it exists. Misstatements can arise from fraud or error and are considered material if, individually or in the aggregate, they could reasonably be expected to influence the economic decisions of users taken on the basis of these financial statements.

As part of an audit in accordance with Canadian generally accepted auditing standards, I exercise professional judgement and maintain professional skepticism throughout the audit. I also:

- Identify and assess the risks of material misstatement of the financial statements, whether due to fraud or error, design and perform audit procedures responsive to those risks, and obtain audit evidence that is sufficient and appropriate to provide a basis for my opinion. The risk of not detecting a material misstatement resulting from fraud is higher than for one resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control.
- Obtain an understanding of internal control relevant to the audit in order to design audit procedures that are appropriate in the circumstances, but not for the purpose of expressing an opinion on the effectiveness of Recovery Alberta's internal control.
- Evaluate the appropriateness of accounting policies used and the reasonableness of accounting estimates and related disclosures made by management.
- Conclude on the appropriateness of management's use of the going concern basis of accounting and, based on the audit evidence obtained, whether a material uncertainty exists related to events or conditions that may cast significant doubt on Recovery Alberta's ability to continue as a going concern. If I conclude that a material uncertainty exists, I am required to draw attention in my auditor's report to the related disclosures in the financial statements or, if such disclosures are inadequate, to modify my opinion. My conclusions are based on the audit

evidence obtained up to the date of my auditor's report. However, future events or conditions may cause Recovery Alberta to cease to continue as a going concern.

- Evaluate the overall presentation, structure and content of the financial statements, including the disclosures, and whether the financial statements represent the underlying transactions and events in a manner that achieves fair presentation.

I communicate with those charged with governance regarding, among other matters, the planned scope and timing of the audit and significant audit findings, including any significant deficiencies in internal control that I identify during my audit.

[Original signed by W. Doug Wylie FCPA, FCMA, ICD.D]
Auditor General

June 3, 2025
Edmonton, Alberta

Statement of Operations

Recovery Alberta

Nine months ended March 31, 2025

(in thousands)

	2025	
	Budget (Note 4)	Actual
Revenues:		
Ministry of Mental Health and Addiction transfers		
Base operating	\$ 726,840	\$ 726,840
One-time base operating	-	85,350
Other operating	60,601	68,771
Recognition of expended deferred capital revenue	-	14
Other government transfers (Note 5)	9,304	9,865
Fees and charges	3,485	2,415
Donations and non-government contributions (Note 6)	1,671	2,512
Other income (Note 7)	4,188	12,461
Net effect of restructuring transaction (Note 3)	-	679
TOTAL REVENUES	806,089	908,907
Expenses:		
Continuing care	3,909	2,688
Community care	380,524	414,507
Acute care	210,360	251,646
Diagnostic and therapeutic services	64,595	70,787
Population and public health	2,792	3,346
Research and education	3,218	3,542
Information technology	24,633	25,188
Support services (Note 8)	88,213	93,314
Administration (Note 9)	27,845	27,201
TOTAL EXPENSES (Schedules 1 and 3)	806,089	892,219
ANNUAL OPERATING SURPLUS	-	16,688
Accumulated surplus, beginning of period	-	-
Accumulated surplus, end of period (Note 18)	\$ -	\$ 16,688

The accompanying notes and schedules are part of these financial statements.

Statement of Financial Position

Recovery Alberta

As at March 31, 2025

(in thousands)

	2025 Actual
Financial Assets:	
Petty cash	\$ 28
Due from Alberta Health Services	251,863
Accounts receivable (Note 11)	25,659
	277,550
Liabilities:	
Accounts payable and accrued liabilities (Note 12)	128,479
Employee future benefits (Note 13)	71,211
Unexpended deferred operating revenue (Note 14)	63,237
Unexpended deferred capital revenue (Note 15)	14
	262,941
NET FINANCIAL ASSETS	14,609
Non-Financial Assets:	
Tangible capital assets (Note 17)	4,198
	4,198
NET ASSETS BEFORE EXPENDED DEFERRED REVENUE	18,807
Expended deferred capital revenue (Note 16)	2,119
NET ASSETS	16,688
Net Assets is comprised of:	
Accumulated surplus (Note 18)	16,688
	\$ 16,688

Contractual Obligations and Contingent Liabilities (Note 19)

The accompanying notes and schedules are part of these financial statements.

Approved by:

[Original signed by Kerry Bales]

Kerry Bales
Board Chair
Recovery Alberta

Statement of Change in Net Financial Assets

Recovery Alberta

Nine months ended March 31, 2025

(in thousands)

	2025	
	Budget ⁽ⁱ⁾	Actual
Annual operating surplus		\$ 16,688
Effect of changes in tangible capital assets:		
Acquisition of tangible capital assets:		
Purchased		(155)
Leased		(355)
Amortization and loss on disposals/write-downs of tangible capital assets		1,049
Effect of other changes:		
Net decrease in expended deferred capital revenue		(169)
Net decrease in prepaid expenses		5,279
Net debt transferred as a result of restructuring transaction		(7,049)
Net effect of restructuring transaction		(679)
Increase in net financial assets for the period		14,609
Net financial assets, beginning of period		-
Net financial assets, end of period		\$ 14,609

(i) No budget values are presented as no budget was approved for the statement of change in net financial assets for the nine months ended March 31, 2025 (Note 4).

The accompanying notes and schedules are part of these financial statements.

Statement of Cash Flows

Recovery Alberta

Nine months ended March 31, 2025

(in thousands)

	2025
	Actual
Operating transactions:	
Annual operating surplus	\$ 16,688
Non-cash items:	
Amortization and loss on disposals/write-downs of tangible capital assets	1,049
Recognition of expended deferred capital revenue	(286)
Change in employee future benefits	6,090
Net effect of restructuring transaction	(679)
(Increase) decrease in:	
Accounts receivable related to operating transactions	(16,881)
Due from Alberta Health Services	(60,584)
Prepaid expenses	5,279
Increase (decrease) in:	
Accounts payable and accrued liabilities	53,503
Unexpended deferred operating revenue	(3,549)
Cash provided by operating transactions	630
Capital transactions:	
Purchased tangible capital assets	(155)
Cash applied to capital transactions	(155)
Financing transactions:	
Restricted capital contributions received	131
Payments on obligations under capital leases	(606)
Cash applied to financing transactions	(475)
Increase in cash	-
Cash, beginning of period	-
Cash, end of period	\$ -

The accompanying notes and schedules are part of these financial statements.

Notes to the Financial Statements

Recovery Alberta

For the nine months ended March 31, 2025

(in thousands)

Note 1 Authority, Purpose and Operations

Recovery Alberta was legally established as a Provincial Health Agency (PHA) under the *Provincial Health Agencies Act* (Alberta), effective July 1, 2024, as a result of the Government of Alberta's initiative to refocus the health care system. Effective September 1, 2024, Alberta Health Services (AHS) transferred to Recovery Alberta all of the powers, duties, responsibilities and functions of AHS necessary so that Recovery Alberta can fulfill its responsibility for the delivery of mental health and addiction services.

Pursuant to Part 2 of the *Provincial Health Agencies Act, RSA 2000 (Alberta)*, effective June 21, 2024, Recovery Alberta, as a PHA is responsible for the delivery of health services and the oversight and coordination of the delivery of health services in the health services sector or sectors for which it was established. For the purposes of the Act, Recovery Alberta shall:

- (a) deliver health services or arrange for the delivery of health services in accordance with section 1.96,
- (b) oversee the delivery of health services by the parties with whom the provincial health agency has entered into agreements to deliver health services and ensure the accountability of those parties,
- (c) monitor and evaluate the delivery of health services in the health services sector,
- (d) implement the provincial health agency's approved health plan, approved operational plan and approved additional plans,
- (e) establish priorities relating to the delivery of health services in the health services sector and regularly assess and modify those priorities,
- (f) ensure that health services are delivered in the health services sector in a manner that is responsive to the needs of individuals and communities and supports the integration of health services,
- (g) work with the oversight Minister, sector Ministers, other provincial health agencies, regional health authorities, provincial health corporations, health services delivery organizations and other health services providers to coordinate the delivery of health services within the health services sector and across health services sectors,
- (h) ensure that the transfer of patients and the responsibility for their care within the health services sector and between health services sectors is integrated, efficient and safe,
- (i) consult regularly with the oversight Minister, sector Ministers, other provincial health agencies, regional health authorities and other persons and bodies with respect to issues relating to health services and the delivery of health services within the health services sector and across health services sectors,
- (j) support reasonable access to quality health services in the health services sector,
- (k) assess the health needs of the population related to the health services sector on an ongoing basis
- (l) promote and protect the health of the population and work toward the prevention of disease and injury related to the health services sector,
- (m) exercise any powers and carry out any duties, responsibilities or functions delegated to the provincial health agency by the oversight Minister or sector Minister under section 1.8, and
- (n) do any other thing required by the oversight Minister, the sector Minister or the regulations.

Recovery Alberta is accountable to the Minister of Mental Health and Addiction (the Minister) for the delivery, oversight and coordination of the mental health and addiction services for Albertans.

The Recovery Alberta financial statements include the assets, liabilities, revenues and expenses associated with its responsibilities.

Note 2 Significant Accounting Policies and Reporting Practices

(a) Basis of Presentation

These financial statements have been prepared in accordance with Canadian Public Sector Accounting Standards. In addition, the financial statements include certain disclosures required by the financial directives issued by the Ministry of Mental Health and Addiction (MHA).

Notes to the Financial Statements (continued)

Recovery Alberta

For the nine months ended March 31, 2025

(in thousands)

Note 2 Significant Accounting Policies and Reporting Practices (continued)

Recovery Alberta is responsible for the delivery, oversight and coordination of the mental health and addiction services in Alberta (Note 1) and contracts with various health service providers to support mental health and addiction services throughout Alberta. The largest of these service providers is AHS, who is providing clinical support and corporate services (Note 20). These financial statements do not include AHS' assets, liabilities or results of operations of these providers. However, the payments for contracts with health service providers such as AHS are recorded as expenses in the Statement of Operations.

(b) Revenue Recognition

All revenues are recorded on an accrual basis, except when the accrual cannot be determined within a reasonable degree of certainty or when estimation is impracticable. Revenues from transactions with performance obligations are recognized when Recovery Alberta provides the promised goods and/or services to a payor. Revenue from transactions with no performance obligations are recognized at their realizable value when Recovery Alberta has the authority to claim or retain an inflow of economic resources and identifies a past transaction or event that gives rise to an asset. Unallocated costs comprising of materials and services contributed by related parties in support of Recovery Alberta's operations, are not recognized in these financial statements.

(i) Government Transfers

Transfers from MHA, other Province of Alberta ministries and agencies, and other government entities are referred to as government transfers. Government transfers are monetary or tangible capital asset transfers where the government making the transfer does not receive any goods or services directly in return, expect to be repaid in the future, or expect a direct financial return.

Government transfers are recorded as deferred revenue when the stipulations together with Recovery Alberta's actions and communications as to the use of the transfer, create a liability. These transfers are recognized as revenue as the stipulations are met and, when applicable, Recovery Alberta complies with the communicated use of the transfer.

All other government transfers, without stipulations for the use of the transfer, are recorded as revenue when the transfer is authorized and Recovery Alberta meets the eligibility criteria.

Deferred revenue consists of unexpended deferred operating revenue, unexpended deferred capital revenue and expended deferred capital revenue. The term deferred revenue in these financial statements refers to the components of deferred revenue as described.

(ii) Donations and Non-Government Contributions

Donations and non-government contributions are received from individuals, corporations, registered charities, and other not-for-profit organizations. Donations and non-government contributions may be unrestricted or externally restricted for operating or capital purposes.

Unrestricted donations, and non-government contributions are recorded as revenue in the year received or in the year the funds are committed to Recovery Alberta if the amount can be reasonably estimated and collection is reasonably assured.

Externally restricted donations and non-government contributions are recorded as deferred revenue if the terms for their use, or the terms along with Recovery Alberta's actions and communications as to their use create a liability. These resources are recognized as revenue as the terms are met and, when applicable, Recovery Alberta complies with the communicated use.

In-kind contributions of services and materials from non-related parties are recorded at fair value when such value can reasonably be determined. While volunteers contribute a significant amount of time each year to assist Recovery Alberta, the value of their services is not recognized as revenue and expenses in the financial statements because fair value cannot be reasonably determined.

Notes to the Financial Statements (continued)

Recovery Alberta

For the nine months ended March 31, 2025

(in thousands)

Note 2 Significant Accounting Policies and Reporting Practices (continued)

(iii) Fees and Charges and Other Income

Fees and charges and other income are considered revenue arising from exchange transactions with performance obligations. These are recognized in the year that goods are delivered or services are provided by Recovery Alberta. Amounts received for which goods or services have not been provided by year end are recorded as deferred revenue.

Other income includes an allocation of investment income recognized by AHS. Portfolio investments are managed centrally by AHS with the income allocated to Recovery Alberta on a pro-rata basis for Recovery Alberta's share of the portfolio's income. The investment income recognized by AHS is used as the basis for the income allocation and includes dividend income, interest income, and realized gains or losses on the sale of portfolio investments.

(c) Expenses

Expenses are reported on an accrual basis. The cost of all goods consumed and services received during the year are expensed.

(d) Financial Instruments

Financial instruments comprise financial assets and financial liabilities. Financial assets are assets that could be used to discharge existing liabilities or finance future operations and are not for consumption in the normal course of operations. Financial liabilities are contractual obligations to deliver cash or another financial asset to another entity or to exchange financial instruments with another entity under conditions that are potentially unfavourable to Recovery Alberta.

All of Recovery Alberta's financial assets and financial liabilities are initially recorded at their fair value and are subsequently measured at cost or amortized cost.

Recovery Alberta's financial instruments include cash, accounts receivable, receivable from Alberta Health Services, and accounts payable and accrued liabilities.

Contractual obligations are evaluated for the existence of embedded derivatives. Recovery Alberta measures and recognizes embedded derivatives separately from the host contract when the economic characteristics and risk of the embedded derivative are not closely related to those of the host contract, when it meets the definition of a derivative, and when the entire contract is not measured at fair value. An election can be made to either measure the entire contract at fair value or measure the value of the derivative component separately when characteristics of the derivative are not closely related to the economic characteristics and risks of the contract itself. Contracts to buy or sell non-financial items for Recovery Alberta's normal course of business are not recognized as financial assets or liabilities. Recovery Alberta does not have any embedded derivatives.

Financial instruments are adjusted by transaction costs incurred on acquisition and financing costs, which are amortized using the effective interest rate method.

All financial assets are assessed for impairment on an annual basis. When a decline is determined to be other than temporary, the amount of the loss is reported as a realized loss on the Statement of Operations.

A financial liability or a part thereof is derecognized when it is extinguished.

Recovery Alberta has not presented a Statement of Remeasurement Gains and Losses, as no activities would result in remeasurement gains or losses.

(e) Accounts receivable

Accounts receivable are recognized at the lower of cost or net recoverable value. A valuation allowance is recognized when recovery is uncertain.

Notes to the Financial Statements (continued)

Recovery Alberta

For the nine months ended March 31, 2025

(in thousands)

Note 2 Significant Accounting Policies and Reporting Practices (continued)

(f) Tangible Capital Assets

Tangible capital assets are recorded at cost less accumulated amortization, which includes amounts that are directly related to the acquisition, design, construction, development, improvement, or betterment of the assets. Cost includes overhead directly attributable to construction and development as well as interest costs that are directly attributable to the acquisition or construction of the asset.

Contributed tangible capital assets from non-related entities are recognized at their fair value at the date of the contribution when fair value can be reasonably determined. When Recovery Alberta cannot determine the fair value, it records such contributions at nominal value.

The costs less residual values of tangible capital assets are amortized over their estimated useful lives on a straight-line basis as follows:

	<u>Useful Life</u>
Equipment	3-20 years

Leases of tangible capital assets which transfer substantially all benefits and risks of ownership to Recovery Alberta are accounted for as leased equipment and are amortized over the shorter of the term of the lease or their estimated useful lives. Obligations under capital leases are recorded at the present value of the minimum lease payments excluding executory costs (e.g. insurance, maintenance costs, etc.). The discount rate used to determine the present value of the lease payments is the lower of Recovery Alberta's rate for incremental borrowing and the interest rate implicit in the lease.

Tangible capital assets are written down to their net recoverable amount when conditions indicate that they no longer contribute to Recovery Alberta's ability to provide goods and services or when the value of future economic benefits associated with the tangible capital assets are less than their net book value. Write-downs are recorded as part of amortization and loss on disposals / write-downs of tangible capital assets.

Intangibles and other assets inherited by right and that have not been purchased are not recognized in these financial statements. Similarly, works of art, historical treasures, and collections are not recognized as tangible capital assets.

(g) Employee Future Benefits

(i) Defined Benefit Pension Plans

Local Authorities Pension Plan (LAPP) and Management Employees Pension Plan (MEPP)

Recovery Alberta participates in the LAPP and MEPP which are multi-employer registered defined benefit pension plans. Recovery Alberta accounts for these plans on a defined contribution basis. Accordingly, the pension expense recorded for these plans in these financial statements is comprised of the employer contributions that Recovery Alberta is required to pay for its employees during the fiscal year. LAPP and MEPP set the employer contribution rates on an annual basis based on actuarially pre-determined amounts that are expected to provide the plans' future benefits.

(ii) Defined Contribution Pension Plans

Group Registered Retirement Savings Plans (GRRSPs)

Recovery Alberta sponsors GRRSPs for certain employee groups. Under the GRRSPs, Recovery Alberta matches a certain percentage of any contribution made by plan participants up to certain limits.

Supplemental Pension Plan (SPP)

The SPP supplements the benefits under Recovery Alberta registered plans that are limited by the *Income Tax Act* (Canada). Recovery Alberta contributes a percentage of an eligible employee's pensionable earnings, in excess of the limits of the *Income Tax Act* (Canada). The SPP provides participants with an account balance at retirement based on the contributions made to the plan and investment income earned on the contributions based on investment decisions made by the participant.

Note 2 Significant Accounting Policies and Reporting Practices (continued)**(iii) Other Benefit Plans****Accumulating Non-Vesting Sick Leave**

Sick leave benefits accumulate with employees' service and are provided by Recovery Alberta to certain employee groups of Recovery Alberta, as defined by employment agreements, to cover illness related absences that are outside of short-term and long-term disability coverage. Benefit amounts are determined and accumulate with reference to employees' earnings at the time they are paid out. The cost of the accumulating non-vesting sick leave benefits is expensed as the benefits are earned.

Recovery Alberta recognizes a liability and expense for accumulating non-vesting sick leave benefits using an actuarial cost method as the employees render services to earn the benefits. The liability and expense is determined using the projected benefit method pro-rated for service and management's best estimates of expected discount rate, inflation, rate of compensation increase, termination and retirement dates, sick leave accumulation and utilization, and mortality. Actuarial gains and losses are amortized on a straight-line basis over the expected average remaining service life of the related employee groups.

Recovery Alberta does not record a liability for sick leave benefits that do not accumulate beyond the current reporting year as these are renewed annually.

Other Benefits

Recovery Alberta provides its employees with basic life, accidental death and dismemberment, short-term disability, long-term disability, extended health, dental, and vision benefits through benefits carriers. Recovery Alberta fully accrues its obligations for employee non-pension future benefits.

(h) Reserves

Certain amounts, as approved by the Recovery Alberta Board, may be set aside in accumulated surplus for use by Recovery Alberta for future purposes. Transfers to, or from, are recorded to the respective reserve account when approved.

(i) Measurement Uncertainty

The financial statements, by their nature, contain estimates and are subject to measurement uncertainty. Measurement uncertainty exists when there is a difference between the recognized or disclosed amount and another reasonably possible amount. These estimates and assumptions are reviewed at least annually. Actual results could differ from the estimates determined by management in these financial statements, and these differences could require adjustment in subsequent reporting years.

The amount recorded for amortization of tangible capital assets is based on the estimated useful life of the related assets while the recognition of expended deferred capital revenue depends on when the terms for the use of the funding are met and, when applicable, Recovery Alberta complies with its communicated use of the funding. The amounts recorded for accumulating non-vesting sick leave are based on various assumptions including the estimated service life of employees, drawdown rate of sick leave banks and rate of salary escalation.

Notes to the Financial Statements (continued)

Recovery Alberta

For the nine months ended March 31, 2025

(in thousands)

Note 2 Significant Accounting Policies and Reporting Practices (continued)

(j) Future Accounting Changes

On April 1, 2026, Recovery Alberta will adopt the following new conceptual framework and accounting standard approved by the Public Sector Accounting Board:

- The Conceptual Framework for Financial Reporting in the Public Sector**
 The Conceptual Framework is the foundation for public sector financial reporting standard setting. It replaces the conceptual aspects of Section PS 1000 Financial Statement Concepts and Section PS 1100 Financial Statement Objectives. The conceptual framework highlights considerations fundamental for the consistent application of accounting issues in the absence of specific standards.
- PS 1202 Financial Statement Presentation**
 Section PS 1202 sets out general and specific requirements for the presentation of information in general purpose financial statements. The financial statement presentation principles are based on the concepts within the Conceptual Framework.

Management is currently assessing the impact of the conceptual framework and the standard on the Recovery Alberta financial statements.

Note 3 Restructuring of AHS and Establishment of Recovery Alberta

In accordance with the Government of Alberta's ministerial orders related to the restructuring of AHS, all responsibilities and the oversight and coordination of the delivery of mental health and addiction services, along with related assets, liabilities, and funding agreements were transferred from AHS to Recovery Alberta effective September 1, 2024. Recovery Alberta is associated with AHS since both entities are accountable to their respective Ministries within the Government of Alberta. Recovery Alberta did not provide or receive any compensation related to the transfer of assets and liabilities.

	September 1, 2024
Financial assets:	
Petty cash	\$ 28
Due from AHS	191,279
Accounts receivable	8,778
	<u>200,085</u>
Liabilities:	
Accounts payable and accrued liabilities	75,227
Employee future benefits	65,121
Unexpended deferred operating revenue	66,786
	<u>207,134</u>
Non-financial assets:	
Tangible capital assets	4,737
Prepaid expenses	5,279
	<u>10,016</u>
Expended deferred capital revenue	<u>2,288</u>
Net effect of restructuring	\$ 679

Notes to the Financial Statements (continued)

Recovery Alberta

For the nine months ended March 31, 2025

(in thousands)

Note 3 Restructuring of AHS and Establishment of Recovery Alberta (continued)

(a) The tangible capital assets transferred to Recovery Alberta includes equipment with a cost of \$18,203 and accumulated amortization of \$13,466 (Note 17). Obligations under capital leases transferred to Recovery Alberta were \$1,522 (Note 12).

The related sector contractual obligations were also transferred to Recovery Alberta. Contracts transferred to Recovery Alberta for \$262,703 were related to the continuum of mental health and addiction services and supports, from prevention and intervention to treatment and recovery.

Recovery Alberta entered into a Transition Services Framework Agreement (TSFA), Cooperation Agreement and Information Management and Information Sharing Agreement with AHS effective September 1, 2024. Together, these agreements will govern the relationship between AHS and Recovery Alberta. Under the TSFA, AHS has agreed to provide transition services to Recovery Alberta. The transition services include all clinical and corporate support services provided by AHS to itself including but not limited to nutrition, food, linen & environmental services, capital management, information technology, protective services, pharmacy services, human resources, financial operations, APL services and health information management.

The TSFA commenced on the effective date and continues until August 31, 2026. Obligations under this agreement are \$264,194 for the year ending March 31, 2026, and \$110,744 for the year ending March 31, 2027 (Note 19).

Note 4 Budget

The nine months ended March 31, 2025 Statement of Operations budget was approved by the Recovery Alberta Board on August 28, 2024, and thereafter submitted to and approved by the Minister of Mental Health and Addiction. No budget was prepared or approved for the Statement of Financial Position as the assets and liabilities that were transferring from AHS to Recovery Alberta, as part of the restructuring transaction, were still being identified at the time of budget submission.

Notes to the Financial Statements (continued)

Recovery Alberta

For the nine months ended March 31, 2025

(in thousands)

Note 5 Other Government Transfers

	Budget	2025
Recognition of expended deferred capital revenue (Note 16)	\$ -	\$ 221
Restricted operating (Note 14)	3,634	4,125
Unrestricted operating	5,670	5,519
	\$ 9,304	\$ 9,865

Other government transfers include \$6,548 (Note 20) transferred from the Province of Alberta, \$3,317 from government entities outside the Province of Alberta and exclude amounts from MHA as these amounts are separately disclosed on the Statement of Operations.

Note 6 Donations and Non-Government Contributions

	Budget	2025
Recognition of expended deferred capital revenue (Note 16)	\$ -	\$ 51
Restricted operating (Note 14)	1,612	1,688
Unrestricted operating	59	773
	\$ 1,671	\$ 2,512

Note 7 Other Income

	Budget	2025
Other income:		
AHS ⁽ⁱ⁾	\$ -	\$ 5,251
Other Province of Alberta Ministries (Note 20)	-	1,356
Other ⁽ⁱⁱ⁾	4,188	5,854
	\$ 4,188	\$ 12,461

⁽ⁱ⁾ Other income from AHS comprises an allocation of investment income recognized by AHS.

⁽ⁱⁱ⁾ Other mainly relates to recoveries for services provided to third parties.

Note 8 Support Services

	Budget	2025
Facilities operations	\$ 41,326	\$ 41,522
Patient health records, food services, and transportation	19,268	19,122
Housekeeping, laundry, and linen	12,332	12,324
Materials management	4,133	4,132
Ancillary operations	2,730	2,730
Other	8,424	13,484
	\$ 88,213	\$ 93,314

Note 9 Administration

	Budget	2025
General administration	\$ 14,196	\$ 13,317
Human resources	6,860	6,860
Finance	3,620	3,620
Communications	3,169	3,404
	\$ 27,845	\$ 27,201

Notes to the Financial Statements (continued)

Recovery Alberta

For the nine months ended March 31, 2025

(in thousands)

Note 10 Financial Risk Management

Recovery Alberta is exposed to a variety of financial risks associated with its financial instruments. These financial risks include market risk, credit risk, and liquidity risk.

(a) Market Risk

Market risk is the risk that the fair value or future cash flows of a financial instrument will fluctuate due to changes in market prices. Market risk is comprised of three types of risk: price risk, interest rate risk, and foreign currency risk. Recovery Alberta does not hold portfolio investments or debt, and therefore market risk is minimal.

(b) Credit Risk

Credit risk is the risk of loss arising from the failure of a counterparty to fully honour its contractual obligations. Recovery Alberta's receivables are exposed to credit risk, however, these amounts are largely due from MHA and AHS; therefore, the credit risk is minimal.

(c) Liquidity Risk

Liquidity risk is the risk that Recovery Alberta will encounter difficulty under both normal and stressed conditions in meeting obligations associated with financial liabilities that are settled by delivery of cash or another financial asset. Recovery Alberta's accounts payable and accrued liabilities are exposed to liquidity risk; however, the risk is minimal as AHS is responsible for settling the majority of Recovery Alberta's obligations mainly through funding provided by MHA.

The following are contractual maturities of the remaining financial liabilities as at March 31, 2025 based on expected undiscounted cash flows.

	Due in less than 1 year	Due in 1-5 years	Due after 5 years
Payroll payable and related accrued liabilities	\$ 121,166	\$ -	\$ -
Trade accounts payable and accrued liabilities	6,042	-	-
Other liabilities	455	816	-
	\$ 127,663	\$ 816	\$ -

Note 11 Accounts Receivable

	2025		
	Gross	Allowance for Doubtful Accounts	Net
Patient accounts receivable	\$ 2,050	\$ 13	\$ 2,037
Other operating transfers receivable ⁽ⁱ⁾	19,812	-	19,812
Other accounts receivable	3,810	-	3,810
	\$ 25,672	\$ 13	\$ 25,659

⁽ⁱ⁾ Other operating transfers receivable includes \$15,943 contributions receivable from MHA.

Notes to the Financial Statements (continued)

Recovery Alberta

For the nine months ended March 31, 2025

(in thousands)

Note 12 Accounts Payable and Accrued Liabilities

	2025
Payroll payable and related accrued liabilities	\$ 121,166
Trade accounts payable and accrued liabilities	6,042
Obligations under capital leases ^(a)	1,271
	\$ 128,479

(a) Obligations under capital leases include vehicle leases for fleet vehicles.

The obligations will be settled between 2026 and 2030 and have an implicit interest rate payable of 3.97%.

Recovery Alberta is committed to making principal payments for obligations under capital leases as follows:

March 31	Minimum Contract Payments
2026	\$ 455
2027	398
2028	246
2029	141
2030	31
	\$ 1,271

Note 13 Employee Future Benefits

	2025
Accrued vacation pay	\$ 59,136
Accumulating non-vesting sick leave ^(a)	12,075
	\$ 71,211

(a) Accumulating Non-Vesting Sick Leave

Sick leave benefits are paid by Recovery Alberta; there are no employee contributions and no assets set aside to support the obligation.

	2025
Funded status – deficit	\$ 13,978
Unamortized net actuarial gain	(1,903)
Accrued benefit liability	\$ 12,075

Key assumptions used in the determination of the accumulating non-vesting sick leave liability are:

	2025
Estimated average remaining service life	13 years
Draw down rate of accumulated non-vesting sick leave bank	24.10%
Discount rate – beginning of year	5.00%
Discount rate – end of year	3.63%
Rate of compensation increase per year	2024-25
	2.00%
	2025-26
	3.00%
	2026-27
	3.00%
	Thereafter
	2.75%

Notes to the Financial Statements (continued)

Recovery Alberta

For the nine months ended March 31, 2025

(in thousands)

Note 13 Employee Future Benefits (continued)

(b) Local Authorities Pension Plan (LAPP)

(i) Recovery Alberta Participation in the LAPP

The majority of Recovery Alberta employees participate in the LAPP. Recovery Alberta is not responsible for future funding of the plan deficit other than through contribution increases. As Recovery Alberta is exposed to the risk of contribution rate increases, the following disclosure is provided to explain this risk.

The LAPP provides for a pension of 1.4% for each year of pensionable service based on the average salary of the highest five consecutive years up to the average Canada Pension Plan's Year's Maximum Pensionable Earnings (YMPE), over the same five consecutive year period and 2.0% on the excess, subject to the maximum pension benefit limit allowed under the *Income Tax Act* (Canada). The maximum pensionable service allowable under the plan is 35 years.

(ii) LAPP Surplus

The LAPP carried out an actuarial valuation as at December 31, 2023 and these results were then extrapolated to December 31, 2024. The LAPP reported a surplus at December 31, 2024 of \$19,557,148.

The 2025 and 2024 LAPP contribution rates are as follows:

Calendar 2025		Calendar 2024	
Employer	Employees	Employer	Employees
8.45% of pensionable earnings up to the YMPE and 11.65% of the excess	7.45% of pensionable earnings up to the YMPE and 10.65% of the excess	8.45% of pensionable earnings up to the YMPE and 11.65% of the excess	7.45% of pensionable earnings up to the YMPE and 10.65% of the excess

(c) Pension Expense

	2025
Local Authorities Pension Plan	\$ 37,754
Defined contribution pension plans and group RRSPs	2,654
Other pension plans	66
	\$ 40,474

Notes to the Financial Statements (continued)

Recovery Alberta

For the nine months ended March 31, 2025

(in thousands)

Note 14 Unexpended Deferred Operating Revenue

(a) Changes in the unexpended deferred operating revenue balance are as follows:

	2025			
	MHA	Other Government ⁽ⁱ⁾	Donors and Non-Government	Total
Balance, beginning of period	\$ -	\$ -	\$ -	\$ -
Balance transferred from AHS September 1, 2024 (Note 3)	64,985	40	1,761	66,786
Received or receivable during the period	63,013	4,625	3,519	71,157
Unexpended deferred operating revenue returned	-	-	(104)	(104)
Transferred to unexpended deferred capital revenue ⁽ⁱⁱ⁾	-	-	(18)	(18)
Recognized as revenue	(68,771)	(4,125)	(1,688)	(74,584)
Balance, end of period	\$ 59,227	\$ 540	\$ 3,470	\$ 63,237

(i) The balance for other government includes (\$36) of unexpended deferred operating revenue received from government entities outside the Province of Alberta. The remaining balance in other government all relates to the Province of Alberta (Note 20).

(ii) The transfer is mainly comprised of restricted capital funding that was used for approved expenditures that did not meet the definition of a tangible capital asset.

(b) The unexpended deferred operating revenue balance at the end of the period is externally restricted for the following purposes:

	2025			
	MHA	Other Government	Donors and Non-Government	Total
Community Care	\$ 56,918	\$ 54	\$ 600	\$ 57,572
Diagnostic and therapeutic services	-	241	1,721	1,962
Acute care	2,233	-	651	2,884
Other	76	245	498	819
	\$ 59,227	\$ 540	\$ 3,470	\$ 63,237

Notes to the Financial Statements (continued)

Recovery Alberta

For the nine months ended March 31, 2025

(in thousands)

Note 15 Unexpended Deferred Capital Revenue

Changes in the unexpended deferred capital revenue balance are as follows:

	2025			
	MHA	Other Government ⁽ⁱ⁾	Donors and Non-Government	Total
Balance, beginning of period	\$ -	\$ -	\$ -	\$ -
Received or receivable during the period	48	48	17	113
Transferred to expended deferred capital revenue	(48)	(34)	(35)	(117)
Transferred from unexpended deferred operating revenue ⁽ⁱⁱ⁾	-	-	18	18
Balance, end of period⁽ⁱⁱⁱ⁾	\$ -	\$ 14	\$ -	\$ 14

(i) The balance for other government all relates to the Province of Alberta.

(ii) The transfer is mainly comprised of restricted capital funding of approved expenditures that did not meet the definition of a tangible capital asset.

(iii) The unexpended deferred capital revenue balance at the end of the period of \$14 is externally restricted by Alberta Health for purpose of equipment purchases.

Note 16 Expended Deferred Capital Revenue

Changes in the expended deferred capital revenue balance are as follows:

	2025			
	MHA	Other Government ⁽ⁱ⁾	Donors and Non-Government	Total
Balance, beginning of period	\$ -	\$ -	\$ -	\$ -
Balance transferred from AHS on September 1, 2024 (Note 3)	43	1,776	469	2,288
Transferred from unexpended deferred capital revenue	48	34	35	117
Recognized as revenue	(14)	(221)	(51)	(286)
Balance, end of period	\$ 77	\$ 1,589	\$ 453	\$ 2,119

(i) The balance in other government all relates to the Province of Alberta (Note 20).

Notes to the Financial Statements (continued)

Recovery Alberta

For the nine months ended March 31, 2025

(in thousands)

Note 17 Tangible Capital Assets

Cost	Balance transferred as at September 1, 2024 (Note 3)	Additions	Disposals/ Write-downs	2025
Equipment	\$ 16,681	\$ 155	\$ (90)	\$ 16,746
Leased equipment	1,522	355	-	1,877
	\$ 18,203	\$ 510	\$ (90)	\$ 18,623

Accumulated Amortization	Balance transferred as at September 1, 2024 (Note 3)	Amortization Expense	Disposals/ Write-downs	2025
Equipment	\$ 13,466	\$ 443	\$ (90)	\$ 13,819
Leased equipment	-	606	-	606
	\$ 13,466	\$ 1,049	\$ (90)	\$ 14,425

	Net Book Value	
	2025	
Equipment	\$	2,927
Leased equipment		1,271
	\$	4,198

Note 18 Accumulated Surplus

Accumulated surplus is comprised of the following:

	2025		
	Unrestricted Surplus	Invested in Tangible Capital Assets ^(a)	Total
Balance, beginning of period	\$ -	\$ -	\$ -
Annual operating surplus	16,688	-	16,688
Net investment in tangible capital assets	(808)	808	-
Balance, end of period	\$ 15,880	\$ 808	\$ 16,688

(a) Invested in Tangible Capital Assets

Invested in tangible capital assets represents the portion of accumulated surpluses that has been invested in the acquisition of Recovery Alberta's assets.

Reconciliation of invested in tangible capital assets:

	2025
Tangible capital assets (Note 17)	\$ 4,198
Less funded by:	
Expended deferred capital revenue (Note 16)	(2,119)
Obligations under capital leases (Note 12)	(1,271)
	\$ 808

Notes to the Financial Statements (continued)

Recovery Alberta

For the nine months ended March 31, 2025

(in thousands)

Note 19 Contractual Obligations and Contingent Liabilities

(a) Contractual Obligations

Contractual obligations are Recovery Alberta's obligations to others that will become liabilities in the future when the terms of those contracts or agreements are met.

The estimated aggregate amount payable for the unexpired terms of these contractual obligations are as follows:

Year ended March 31	Services ⁽ⁱ⁾	Other ⁽ⁱⁱ⁾	Total
2026	\$ 384,806	\$ 4,501	\$ 389,307
2027	139,190	2,250	141,440
2028	4,079	2,250	6,329
2029	360	-	360
2030	50	-	50
Thereafter	-	-	-
March 31, 2025	\$ 528,485	\$ 9,001	\$ 537,486

⁽ⁱ⁾ Service obligations include \$375M in contractual commitments under the Transition Services Framework Agreement (Note 3).

⁽ⁱⁱ⁾ Other obligations include contractual commitments to the Rural Capacity Investment Fund to fund nursing recruitment and retention strategies in rural and remote areas of Alberta.

(b) Contingent Liabilities

(i) Legal Claims

Recovery Alberta is subject to legal claims during its normal course of business. Recovery Alberta recognizes a liability when the assessment of a claim indicates that a future event is likely to confirm that a liability has been incurred at the date of the financial statements and the amount of the contingent loss can be reasonably estimated.

Recovery Alberta has been named in 1 legal claim related to conditions in existence at March 31, 2025 where the likelihood of the occurrence of a future event confirming a contingent loss is not determinable. This claim has no specified amount. The resolution of indeterminable claims may result in a liability, if any, that is different than the claimed amount. Subsequent to year end, a decision was made in the court of appeals, which changed Recovery Alberta's assessment to unlikely.

(ii) Collective Agreements

Recovery Alberta currently has 5 collective agreements that have expired as at March 31, 2025. Given that negotiations are ongoing or have not commenced, no additional disclosures have been made.

Notes to the Financial Statements (continued)

Recovery Alberta

For the nine months ended March 31, 2025

(in thousands)

Note 20 Related Parties

Transactions with related parties are included within these financial statements, unless otherwise stated.

The Minister appoints all members of the Recovery Alberta Board. The viability of Recovery Alberta's operations depends on transfers from MHA. Transactions between Recovery Alberta and MHA are reported and disclosed in the Statement of Operations, the Statement of Financial Position, and the Notes to the Financial Statements, and are therefore excluded from the tables below.

Related parties also include key management personnel of Recovery Alberta. Recovery Alberta has defined key management personnel to include those disclosed in Schedules 2A and 2B of these financial statements, except management reporting to CEO direct reports. Related party transactions with key management personnel primarily consist of compensation related payments and are undertaken on similar terms and conditions to those adopted if the entities were dealing at arm's length.

Recovery Alberta is a related party with respect to those entities consolidated or included on a modified equity basis in the financial statements of the Province of Alberta. Entities consolidated or included on a modified equity basis have been grouped with their respective ministry and transactions between Recovery Alberta and the other ministries are recorded at their exchange amount as follows:

	Revenues ^(a)	Expenses
	2025	2025
AHS ^(b)	\$ -	\$ 165,097
Ministry of Assisted Living and Social Services ^(c)	4,937	151
School Boards ^(d)	938	9,269
Other ministries	2,177	-
Total for the period	\$ 8,052	\$ 174,517

	Receivable from	Payable to
	2025	2025
AHS ^(b)	\$ 251,863	\$ -
Ministry of Assisted Living and Social Services ^(c)	1,153	-
School Boards ^(d)	363	397
Other ministries	140	-
Balance, end of period	\$ 253,519	\$ 397

- (a) Revenues with Province of Alberta ministries include other government transfers of \$6,548 (Note 5), and other income of \$1,356 (Note 7).
- (b) The transactions with AHS relate to services provided by AHS in accordance with the TSFA. The period end receivable relates primarily to the restructuring transaction and represents Recovery Alberta's share of cash and investments centrally managed by AHS (Note 3). The receivable from AHS is unsecured, non-interest bearing and has no fixed terms of repayment.
- (c) Transactions with the Ministry of Assisted Living and Social Services relate to various health programs promoting wellness aimed to improve quality of life and integration of patients back in community living.
- (d) Transactions with the School Boards relate to providing health programs in schools promoting Mental Health in Children and Youth. The provincial School Boards are accountable to and funded by the Ministry of Education and Childcare.

At March 31, 2025, Recovery Alberta has recorded deferred revenue from other ministries within the Province of Alberta, excluding MHA, of \$576 related to unexpended deferred operating revenue (Note 14) and \$1,589 related to expended deferred capital revenue (Note 16).

Notes to the Financial Statements (continued)

Recovery Alberta

For the nine months ended March 31, 2025

Note 21 Segment Disclosure

The Schedule of Segment Disclosures – *Schedule 3* is intended to enable users to better understand the reporting entity and identify the resources allocated to the major activities of Recovery Alberta.

Recovery Alberta's revenues, as reported on the Statement of Operations, are most informatively presented by source and are not reasonably assignable to the reportable segments. For each reported segment, the expenses are directly or reasonably attributable to the segment.

The segments have been selected based on the presentation that is adopted for the financial reporting, planning and budget processes, and represent the major distinguishable activities of Recovery Alberta.

Segments include:

(a) Continuing care

Continuing care is comprised of mental health and addiction psychiatric long-term care.

(b) Community care

Community care is comprised of community bed-based services such as detoxification, addiction treatment, and mental health supports. This includes the provision of addiction and mental health services via outpatient, virtual, and outreach services.

(c) Acute care

Acute care is comprised predominantly of mental health and addiction patient-care units.

(d) Diagnostic and therapeutic services

Diagnostic and therapeutic services is comprised of rehabilitation services such as physiotherapy, occupational therapy, respiratory therapy and speech-language pathology, diagnostic, and pharmacy services.

(e) Population and public health

Population and public health is comprised primarily of health promotion and harm reduction services.

(f) Research and education

Research and education is comprised primarily of costs pertaining to formally organized mental health and addiction research and education.

(g) Information technology

Information technology is comprised of costs pertaining to the provision of service and consultation in the design, development, implementation, security and support of information technology services and systems.

(h) Support services

Support services is comprised of building maintenance operations (including utilities), materials management (including purchasing, central warehousing, distribution, and sterilization), housekeeping, patient registration, health records, food services, emergency preparedness, and infection prevention and control.

(i) Administration

Administration is comprised of human resources, finance, communications, and general administration. General administration includes senior executives and many functions such as planning and development, quality assurance, patient safety, insurance, privacy, public relations, risk management, internal audit, legal, and coordination of virtual health services.

Notes to the Financial Statements (continued)

Recovery Alberta

For the nine months ended March 31, 2025

Note 22 Subsequent Events

The Government of Alberta also announced that a new shared services entity is expected to be established and become operational in 2025. This entity will provide corporate and support services for all four provincial health agencies, and service providers. The operational date and the full financial effect, including the impact on Recovery Alberta's assets, liabilities and operations is currently unknown.

Note 23 Approval of Financial Statements

The financial statements were approved by the Recovery Alberta Board on June 3, 2025, and submitted to the Ministry of Mental Health and Addiction.

Schedule 1: Expenses – Detailed by Object

Recovery Alberta

For the nine months ended March 31, 2025

(in thousands)

	2025	
	Budget (Note 4)	Actual
Salaries and benefits	\$ 484,130	\$ 565,117
Contracts with health service providers	292,571	296,170
Drugs and gases	7,557	7,754
Medical supplies	1,517	1,181
Other contracted services	8,956	11,974
Other ^(a)	11,358	8,974
Amortization and loss on disposals/write-downs of tangible capital assets (Note 17)	-	1,049
	\$ 806,089	\$ 892,219
(a) Significant amounts included in Other are:		
Travel	\$ 1,920	\$ 1,607
Office supplies and courier	3,164	1,442
Housekeeping, laundry and linen, staff wearing apparel, plant maintenance and biomedical engineering supplies	1,051	955
Food and dietary supplies	692	890
Equipment expense	959	748
Minor equipment purchases	286	573
Building rent	432	441
Education	284	232
Building and ground expenses	190	198
Licences, fees, and memberships	192	77
Insurance and liability claims	14	24
Telecommunications	28	5
Utilities	2	3
Other	2,144	1,779
	\$ 11,358	\$ 8,974

Schedule 2: Remuneration and Benefits

Recovery Alberta

For the nine months ended March 31, 2025

(in thousands)

Schedule 2A – Board Remuneration

	Term	2025 Remuneration
Board Chair		
Kerry Bales	Since Jul 1, 2024	\$ -
Total Board		
		\$ -

The Board Chair was not remunerated for this position.

Schedule 2B – Executive Remuneration and Benefits

For the Current Fiscal Period	2025						
	FTE ^(a)	Base Salary ^(b, f)	Other Cash Benefits ^(c)	Other Non-Cash Benefits ^(d)	Subtotal	Severance ^(e)	Total
Board Direct Reports							
Kerry Bales – Chief Executive Officer ^(g, p)	0.75	\$ 319	\$ 5	\$ 79	\$ 403	\$ -	\$ 403
CEO Direct Reports							
Chief Operating Officer ^(h, q)	0.58	163	1	46	210	-	210
Interim Chief Medical Officer ^(i, r)	0.40	212	-	-	212	-	212
Lead, Communications ^(j, q)	0.33	48	-	16	64	-	64
General Counsel ^(k, q)	0.35	70	3	18	91	-	91
Lead, Strategy, Planning and Performance ^(l, q)	0.58	86	5	22	113	-	113
Lead, Medical Affairs ^(m, q)	0.58	91	-	24	115	-	115
Lead, People and Culture ^(n, q)	0.58	99	-	25	124	-	124
Lead, Finance ^(o, q)	0.58	91	-	29	120	-	120
Total Executive							
	4.73	\$ 1,179	\$ 14	\$ 259	\$ 1,452	\$ -	\$ 1,452
Management Reporting to CEO Direct Reports							
	3.37	\$ 708	\$ 3	\$ 163	\$ 874	\$ -	\$ 874

Schedule 2: Remuneration and Benefits (continued)

Recovery Alberta

For the nine months ended March 31, 2025

(in thousands)

Schedule 2C – Executive Supplemental Pension Plan

Certain employees will receive retirement benefits that supplement the benefits limited under the registered plans for service. The Supplemental Pension Plan (SPP) is a defined contribution plan. The amounts in this table represent the total SPP benefits earned by the individual during the fiscal period. The current period benefit costs for SPP included in other non-cash benefits disclosed in Schedule 2B are prorated for the period of time the individual was in their position directly reporting to the Board or directly reporting to the Chief Executive Officer. Only individuals holding a position directly reporting to the Board or Chief Executive Officer during the current fiscal period are disclosed.

	2025				
	SPP				
	Current Period Benefit Costs ⁽¹⁾	Total	Account Balance ⁽²⁾ July 1, 2024	Change During the Period ⁽³⁾	Account Balance ⁽²⁾ March 31, 2025
Kerry Bales - Chief Executive Officer ⁽⁴⁾	\$ 15	\$ 15	\$ -	\$ 320	\$ 320
Chief Operating Officer ⁽⁴⁾	4	4	-	7	7
Interim Chief Medical Officer ⁽⁵⁾	-	-	-	-	-
Lead, Communications ⁽⁶⁾	-	-	-	-	-
General Counsel ⁽⁶⁾	-	-	-	-	-
Lead, Strategy, Planning and Performance ⁽⁶⁾	-	-	-	-	-
Lead, Medical Affairs ⁽⁶⁾	-	-	-	-	-
Lead, People and Culture ⁽⁶⁾	-	-	-	-	-
Lead, Finance ⁽⁶⁾	-	-	-	-	-

- (1) The SPP current period benefit costs are Recovery Alberta contributions earned in the period.
- (2) The account balance represents the total cumulative earned contributions to the SPP as well as cumulative investment gains or losses on the contributions.
- (3) Changes in the account balance include the current benefit costs and investment gains or losses related to the account.
- (4) The incumbent's account balance transitioned from AHS effective September 1, 2024.
- (5) The incumbent is not an employee of Recovery Alberta, and therefore not eligible for enrolment in the SPP.
- (6) The incumbent's pensionable earnings were below the threshold for enrolment in the SPP.

Schedule 2: Remuneration and Benefits (continued)

Recovery Alberta

For the nine months ended March 31, 2025

(in thousands)

Footnotes to the Schedules of Remuneration and Benefits

Definitions

- a. For this schedule, full time equivalents (FTE) are determined by actual hours earned divided by 2,022.75 annual base hours.
- b. Base salary is regular salary and includes all payments earned related to actual hours earned other than those reported as other cash benefits.
- c. Other cash benefits include, as applicable, honoraria, acting pay, membership fees, car allowance, and lump sum payments. Relocation expenses are excluded from compensation disclosure as they are considered to be recruiting costs to Recovery Alberta and not part of compensation unless related to severance. Expense reimbursements are also excluded from compensation disclosure except where the expenses meet the definition of the other cash benefits listed above.
- d. Other non-cash benefits include:
 - Employer's current period benefit costs of the supplemental pension plan as defined in Schedule 2C
 - Employer's share of employee benefit contributions and payments made on behalf of employees including pension, health care, dental and vision coverage, out-of-country medical benefits, group life insurance, accidental disability and dismemberment insurance, and long and short-term disability plans
 - Vacation accruals, and
 - Employer's share of the cost of additional benefits including sabbaticals or other special leave with pay.
- e. Severance includes direct or indirect payments to individuals upon termination which are not included in other cash benefits or other non-cash benefits.

Executive

- f. Base salary reported for executives are the actual payments earned during the period, and is therefore contingent on the number of Recovery Alberta's work days in the period. For the period ended March 31, 2025, the number of work days at Recovery Alberta was 196.
- g. Kerry Bales was appointed to the position of Chief Executive Officer effective July 1, 2024. The incumbent was also simultaneously appointed Board Chair.
- h. The incumbent held the position of Interim Chief Program Officer, Addiction and Mental Health and Correctional Health Services at AHS as at September 1, 2024 at which time the position transitioned to Recovery Alberta as per Ministerial Order 801/2024, and they became a direct report to the Chief Executive Officer. The incumbent was subsequently appointed to the position of Chief Operating Officer effective November 11, 2024.
- i. The incumbent was appointed Interim Chief Medical Officer effective September 1, 2024. The incumbent's remuneration is as per the terms of a Medical Administrative Services Agreement.
- j. The incumbent was appointed to the position of Lead, Communications effective December 2, 2024.
- k. The incumbent was appointed to the position of General Counsel effective November 25, 2024.
- l. The incumbent held the position of Interim Director, Addiction and Mental Health Transition at AHS as at September 1, 2024 at which time the position transitioned to Recovery Alberta as per Ministerial Order 801/2024, and they became a direct report to the Chief Executive Officer. The incumbent was subsequently appointed to the position of Lead, Strategy, Planning and Performance effective January 20, 2025.
- m. The incumbent held the position of Theme Lead, Medical Affairs at AHS as at September 1, 2024 at which time the position transitioned to Recovery Alberta as per Ministerial Order 801/2024, and they became a direct report to the Chief Executive Officer. The incumbent was subsequently appointed to the position of Lead, Medical Affairs effective January 20, 2025.
- n. The incumbent held the position of Theme Lead, People and Culture at AHS as at September 1, 2024 at which time the position transitioned to Recovery Alberta as per Ministerial Order 801/2024, and they became a direct report to the Chief Executive Officer. The incumbent was subsequently appointed to the position of Lead, People and Culture effective January 20, 2025.
- o. The incumbent held the position of Theme Lead, Finance and Corporate Services at AHS as at September 1, 2024 at which time the position transitioned to Recovery Alberta as per Ministerial Order 801/2024, and they became a direct report to the Chief Executive Officer. The incumbent was subsequently appointed to the position of Lead, Finance effective January 20, 2025.

Termination Obligations

- p. In the case of termination without just cause by Recovery Alberta, the incumbent shall receive severance pay in the amount of the lesser of (i) 52 weeks' salary and benefits; or (ii) the greater of the number of weeks between the date of termination and the term date of June 30, 2030; or the minimum notice period required by the Alberta *Employee Standards Code*.
- q. The incumbent's termination benefits have not been predetermined, and as such is deemed to be the minimum notice period required by the Alberta *Employee Standards Code*.
- r. There is no severance associated with the Medical Administrative Services Agreement.

Schedule 3: Segment Disclosures

Recovery Alberta

For the nine months ended March 31, 2025

(in thousands)

	2025							
	Salaries and benefits	Contracts with health service providers	Drugs and gases	Medical supplies	Other contracted services	Other	Amortization and loss on disposals/write-downs of tangible capital assets	Total
Continuing care	\$ 341	\$ 2,347	\$ -	\$ -	\$ -	\$ -	\$ -	\$ 2,688
Community care	281,518	116,947	4,543	423	5,175	5,394	507	414,507
Acute care	218,534	22,128	3,207	708	4,366	2,283	420	251,646
Diagnostic and therapeutic services	46,925	22,812	4	40	486	500	20	70,787
Population and public health	3,151	140	-	2	-	44	9	3,346
Research and education	1,493	2,043	-	-	-	6	-	3,542
Information technology	642	24,546	-	-	-	-	-	25,188
Support services	7,693	84,068	-	5	847	618	83	93,314
Administration	4,820	21,139	-	3	1,100	129	10	27,201
Total	\$ 565,117	\$ 296,170	\$ 7,754	\$ 1,181	\$ 11,974	\$ 8,974	\$ 1,049	\$ 892,219

Canadian Centre of Recovery Excellence Financial Statements



Table of Contents

Management's Responsibility for Financial Reporting	110
Independent Auditor's Report	111
Statement of Operations	114
Statement of Financial Position	115
Statement of Change in Net Financial Assets	116
Statement of Cash Flows.....	117
Notes to the Financial Statements	118
Schedule 1: Expenses – Detailed by Object	124
Schedule 2: Salary and Benefits Disclosure	125
Schedule 3: Actual Results Compared with Budget.....	126
Schedule 4: Related Party Transactions.....	127

Management’s Responsibility for Financial Reporting

The accompanying financial statements are the responsibility of management and have been reviewed and approved by Senior Management. The financial statements were prepared in accordance with Canadian Public Sector Accounting Standards, and of necessity, include some amounts that are based on estimates and judgement.

To discharge its responsibility for the integrity and objectivity of financial reporting, management maintains a system of internal accounting controls comprising written policies, standards and procedures, and a formal authorization structure. This system provides management with reasonable assurance that transactions are in accordance with governing legislation and are properly authorized, reliable financial records are maintained, and assets are adequately safeguarded.

The Canadian Centre of Recovery Excellence’s Board of Directors meets with management and the Auditor General of Alberta to review financial matters and approves the financial statements upon finalization of the audit. The Auditor General of Alberta has open and complete access to the Board of Directors.

The Auditor General of Alberta provides an independent audit of the financial statements. His examination is conducted in accordance with Canadian Generally Accepted Auditing Standards and includes tests and procedures, which allow him to report on the fairness of the financial statements prepared by management.

On behalf of the Canadian Centre of Recovery Excellence.

[Original signed by Kym Kauffman]

Chief Executive Officer
Kym Kauffman
May 14, 2025

[Original signed by Naren Makwana]

Executive Director, Finance & Corporate Services
Naren Makwana
May 14, 2025

Independent Auditor's Report



**Auditor
General**
OF ALBERTA

To the Board of Directors of the Canadian Centre of Recovery Excellence

Report on the Financial Statements

Opinion

I have audited the financial statements of the Canadian Centre of Recovery Excellence, which comprise the statement of financial position as at March 31, 2025, and the statements of operations, change in net financial assets, and cash flows for the period from June 21, 2024 to March 31, 2025, and notes to the financial statements, including a summary of significant accounting policies.

In my opinion, the accompanying financial statements present fairly, in all material respects, the financial position of the Canadian Centre of Recovery Excellence as at March 31, 2025, and the results of its operations, its changes in net financial assets, and its cash flows for the period then ended in accordance with Canadian public sector accounting standards.

Basis for opinion

I conducted my audit in accordance with Canadian generally accepted auditing standards. My responsibilities under those standards are further described in the *Auditor's Responsibilities for the Audit of the Financial Statements* section of my report. I am independent of the Canadian Centre of Recovery Excellence in accordance with the ethical requirements that are relevant to my audit of the financial statements in Canada, and I have fulfilled my other ethical responsibilities in accordance with these requirements. I believe that the audit evidence I have obtained is sufficient and appropriate to provide a basis for my opinion.

Other information

Management is responsible for the other information. The other information comprises the information included in the *Annual Report*, but does not include the financial statements and my auditor's report thereon. The *Annual Report* is expected to be made available to me after the date of this auditor's report.

My opinion on the financial statements does not cover the other information and I do not express any form of assurance conclusion thereon.

In connection with my audit of the financial statements, my responsibility is to read the other information identified above and, in doing so, consider whether the other information is materially inconsistent with the financial statements or my knowledge obtained in the audit, or otherwise appears to be materially misstated.

If, based on the work I will perform on this other information, I conclude that there is a material misstatement of this other information, I am required to communicate the matter to those charged with governance.

Responsibilities of management and those charged with governance for the financial statements

Management is responsible for the preparation and fair presentation of the financial statements in accordance with Canadian public sector accounting standards, and for such internal control as management determines is necessary to enable the preparation of the financial statements that are free from material misstatement, whether due to fraud or error.

In preparing the financial statements, management is responsible for assessing the Canadian Centre of Recovery Excellence's ability to continue as a going concern, disclosing, as applicable, matters related to going concern and using the going concern basis of accounting unless an intention exists to liquidate or to cease operations, or there is no realistic alternative but to do so.

Those charged with governance are responsible for overseeing the Canadian Centre of Recovery Excellence's financial reporting process.

Auditor's responsibilities for the audit of the financial statements

My objectives are to obtain reasonable assurance about whether the financial statements as a whole are free from material misstatement, whether due to fraud or error, and to issue an auditor's report that includes my opinion. Reasonable assurance is a high level of assurance, but is not a guarantee that an audit conducted in accordance with Canadian generally accepted auditing standards will always detect a material misstatement when it exists. Misstatements can arise from fraud or error and are considered material if, individually or in the aggregate, they could reasonably be expected to influence the economic decisions of users taken on the basis of these financial statements.

As part of an audit in accordance with Canadian generally accepted auditing standards, I exercise professional judgement and maintain professional skepticism throughout the audit. I also:

- Identify and assess the risks of material misstatement of the financial statements, whether due to fraud or error, design and perform audit procedures responsive to those risks, and obtain audit evidence that is sufficient and appropriate to provide a basis for my opinion. The risk of not detecting a material misstatement resulting from fraud is higher than for one resulting from error, as fraud may involve collusion, forgery, intentional omissions, misrepresentations, or the override of internal control.
- Obtain an understanding of internal control relevant to the audit in order to design audit procedures that are appropriate in the circumstances, but not for the purpose of expressing an opinion on the effectiveness of the Canadian Centre of Recovery Excellence's internal control.
- Evaluate the appropriateness of accounting policies used and the reasonableness of accounting estimates and related disclosures made by management.
- Conclude on the appropriateness of management's use of the going concern basis of accounting and, based on the audit evidence obtained, whether a material uncertainty exists related to events or conditions that may cast significant doubt on the Canadian Centre of Recovery Excellence's ability to continue as a going concern. If I conclude that a material uncertainty exists, I am required to draw attention in my auditor's report to the related disclosures in the financial statements or, if such disclosures are inadequate, to modify my opinion. My conclusions are based on the audit evidence obtained up to the date of my auditor's report. However, future events or conditions may cause the Canadian Centre of Recovery Excellence to cease to continue as a going concern.

- Evaluate the overall presentation, structure and content of the financial statements, including the disclosures, and whether the financial statements represent the underlying transactions and events in a manner that achieves fair presentation.

I communicate with those charged with governance regarding, among other matters, the planned scope and timing of the audit and significant audit findings, including any significant deficiencies in internal control that I identify during my audit.

[Original signed by W. Doug Wylie FCPA, FCMA, ICD.D]
Auditor General

May 14, 2025
Edmonton, Alberta

Statement of Operations

Canadian Centre of Recovery Excellence For the Period June 21, 2024 to March 31, 2025

(in thousands)

	2025	
	Budget	Actual
	(Note 4)	
Revenues (Note 2(b))		
Government of Alberta grants	\$ 4,300	\$ 4,300
Investment income	-	87
	4,300	4,387
Expenses (Note 2(b) and Schedule 1)		
Administration	1,837	810
Evaluation and Research	1,914	1,226
Communications and Engagement	549	183
	4,300	2,219
Annual operating surplus	-	2,168
Accumulated operating surplus, beginning of period	-	-
Accumulated operating surplus, end of period	\$ -	\$ 2,168

The accompanying notes and schedules are part of these financial statements.

Statement of Financial Position

Canadian Centre of Recovery Excellence
For the Period June 21, 2024 to March 31, 2025

(in thousands)

	2025
Financial Assets	
Cash (Note 5)	\$ 2,941
Accounts receivable	8
	<u>2,949</u>
Liabilities	
Accounts payable and other accrued liabilities	441
Unspent deferred contributions (Note 7)	688
	<u>1,129</u>
Net Financial Assets	<u>1,820</u>
Non-Financial Assets	
Tangible capital assets (Note 9)	342
Prepaid expenses	6
	<u>348</u>
Net Assets	<u>\$ 2,168</u>
Net Assets	
Accumulated operating surplus	<u>\$ 2,168</u>
 Contractual obligations (Note 10)	

The accompanying notes and schedules are part of these financial statements.

Statement of Change in Net Financial Assets

Canadian Centre of Recovery Excellence For the Period June 21, 2024 to March 31, 2025

(in thousands)

	2025	
	Budget	Actual
	(Note 4)	
Annual operating surplus	\$ -	\$ 2,168
Acquisition of tangible capital assets (Note 9)	-	(345)
Amortization of tangible capital assets (Note 9)	-	3
(Increase) in prepaid expenses	-	(6)
Increase in net financial assets in the period	-	1,820
Net financial assets, beginning of period	-	-
Net financial assets, end of period	\$ -	\$ 1,820

The accompanying notes and schedules are part of these financial statements.

Statement of Cash Flows

Canadian Centre of Recovery Excellence For the Period June 21, 2024 to March 31, 2025

(in thousands)

	2025
Operating Transactions	
Annual operating surplus	\$ 2,168
Non-cash items included in annual operating surplus:	
Amortization of tangible capital assets (Note 9)	3
(Increase) in accounts receivable	(8)
(Increase) in prepaid expenses	(6)
Increase in accounts payable and other accrued liabilities	441
Increase in unspent deferred contributions (Note 7)	688
Cash provided by operating transactions	<u>3,286</u>
Capital Transactions	
Acquisition of tangible capital assets (Note 9)	<u>(345)</u>
Cash applied to capital transactions	<u>(345)</u>
Increase in cash	2,941
Cash at beginning of period	<u>-</u>
Cash at end of period	<u>\$ 2,941</u>

The accompanying notes and schedules are part of these financial statements.

Notes to the Financial Statements

Canadian Centre of Recovery Excellence

Year ended March 31, 2025

(in thousands)

Note 1 AUTHORITY

The Canadian Centre of Recovery Excellence (CoRE) is a crown corporation established on June 21, 2024 under the *Canadian Centre of Recovery Excellence Act*.

Pursuant with the act, CoRE has a mandate to empower decision makers with research and information to advance recovery-oriented mental health and addiction services. CoRE does this by informing best practices, conducting program evaluation and analyzing data to support the development of objective and practical policy.

CoRE is exempt from income taxes under the *Income Tax Act*.

Comparative figures are not presented as this is CoRE's first period of operations.

Note 2 SUMMARY OF SIGNIFICANT ACCOUNTING POLICIES AND REPORTING PRACTICES

These financial statements are prepared in accordance with Canadian Public Sector Accounting Standards (PSAS).

(a) Reporting Entity

The financial statements reflect the assets, liabilities, revenues and expenses of CoRE.

(b) Basis of Financial Reporting

Revenues

All revenues are reported on the accrual basis of accounting.

Cash received for which services have not been provided by year end is recognized as unearned revenue.

Revenues from transactions with no performance obligations are recognized at their realizable value when CoRE has the authority to claim or retain an inflow of economic resources and identifies a past transaction or event that gives rise to an asset.

Government of Alberta grants

Transfers from the Government of Alberta are referred to as provincial grants.

Provincial grants and associated externally restricted investment income are recognized as deferred contributions when the stipulations together with CoRE's actions and communications as to the use of the grants, create a liability. These grants are recognized as revenue as the stipulations are met and, when applicable, CoRE complies with its communicated use of these grants.

All other provincial grants, without stipulations for the use of the transfer, are recognized as revenue when the grant is authorized and CoRE meets the eligibility criteria (if any).

Investment income

Investment income includes interest income held at a Canadian chartered bank.

Notes to the Financial Statements (continued)

Canadian Centre of Recovery Excellence

Year ended March 31, 2025

(in thousands)

Note 2 SUMMARY OF SIGNIFICANT ACCOUNTING POLICIES AND REPORTING PRACTICES (CONT'D)

(b) Basis of Financial Reporting (Cont'd)

Expenses

Expenses are reported on an accrual basis. The cost of all goods consumed and services received during the year are expensed.

Valuation of Financial Assets and Liabilities

CoRE's financial assets and liabilities are generally measured as follows:

<u>Financial Statement Component</u>	<u>Measurement</u>
Cash	Cost
Accounts receivable	Lower of cost or net recoverable value
Accounts payable and other accrued liabilities	Cost

CoRE does not have any financial instruments classified in the fair value category and does not hold equities traded in an active market, nor engage in derivative contracts or foreign currency transactions. CoRE is not exposed to remeasurement gains or losses and, consequently, a statement of remeasurement gains and losses is not presented.

Financial Assets

Financial assets are assets that could be used to discharge existing liabilities or finance future operations and are not for consumption in the normal course of operations.

Financial assets are CoRE's financial claims on external organizations and individuals at the year end.

Cash

Cash is held at a Canadian chartered bank.

Accounts Receivable

Accounts receivable are recognized at the lower of cost or net recoverable value. A valuation allowance is recognized when recovery is uncertain.

Liabilities

Liabilities are present obligations of CoRE to external organizations and individuals arising from past transactions or events occurring before year end, the settlement of which is expected to result in the future sacrifice of economic benefits. They are recognized when there is an appropriate basis of measurement and management can reasonably estimate the amounts.

Non-Financial Assets

Non-financial assets are acquired, constructed, or developed assets that do not normally provide resources to discharge existing liabilities, but instead:

- (a) are normally employed to deliver services;
- (b) may be consumed in the normal course of operations; and
- (c) are not for sale in the normal course of operations.

Non-financial assets are limited to tangible capital assets and prepaid expenses.

Notes to the Financial Statements (continued)

Canadian Centre of Recovery Excellence

Year ended March 31, 2025

(in thousands)

**Note 2 SUMMARY OF SIGNIFICANT ACCOUNTING POLICIES AND REPORTING PRACTICES
(CONT'D)**

(b) Basis of Financial Reporting (Cont'd)

Tangible Capital Assets

Tangible capital assets are recognized at cost less amortization, which includes amounts that are directly related to the acquisition, design, construction, development, improvement or betterment of the assets. Cost includes overhead directly attributable to construction and development, as well as interest costs that are directly attributable to the acquisition or construction of the asset.

The cost, less residual value, of the tangible capital assets, excluding work-in-progress, is amortized on a straight-line basis over their estimated useful lives as follows:

Furniture and office equipment	10 years
Computer hardware and software	5 years

Tangible capital assets are written down when conditions indicate that they no longer contribute to CoRE's ability to provide services, or when the value of future economic benefits associated with the tangible capital assets are less than their book value. The net write-downs are accounted for as expenses in the Statement of Operations.

Prepaid Expenses

Prepaid expenses are recognized at cost and amortized based on the terms of the agreement.

Measurement Uncertainty

Measurement uncertainty exists when there is a variance between the recognized or disclosed amount and another reasonably possible amount. The amounts recognized for amortization of tangible capital assets are based on estimates of the useful life of the related assets. Actual results could differ from estimates.

Note 3 FUTURE CHANGES IN ACCOUNTING STANDARDS

On April 1, 2026, CoRE will adopt the following new conceptual framework and accounting standard approved by the Public Sector Accounting Board:

- **The Conceptual Framework for Financial Reporting in the Public Sector**

The Conceptual Framework is the foundation for public sector financial reporting standard setting. It replaces the conceptual aspects of Section PS 1000 Financial Statement Concepts and Section PS 1100 Financial Statement Objectives. The conceptual framework highlights considerations fundamental for the consistent application of accounting issues in the absence of specific standards.

- **PS 1202 Financial Statement Presentation**

Section PS 1202 sets out general and specific requirements for the presentation of information in general purpose financial statements. The financial statement presentation principles are based on the concepts within the Conceptual Framework.

Management is currently assessing the impact of the conceptual framework and the standard on the financial statements.

Notes to the Financial Statements (continued)

Canadian Centre of Recovery Excellence

Year ended March 31, 2025

(in thousands)

Note 4 BUDGET

CoRE's 2024 - 2025 operating budget was submitted to and approved by the Minister of Mental Health and Addiction.

Note 5 CASH

Cash is held in an account with a Canadian chartered bank and earns interest calculated based on the average monthly cash balance. The funds are available to be withdrawn upon request. CoRE's interest rate arrangement became effective on August 1, 2024. During the 8-month period ended March 31, 2025, CoRE earned interest at an annual average rate of 3.60%.

Cash includes restricted funds of \$688 as reflected in unspent deferred contributions (Note 7).

Note 6 FINANCIAL RISK MANAGEMENT

CoRE has the following financial instruments: cash, accounts receivable, accounts payable and other accrued liabilities.

CoRE has exposure to the following risks from its use of financial instruments: interest rate risk, liquidity risk and credit risk. CoRE does not have exposure to price risk as it is fully funded by the Government of Alberta. CoRE does not have exposure to foreign exchange risk as its foreign transactions are insignificant.

(a) Interest rate risk

Interest rate risk is the risk that the rate of return and future cash flows on CoRE's cash held in bank accounts will fluctuate because of changes in market interest rates. As CoRE holds bank deposits that are interest bearing and accounts payable that are non-interest bearing, the Corporation is not exposed to significant interest rate risk relating to its financial assets and liabilities.

(b) Liquidity risk

Liquidity risk is the risk that CoRE will encounter difficulty in meeting obligations associated with financial liabilities. CoRE enters into transactions to purchase goods and services on credit. Liquidity risk is measured by reviewing CoRE's future net cash flows for the possibility of negative net cash flow. CoRE manages the liquidity risk resulting from its accounts payable obligations by maintaining adequate cash resources in interest bearing bank deposits.

(c) Credit Risk

Credit risk is the risk of loss arising from the failure of a counterparty to fully honor its contractual obligations. CoRE is not exposed to any significant credit risk from potential non-payment of accounts receivable. As at March 31, 2025, the balance of accounts receivable does not contain amounts that are uncollectible.

Notes to the Financial Statements (continued)

Canadian Centre of Recovery Excellence

Year ended March 31, 2025

(in thousands)

Note 7 UNSPENT DEFERRED CONTRIBUTIONS

	2025
	Ministry of Mental Health and Addiction
Balance at beginning of the period	\$ -
Cash received or receivable during the period	688
Recognized as revenue	-
Balance at end of period	\$ 688

The unspent deferred contribution from the Ministry of Mental Health and Addiction at the end of the period is restricted to conducting initial population based mental health and addiction surveys.

Note 8 BENEFIT PLAN

CoRE participates in the Local Authorities Pension Plan (LAPP), a multi-employer defined benefit pension plan. CoRE accounts for this multi-employer pension plan on a defined contribution basis. CoRE is not responsible for future funding of the plan deficit other than through contribution increases. Pension expense recorded in the financial statements is equivalent to CoRE's annual contributions of \$60 for the period ended March 31, 2025.

At December 31, 2024, the Local Authorities Pension Plan reported a surplus of \$19,557,148 (2023 – surplus of \$15,056,661).

Note 9 TANGIBLE CAPITAL ASSETS

	2025		
	Furniture & Equipment	Computer Hardware & Software	Total
Estimated Useful Life	10 years	5 years	
Historical Cost			
Beginning of the period	\$ -	\$ -	\$ -
Additions	209	136	345
	209	136	345
Accumulated Amortization			
Beginning of the period	-	-	-
Amortization expense	-	3	3
	-	3	3
Net book value at March 31, 2025	\$ 209	\$ 133	\$ 342

Notes to the Financial Statements (continued)

Canadian Centre of Recovery Excellence

Year ended March 31, 2025

(in thousands)

Note 10 CONTRACTUAL OBLIGATIONS

Contractual obligations are obligations of CoRE to others that will become liabilities in the future when the terms of those contracts or agreements are met.

Estimated payment requirements for each of the next five years and thereafter are as follows:

Year ended March 31	Operating Lease and Contracts
2025 – 26	\$ 196
2026 – 27	219
2027 – 28	228
2028 – 29	235
2029 – 30	241
Thereafter	84
	<u>\$ 1,203</u>

Note 11 APPROVAL OF THE FINANCIAL STATEMENTS

The Board approved CoRE's financial statements on May 14, 2025.

Schedule 1: Expenses – Detailed by Object

Canadian Centre of Recovery Excellence For the Period June 21, 2024 to March 31, 2025

(in thousands)

	2025	
	Budget (Note 4)	Actual
Salaries, wages, and employee benefits	\$ 2,293	\$ 1,379
Supplies and services	2,007	837
Amortization of tangible capital assets (Note 9)	-	3
Total expenses	\$ 4,300	\$ 2,219

Schedule 2: Salary and Benefits Disclosure

Canadian Centre of Recovery Excellence For the Period June 21, 2024 to March 31, 2025

(in thousands)

	2025			
	Base Salary ⁽¹⁾	Other Cash Benefits ⁽²⁾	Other Non-Cash Benefits ⁽³⁾	Total
Board Chair & Chief Executive Officer ⁽⁴⁾	\$ 184	\$ 38	\$ 11	\$ 233
Chief Scientific Officer ⁽⁵⁾	223	-	-	223
Chief Information Officer ⁽⁶⁾	29	-	6	35
Chief Privacy Officer / General Counsel ⁽⁷⁾	53	-	11	64
Executive Director, Finance & Corporate Services ⁽⁸⁾	89	-	17	106
Executive Director, Communications & Engagement ⁽⁹⁾	101	-	16	117
	<u>\$ 679</u>	<u>\$ 38</u>	<u>\$ 61</u>	<u>\$ 778</u>

(1) Base salary includes regular salary.

(2) Other cash benefits include pay-in-lieu benefits and vehicle allowance. There were no bonuses paid in 2025.

(3) Other non-cash benefits include employer's share of benefits and contributions or payments made on behalf of employees including pension, health care, dental coverage, and professional memberships.

(4) The incumbent held the position effective July 1, 2024. The Board Chair does not receive remuneration for participation on the Board.

(5) The incumbent held the position effective July 1, 2024. CoRE provides compensation to this incumbent as part of the agreement with their professional corporation.

(6) The incumbent held the position effective February 10, 2025.

(7) The incumbent held the position effective August 12, 2024. The incumbent is employed on a 0.4 Full-Time Equivalent basis.

(8) The incumbent held the position effective September 23, 2024.

(9) The incumbent held the position effective July 31, 2024.

Schedule 3: Actual Results Compared with Budget

Canadian Centre of Recovery Excellence For the Period June 21, 2024 to March 31, 2025

(in thousands)

	Budget (Note 4)	Adjustments ⁽¹⁾	Adjusted Budget	Actual
Revenue				
Government of Alberta grants	\$ 4,300	\$ -	\$ 4,300	\$ 4,300
Investment income	-	-	-	87
	4,300	-	4,300	4,387
Expenses				
Administration	1,837	119	1,956	810
Evaluation and Research	1,914	(22)	1,892	1,226
Communications and Engagement	549	(97)	452	183
	4,300	-	4,300	2,219
Annual Operating Surplus	-	-	-	2,168
Capital				
Capital Investment	-	-	-	345
Less: Amortization of tangible capital assets	-	-	-	(3)
Net capital investment	-	-	-	342
Surplus	\$ -	\$ -	\$ -	\$ 1,826

(1) Adjustments reflect changes to the original budget approved by the Ministry of Mental Health and Addiction.

Schedule 4: Related Party Transactions

Canadian Centre of Recovery Excellence

For the Period June 21, 2024 to March 31, 2025

(in thousands)

Related parties are those entities consolidated or accounted for on a modified equity basis in the Government of Alberta's Consolidated Financial Statements. Related parties also include key management personnel and close family members of those individuals in CoRE. CoRE and its employees paid or collected certain taxes set by regulation. These amounts were incurred in the normal course of business, reflect charges applicable to all users, and have been excluded from this schedule.

CoRE had the following transactions with related parties recorded in the Statement of Operations and the Statement of Financial Position at the amount of consideration agreed upon between the related parties.

	2025
Revenues	
Grants	\$ 4,300
	<u>\$ 4,300</u>
Expenses	
Other services	-
	<u>\$ -</u>
Receivable from related parties	-
	<u>\$ -</u>
Payable to related parties	-
	<u>\$ -</u>
Transferred from related parties	688
	<u>\$ 688</u>

At March 31, 2025, CoRE recorded \$688 in deferred revenue from the Ministry of Mental Health and Addiction related to unspent deferred contributions (Note 7).

Financial Information

Table of Contents

Lapse/Encumbrance (unaudited)129

Deferred Contributions (unaudited)130

Lapse/Encumbrance (unaudited)**Year ended March 31, 2025***(in thousands)*

Expense Vote by Program	Voted Estimate ⁽¹⁾	Supplementary Estimate ⁽²⁾	Adjustments ⁽³⁾	Voted Estimate	Voted Actuals ⁽⁴⁾	Expended/ (Unexpended)
Operating Expense						
1 Ministry Support Services						
1.1 Minister's Office	\$ 822	\$ -	\$ -	\$ 822	\$ 781	\$ (41)
1.2 Deputy Minister's Office	926	-	-	926	833	(93)
1.3 Corporate Services	11,705	120	-	11,825	11,973	148
	<u>13,453</u>	<u>120</u>	<u>-</u>	<u>13,573</u>	<u>13,587</u>	<u>14</u>
2 Mental Health and Addiction						
2.1 Program Support	10,448	-	-	10,448	9,737	(711)
2.2 Compliance and Monitoring	2,061	-	-	2,061	1,991	(70)
2.3 Mental Health Review Panel	2,191	-	-	2,191	1,945	(246)
2.4 Prevention and Early Intervention	30,826	-	(6,500)	24,326	20,832	(3,494)
2.5 Treatment and Recovery	136,819	-	(14,500)	122,319	132,699	10,380
2.6 Initiatives that Reduce Harm	35,784	-	(2,600)	33,184	28,846	(4,338)
2.7 Children and Youth	56,235	-	(12,400)	43,835	37,882	(5,953)
2.8 Mental Health and Addiction Services	1,126,938	137,382	45,190	1,309,510	1,309,510	-
2.9 Centre of Recovery Excellence (CoRE)	5,000	-	-	5,000	4,975	(25)
	<u>1,406,302</u>	<u>137,382</u>	<u>9,190</u>	<u>1,552,874</u>	<u>1,548,417</u>	<u>(4,457)</u>
Capital Grants						
2 Addiction and Mental Health						
2.4 Prevention and Early Intervention	-	-	4,032	4,032	-	(4,032)
2.5 Treatment and Recovery	98,957	-	4,778	103,735	72,530	(31,205)
2.6 Initiatives that Reduce Harm	-	-	-	-	400	400
2.7 Children and Youth	36,000	-	(18,000)	18,000	18,000	-
	<u>134,957</u>	<u>-</u>	<u>(9,190)</u>	<u>125,767</u>	<u>90,930</u>	<u>(34,837)</u>
	<u>1,554,712</u>	<u>137,502</u>	<u>-</u>	<u>1,692,214</u>	<u>1,652,934</u>	<u>(39,280)</u>
	<u>\$ 1,554,712</u>	<u>\$ 137,502</u>	<u>\$ -</u>	<u>\$ 1,692,214</u>	<u>\$ 1,652,934</u>	<u>\$ (39,280)</u>
Capital Investment Vote by Program						
1 Ministry Support Services						
1.3 Corporate Services	25	-	400	425	312	(113)
2 Mental Health and Addiction						
2.5 Treatment and Recovery	5,000	-	600	5,600	955	(4,645)
	<u>5,025</u>	<u>-</u>	<u>1,000</u>	<u>6,025</u>	<u>1,267</u>	<u>(4,758)</u>
	<u>\$ 5,025</u>	<u>\$ -</u>	<u>\$ 1,000</u>	<u>\$ 6,025</u>	<u>\$ 1,267</u>	<u>\$ (4,758)</u>
Contingency Vote by Program						
2 Mental Health and Addiction						
2.5 Treatment and Recovery - Capital Investment	-	-	4,000	4,000	1,772	(2,228)
2.8 Mental Health and Addiction Services	-	-	33,160	33,160	33,160	-
	<u>-</u>	<u>-</u>	<u>37,160</u>	<u>37,160</u>	<u>34,932</u>	<u>\$ (2,228)</u>
Total	<u>\$ 1,559,737</u>	<u>\$ 137,502</u>	<u>\$ 38,160</u>	<u>\$ 1,735,399</u>	<u>\$ 1,689,133</u>	<u>\$ (46,266)</u>
Lapse/(Encumbrance)						<u>\$ (46,266)</u>

⁽¹⁾ As per "Expense Vote by Program" and "Capital Investment Vote by Program" page 165 of the 2024-25 Government Estimates.

⁽²⁾ Per the Supplementary Supply Estimates approved on March 20, 2025.

⁽³⁾ Adjustments include supply vote transfers for "Contingency" as approved by the Lieutenant Governor in Council under the direction of the Minister of Finance. The Contingency supply vote consists of a provisional funding authority transferable to any ministry. Upon approval by the Lieutenant Governor in Council, the President of Treasury Board and Minister of Finance may either spend or transfer all or a portion of this supply vote to another Minister for public emergencies, disasters, or unanticipated costs.

⁽⁴⁾ Actuals exclude non-voted amounts as no cash disbursement is required (non-cash amounts), or because the Legislative Assembly has already provided the funding authority pursuant to a statute other than an appropriation act. Non-cash amounts (such as amortization, accretion, valuation adjustments and other provisions) are excluded as these amounts do not require any expenditure or payment of public money. Year-end expense accruals & payables which will immediately require a cash outlay (payment of public money) to settle or otherwise extinguish the liabilities are included in Actuals.

Deferred Contributions (unaudited)**Year ended March 31, 2025***(in thousands)*

	2025	2024 Restated
Unspent deferred contributions		
Deferred operating contributions	\$ 29,244	\$ 33,900
Deferred capital contributions	14	1,147
⁽¹⁾ Deferred contributions	\$ 29,258	\$ 35,047

⁽¹⁾ Deferred contributions consist primarily of federal government funding and contributions from other Government of Alberta entities.

Annual Report Extracts and Other Statutory Reports

Public Interest Disclosure (Whistleblower Protection) Act

Section 32 of the *Public Interest Disclosure (Whistleblower Protection) Act* reads:

32(1) Every chief officer must prepare a report annually on all disclosures that have been made to the designated officer of the department, public entity or office of the Legislature for which the chief officer is responsible.

(2) The report under subsection (1) must include the following information:

- (a) the number of disclosures received by the designated officer, the number of disclosures acted on and the number of disclosures not acted on by the designated officer;
- (b) the number of investigations commenced by the designated officer as a result of disclosures;
- (c) in the case of an investigation that results in a finding of wrongdoing, a description of the wrongdoing and any recommendations made or corrective measures taken in relation to the wrongdoing or the reasons why no corrective measure was taken.

(3) The report under subsection (1) must be included in the annual report of the department, public entity or office of the Legislature if the annual report is made publicly available.

There were no disclosures of wrongdoing filed with my office for your department between April 1, 2024, and March 31, 2025.